

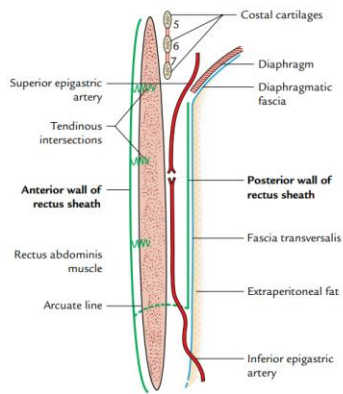


Fig. 3.22 Anterior and posterior walls of rectus sheath as seen in sagittal section. Note: Tendinous intersections are attached only to the anterior wall.

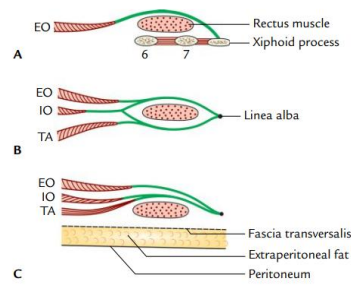


Fig. 3.24 Formation of the rectus sheath as seen in transverse sections through rectus abdominis and its sheaths at three different levels: A, above the costal margin; B, between costal margin and arcuate line; C, below the arcuate line (EO = external oblique, IO = internal oblique, TA = transversus abdominis).

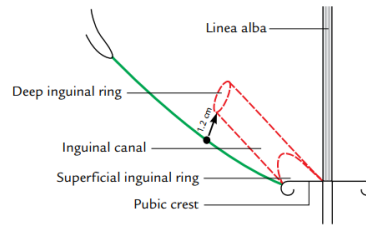


Fig. 4.9 Surface marking of the inguinal canal.

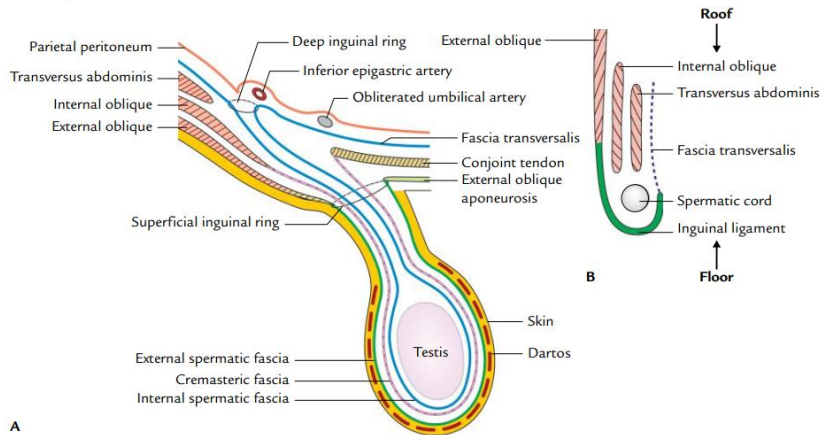


Fig. 4.10 Boundaries of the inguinal canal: A, anterior and posterior walls as seen in coronal section; B, roof and floor as seen in sagittal section.

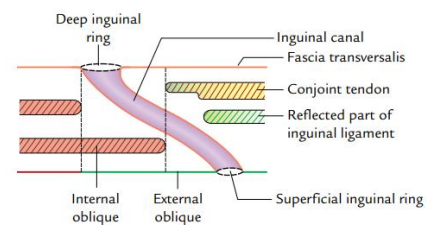


Fig. 4.13 Structures protecting the anterior and posterior walls of the inguinal canal.

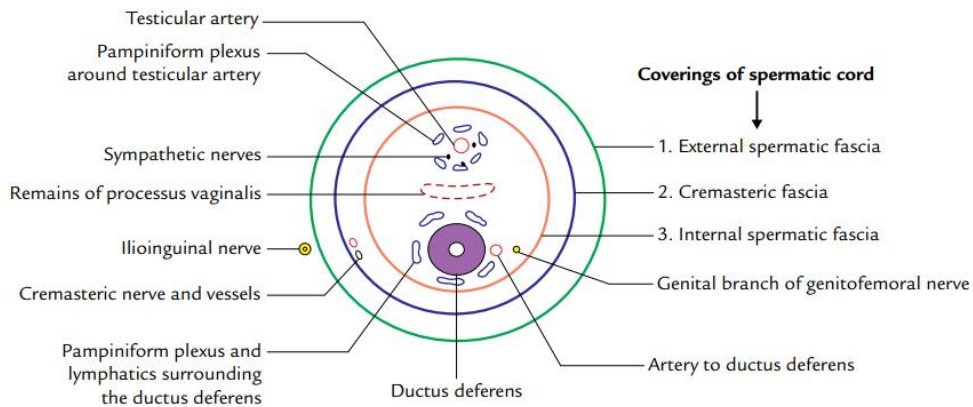


Fig. 4.12 Transverse section of the spermatic cord showing its covering content.

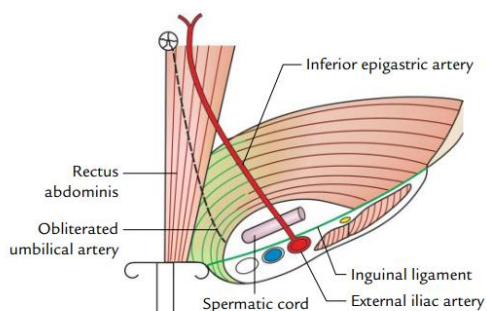


Fig. 4.15 Boundaries of the inguinal (Hesselbach's) triangle.

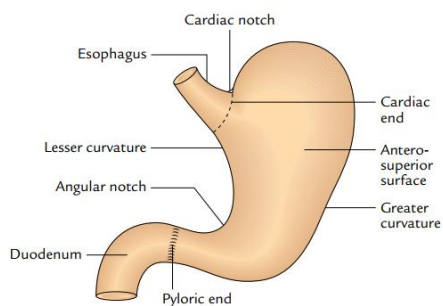


Fig. 7.3 External features of the stomach.

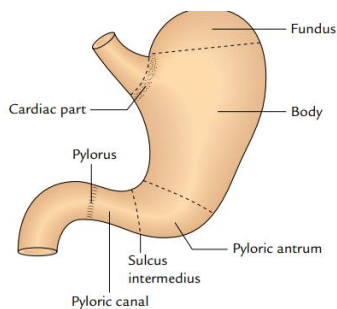
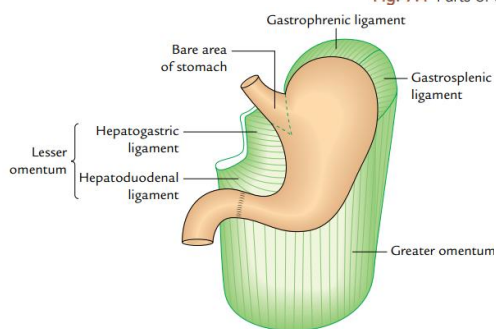


Fig. 7.4 Parts of the stomach.



Peritoneal relations of the stomach.

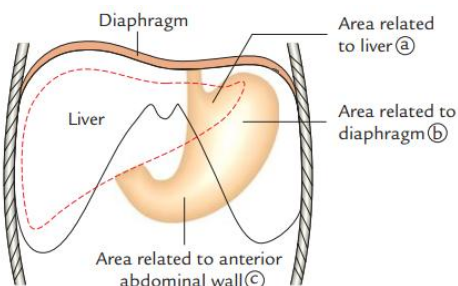
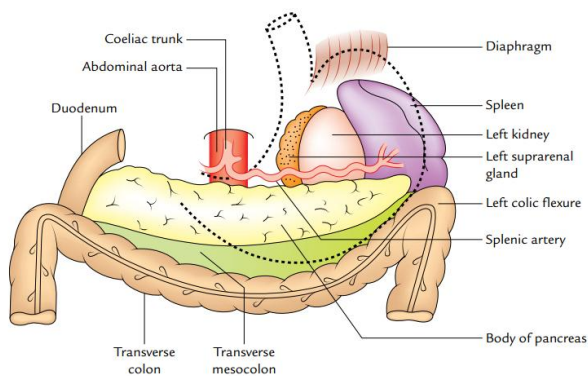
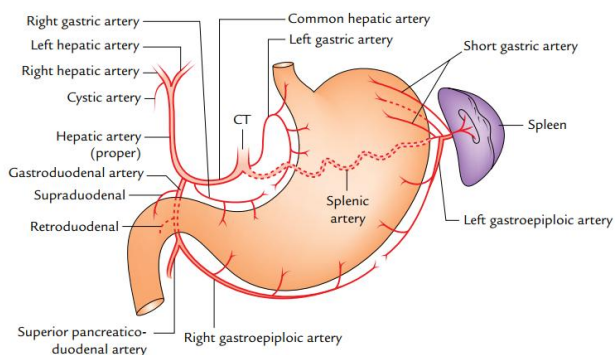


Fig. 7.6 Anterior relations of the stomach.



7.7 The "stomach bed."



2 Arteries of the stomach (CT = coeliac trunk).

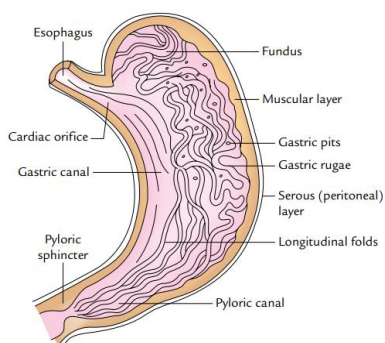
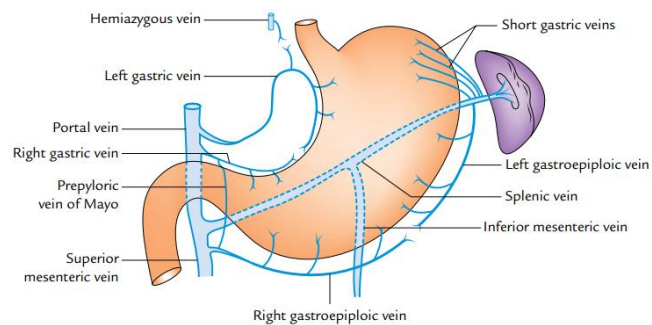
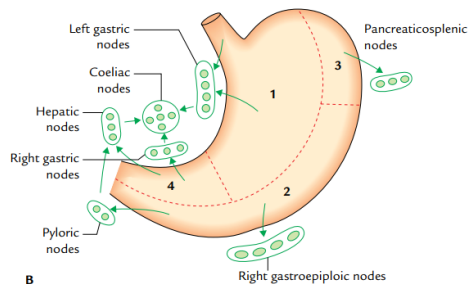


Fig. 7.10 Interior of the stomach showing folds in mucous lining



Venous drainage of the stomach.



h: A, lymphatic territories; B, lymph node groups draining the lymphatic

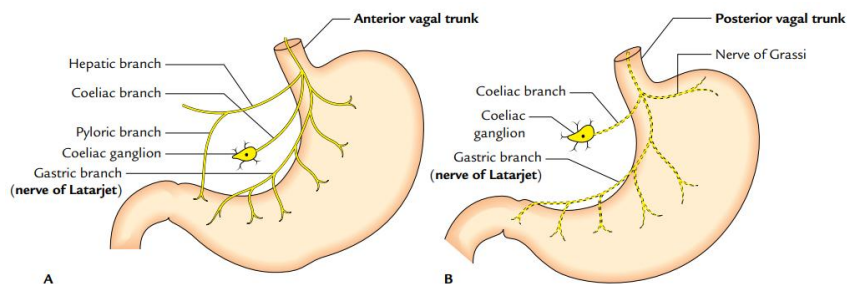


Fig. 7.15 Parasympathetic innervation of the stomach: A, distribution of the anterior vagal trunk; B, distribution of the posterior vagal trunk.

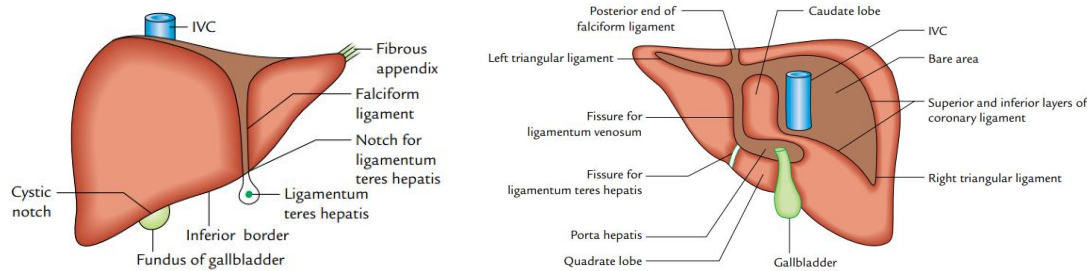


Fig. 8.3 Liver—viewed from the front.

Liver—viewed from behind.

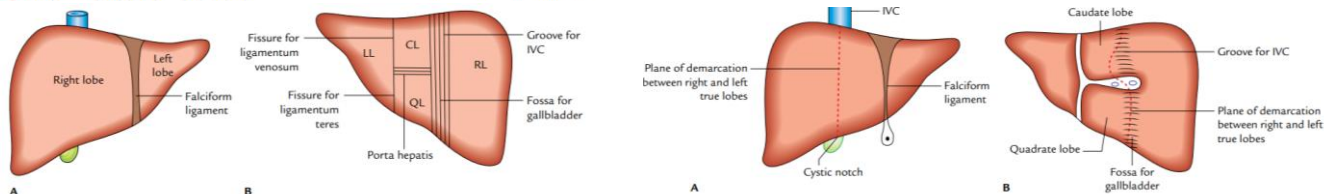
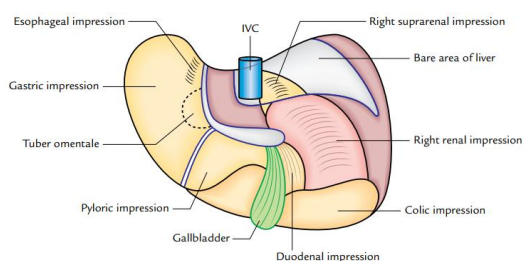


Fig. 8.6 Anatomical lobes of the liver: A, on the diaphragmatic surface of the liver; B, on the visceral surface of the liver. Fig. 8.7 True/physiological lobes of the liver: A, plane of demarcation on anterosuperior surface; B, visceral surface is shown by the interrupted red line. RL = right lobe, LL = left lobe, CL = caudate lobe, QL = quadrate lobe.



Visceral relations of the liver (relations of the inferior/visceral surface of the liver).

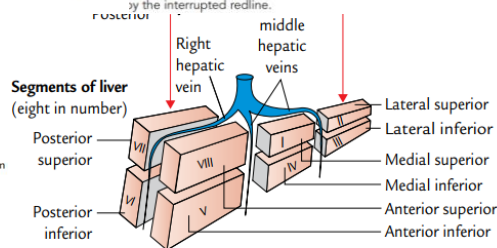
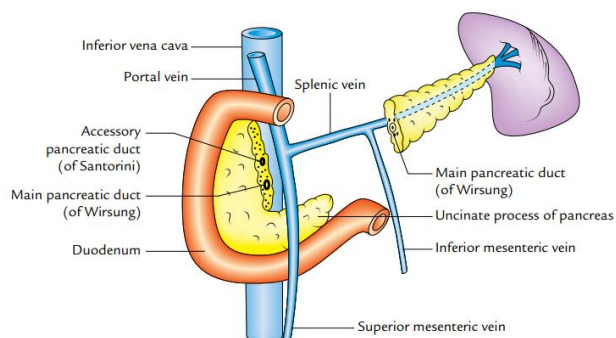


Fig. 8.8 Segments of the liver.



Formation of the portal vein behind the neck of pancreas.

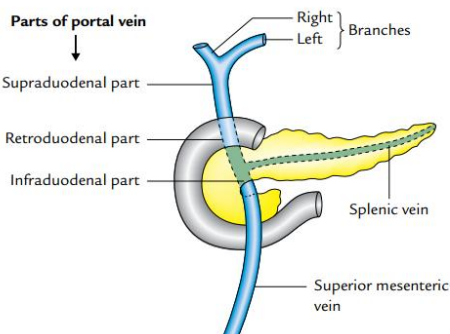
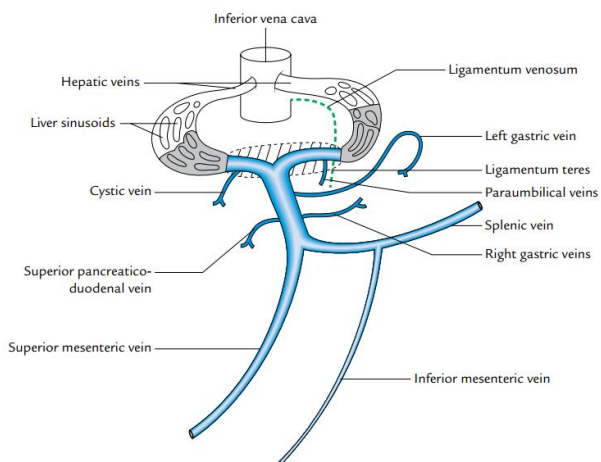


Fig. 9.24 Course and parts of the portal vein.



Tributaries of the portal vein.

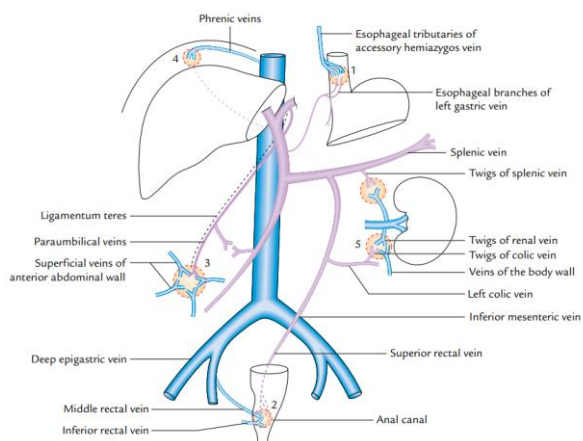


Fig. 9.26 Sites of portocaval anastomosis: 1 = lower end of esophagus; 2 = anal canal; 3 = in the region of umbilicus; 4 = at the bare area of liver; 5 = between the colic veins and the renal veins.

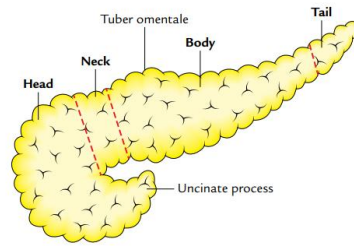
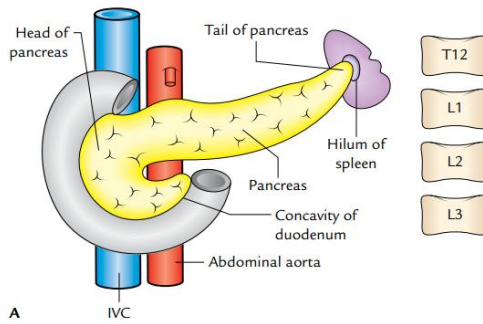
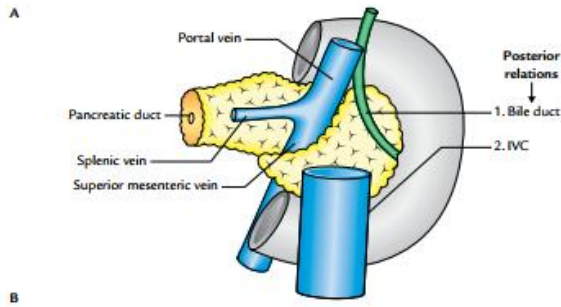
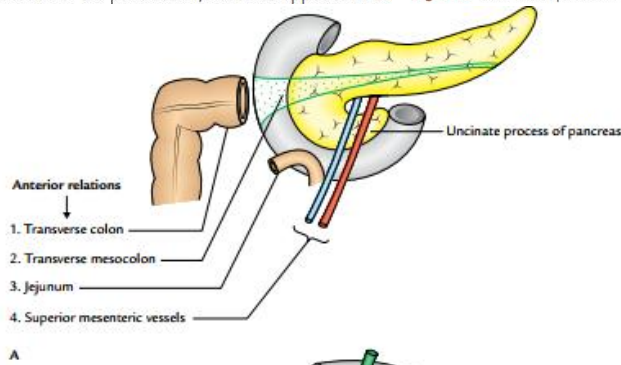


Fig. 9.12 Location of the pancreas: A, head lies opposite L1 and T12. Fig. 9.13 Parts of the pancreas.



9.14 Relations of the head of pancreas: A, anterior relations; B, posterior relations.

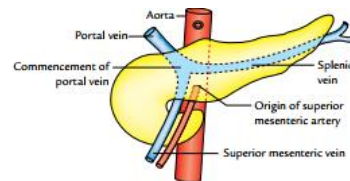


Fig. 9.15 Posterior relations of the neck of pancreas.

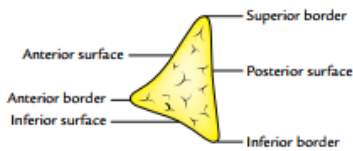


Fig. 9.16 Surfaces and borders of the body of pancreas as seen in cross section.

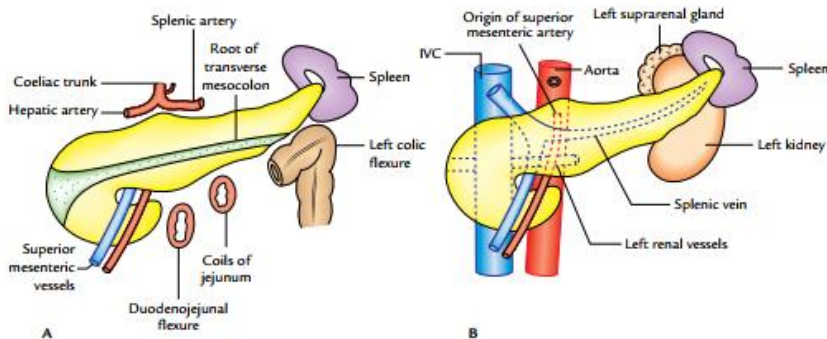


Fig. 9.17 Relations of the body of pancreas: A, anterior and inferior relations; B, posterior relations.

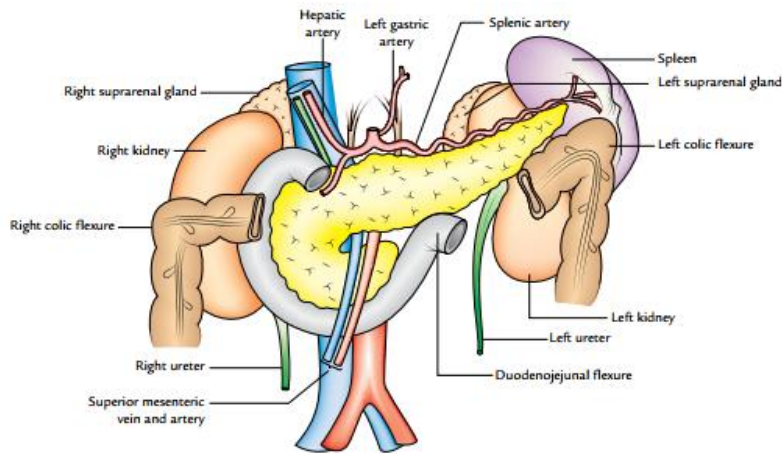
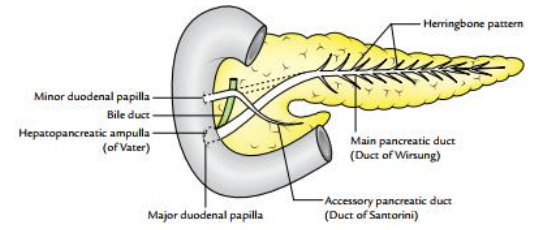


Fig. 9.18 Visceral relations of the different parts of the pancreas.



Pancreatic ducts.

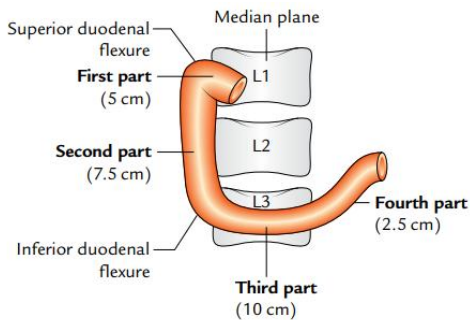
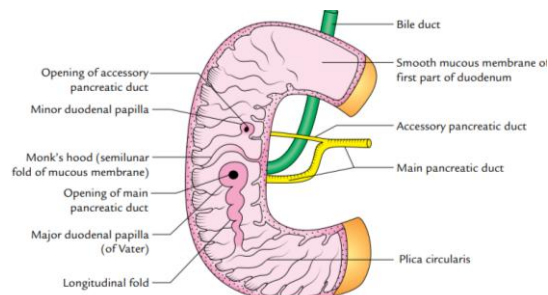


Fig. 9.2 Parts of the duodenum.



Interior of the duodenum.

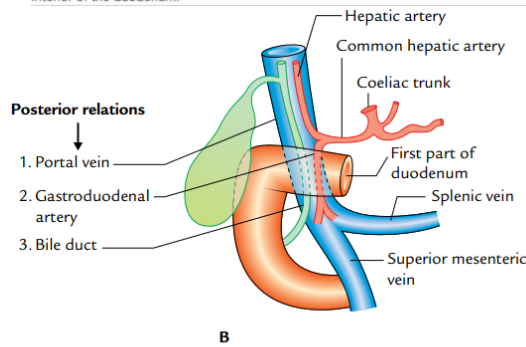
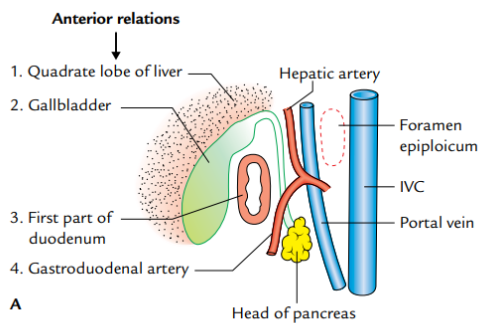


Fig. 9.3 Relations of the first part of the duodenum: A, anterior relations as seen in sectional view; B, posterior relations as seen from anterior aspect (IVC = inferior vena cava).

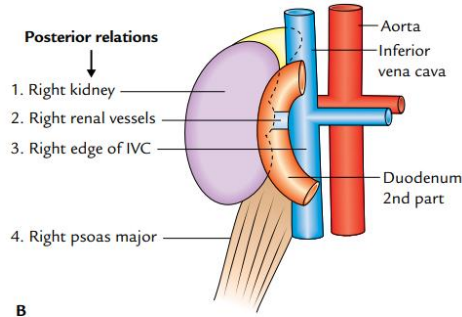
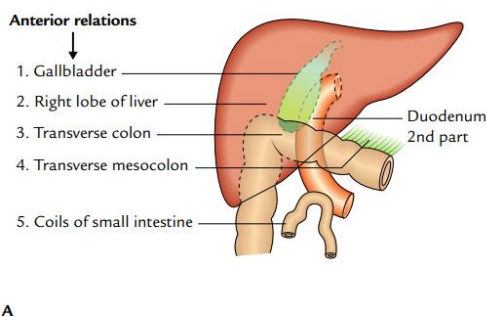
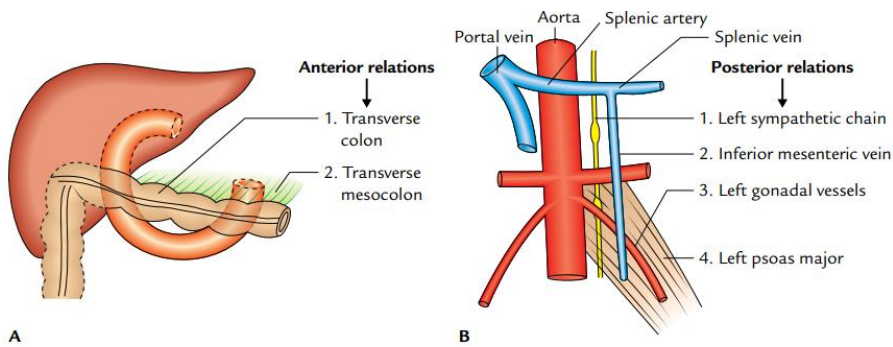
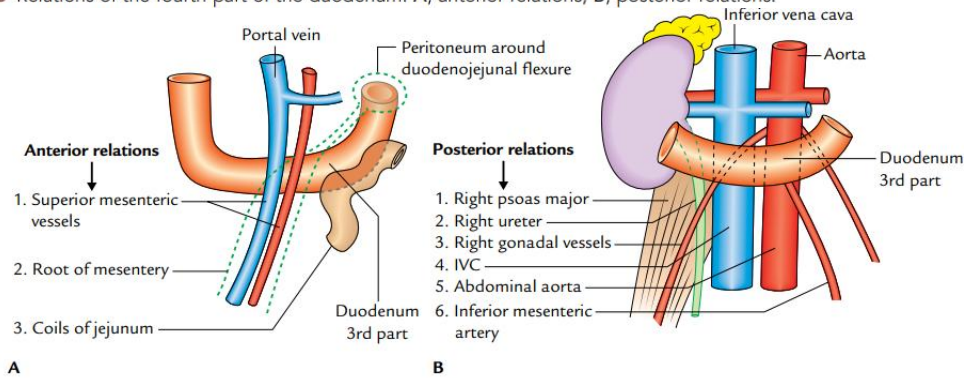


Fig. 9.4 Relations of the second part of the duodenum: A, anterior relations; B, posterior relations.



6 Relations of the fourth part of the duodenum: A, anterior relations; B, posterior relations.



1.5 Relations of the third part of the duodenum: A, anterior relations; B, posterior relations.

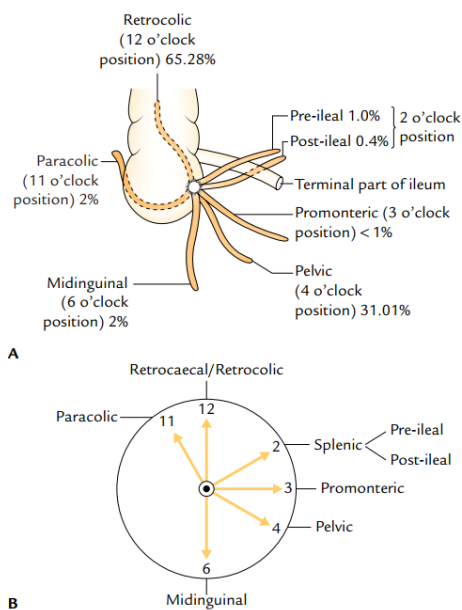


Fig. 10.15 Positions of vermiform appendix (after Treves): A, actual positions; B, positions according to the needle of clock.

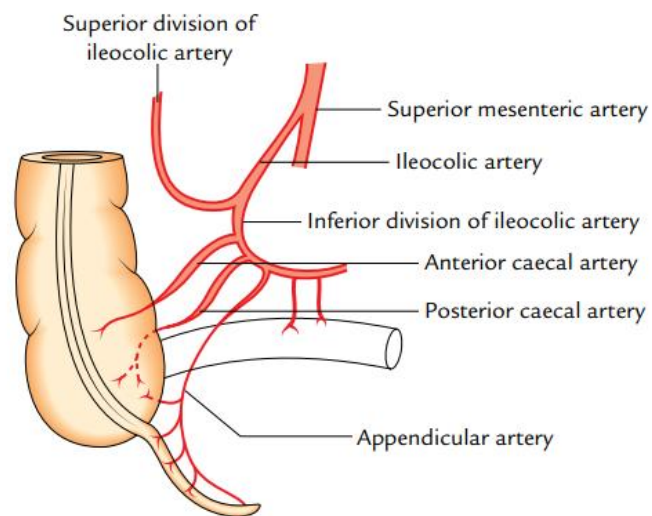
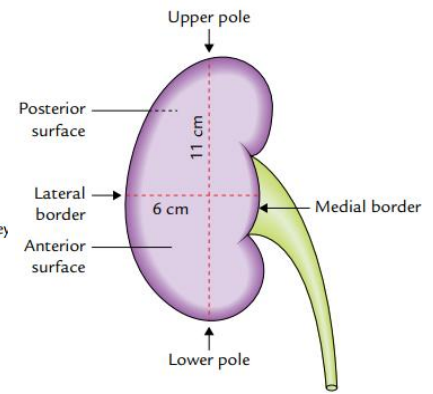
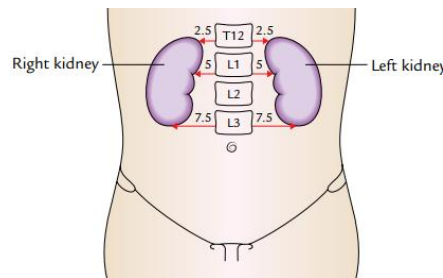
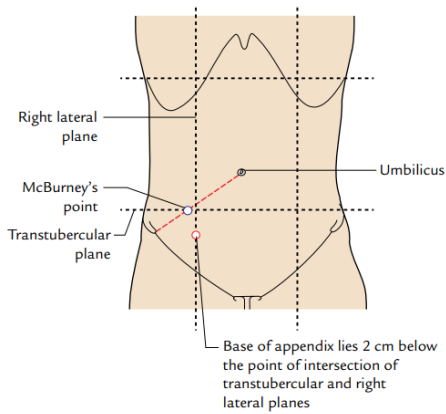
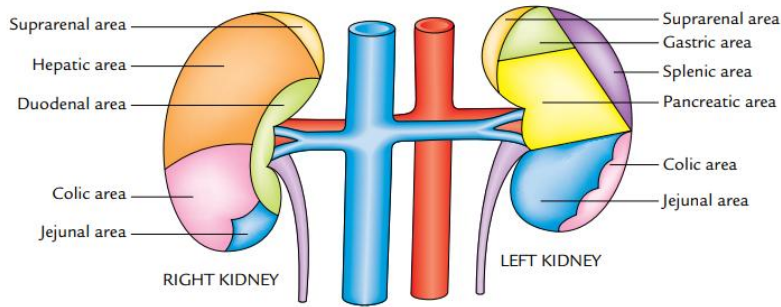


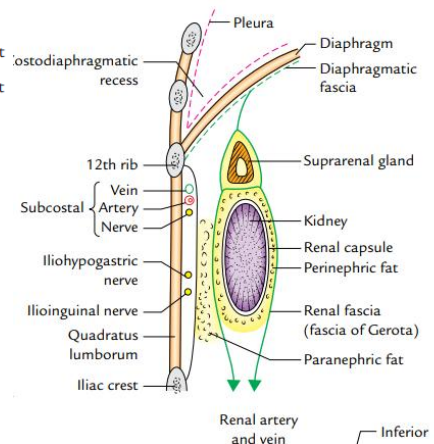
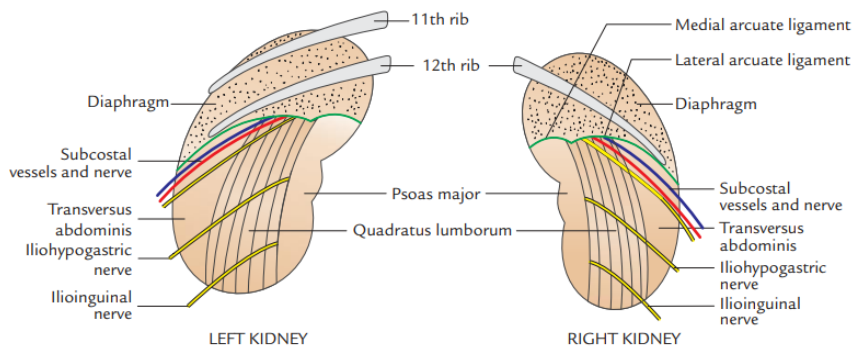
Fig. 10.13 Arterial supply of the caecum and appendix.



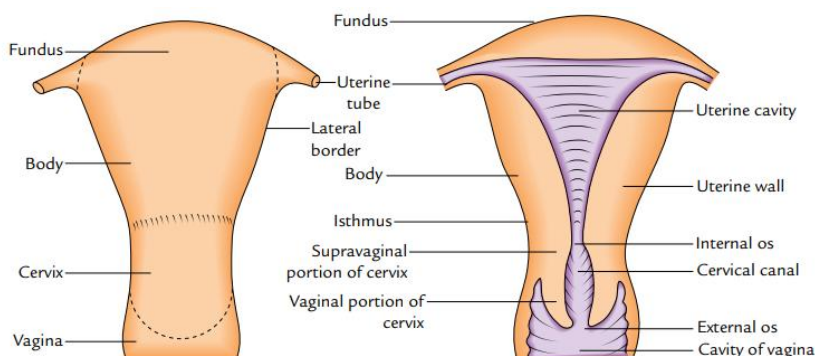
11.3 External features and measurements c



Anterior relations of the kidneys.



Posterior relations of the kidneys.



Subdivisions of the uterus.

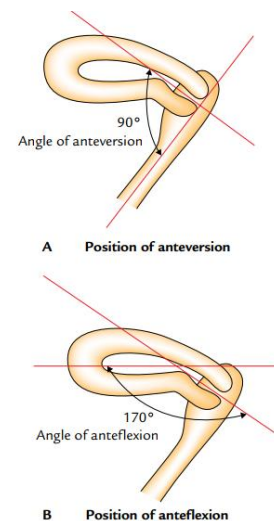
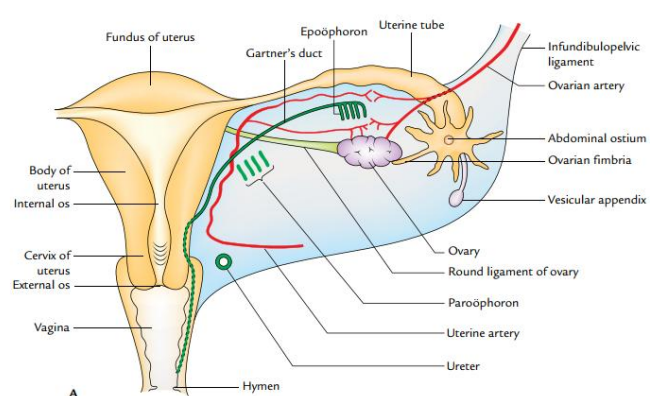
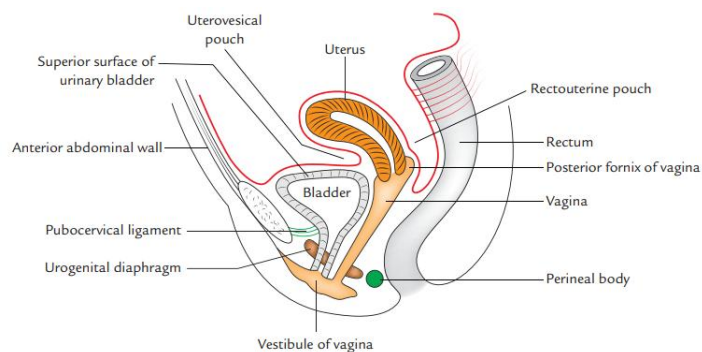


Fig. 18.11 Position of the uterus: A, angle of anteversion (angle between long axis of the vagina and long axis of the cervix); B, angle of anteversion (angle between long axis of the cervix and long axis of the body of uterus).



Relations of the uterus.

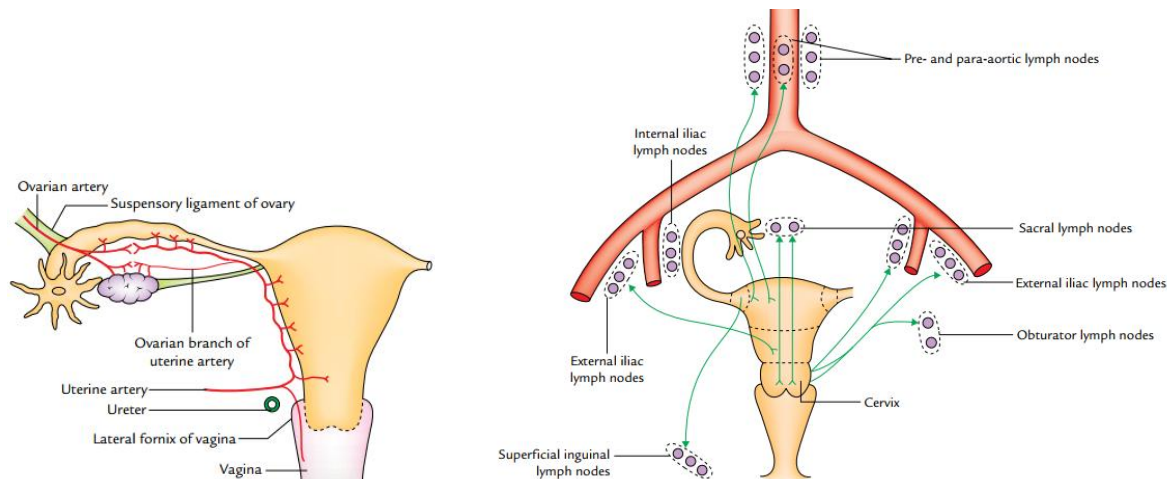


Fig. 18.15 Arterial supply of the uterus.

Fig. 18.17 Lymphatic drainage of the uterus.

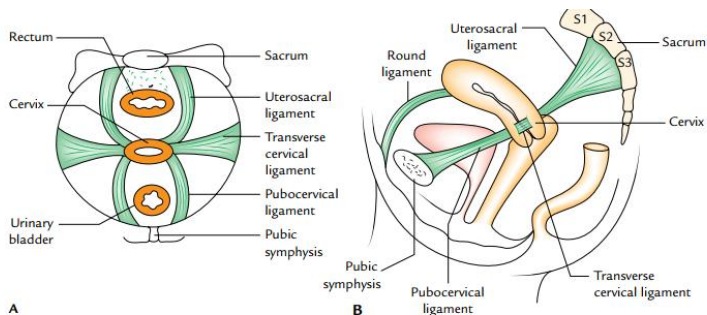


Fig. 18.18 Ligamentous (fibromuscular) supports of the uterus: A, as seen from above; B, lateral view. Note all these ligaments except the round ligaments are formed by the condensation of visceral layer of pelvic fascia.

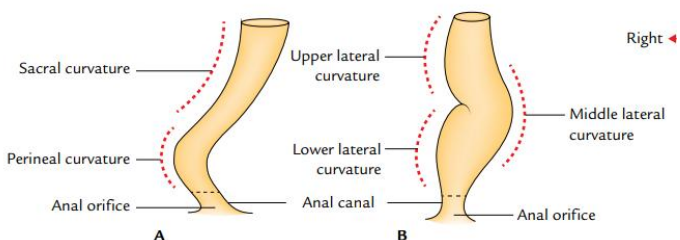


Fig. 19.2 Curvatures (flexures) of the rectum: A, anteroposterior curvatures; B, lateral curvatures.

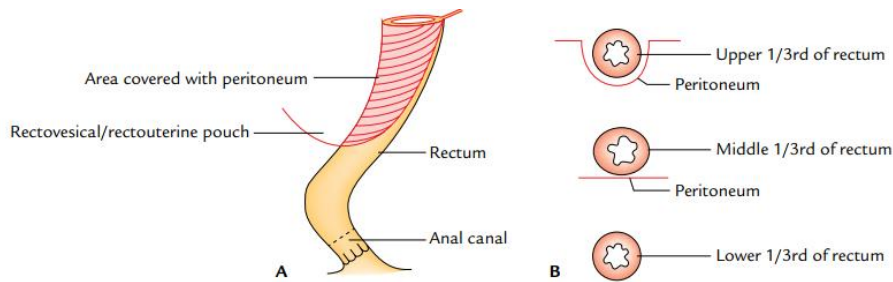
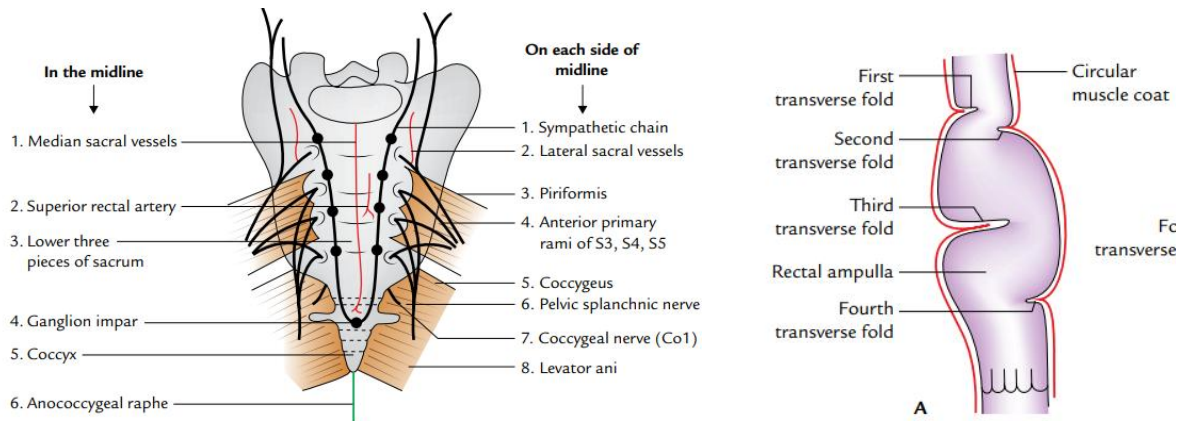


Fig. 19.4 Peritoneal relations of the rectum: A, in sagittal section; B, in cross sections of the levels of upper 1/3rd, middle 1/3rd, and lower 1/3rd.



Posterior relations of the rectum.

Transverse folds within the lumen of the rectum: A

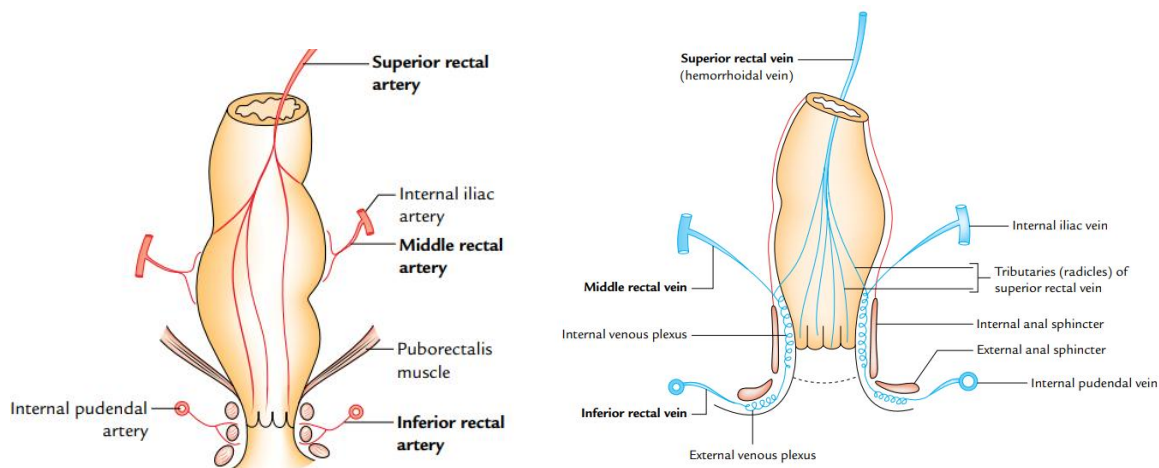
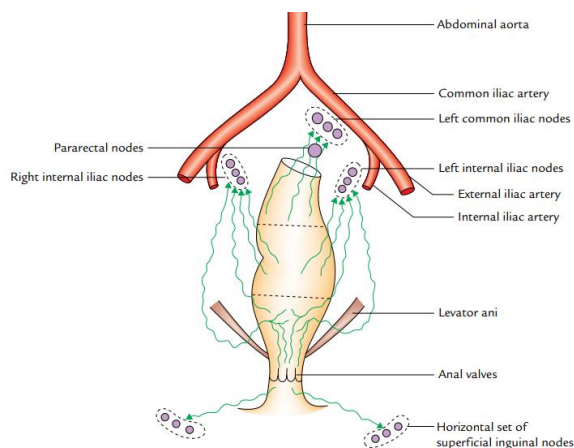


Fig. 19.8 Arterial supply of the rectum and anal canal.

Venous drainage of the rectum and anal canal.



Lymphatic drainage of the rectum and anal canal.

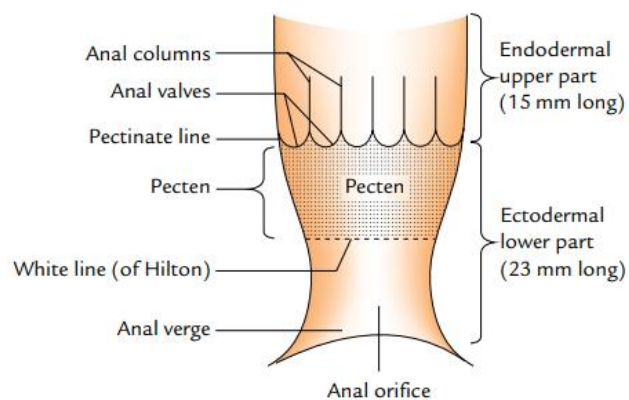
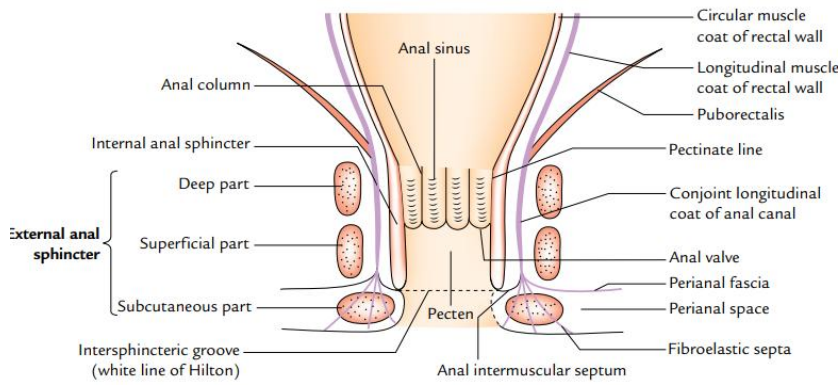


Fig. 19.11 Interior of the anal canal.



A coronal section of anal canal showing internal and external anal sphincters.

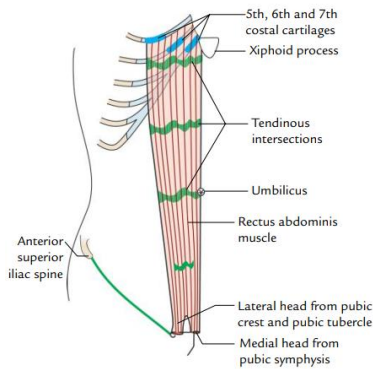


Fig. 3.21 Origin and insertion of rectus abdominis (right side).

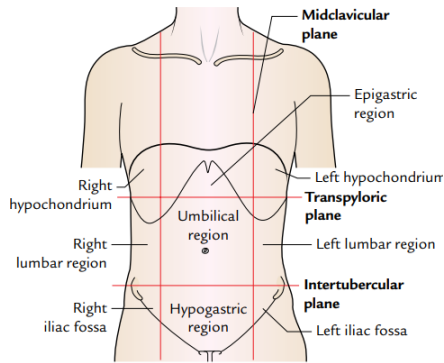


Fig. 6.1 Nine regions of the abdomen.

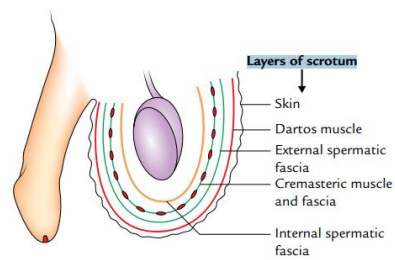


Fig. 5.10 Layers of the scrotum.

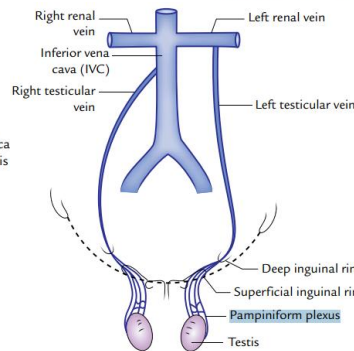
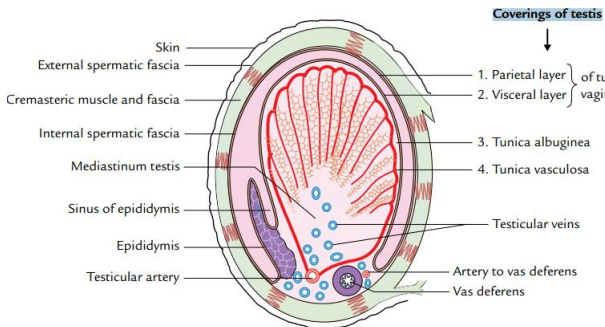
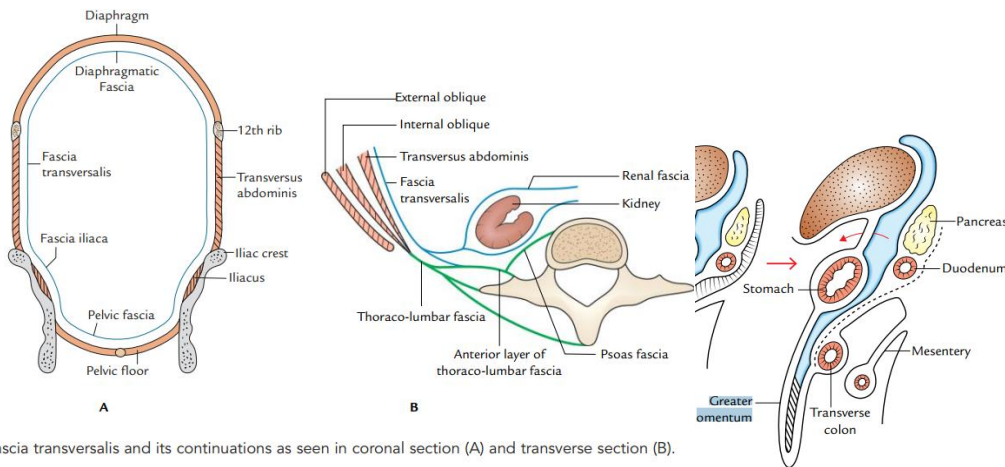


Fig. 5.17 Venous drainage of the testis.



Fascia transversalis and its continuations as seen in coronal section (A) and transverse section (B).

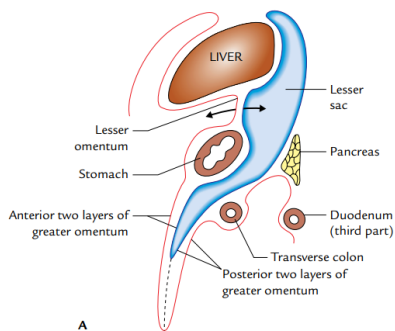
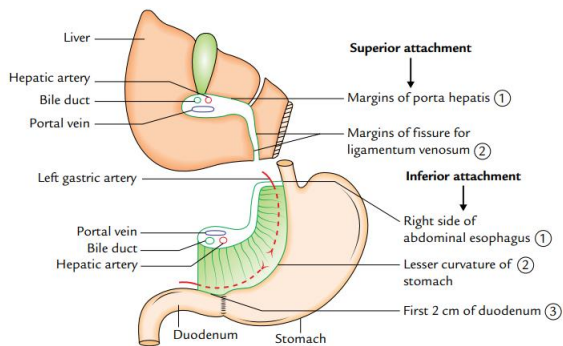


Fig. 6.24 Lesser sac: A, as seen in the sagittal section; B, as A = aorta).

Attachments and contents of the lesser omentum.

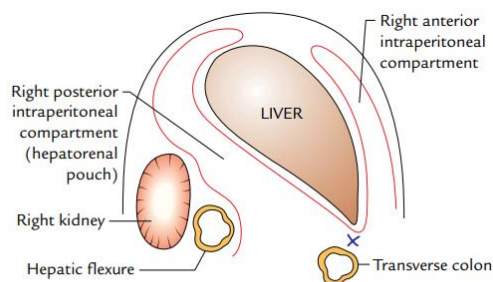


Fig. 6.23 Hepatorenal pouch as seen in the sagittal section.

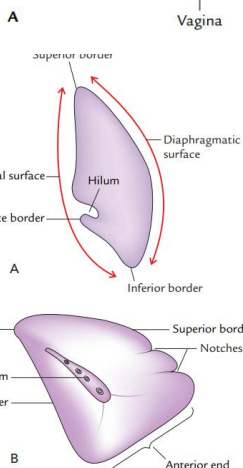
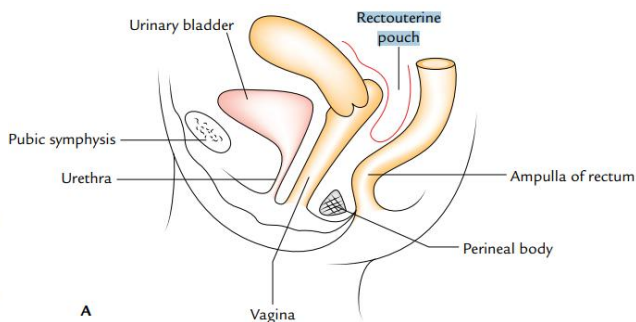


Fig. 7.18 A, External features of the spleen: A, as seen in the transverse section; B, as seen from the visceral surface. Note the characteristic notches on the superior border of the spleen.

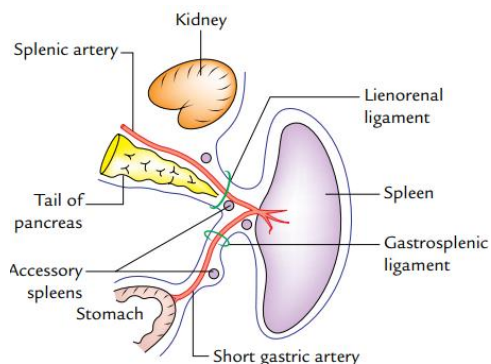


Fig. 7.19 Peritoneal relations of the spleen.

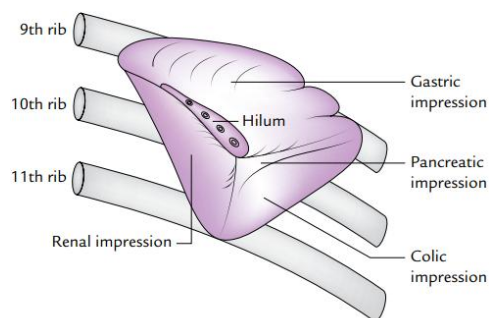
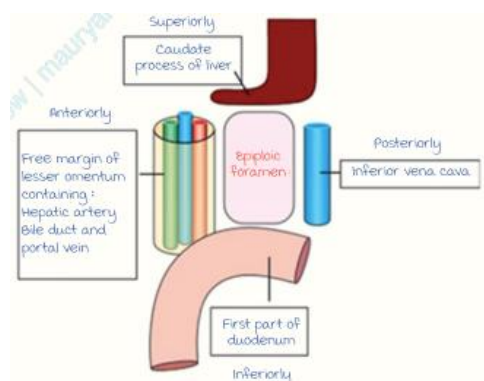
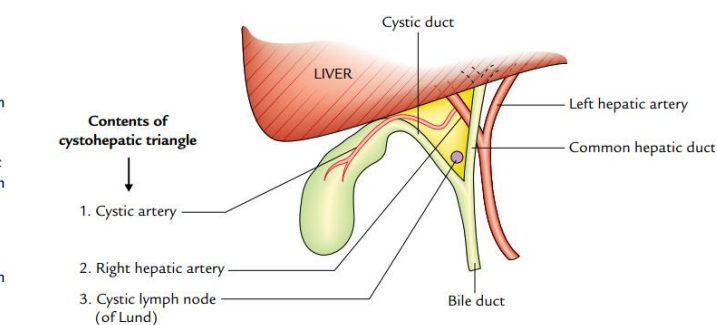


Fig. 7.20 Visceral surface of the spleen showing different impressions.



Boundaries and contents of the cystohepatic triangle (yellow-coloured area).

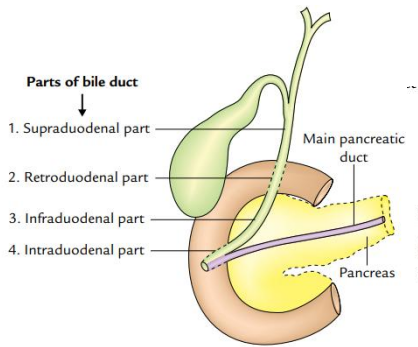


Fig. 8.18 Parts of the bile duct (dark green colour).

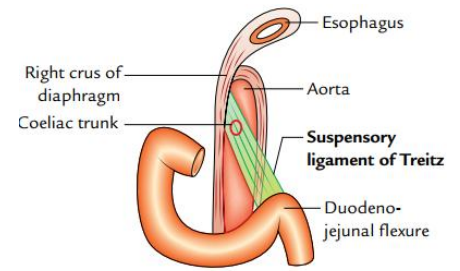
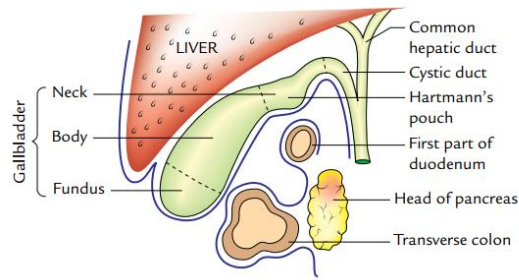


Fig. 8.19 Suspensory ligament of Treitz.

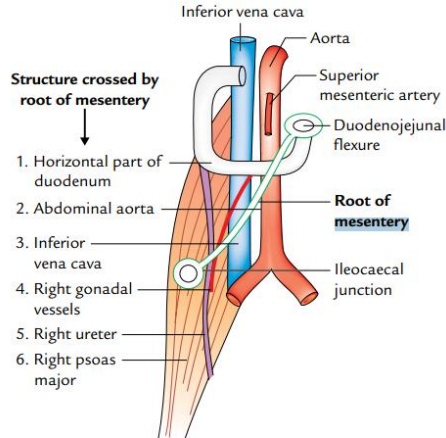


Fig. 6.13 Structures crossed by the root of the mesentery

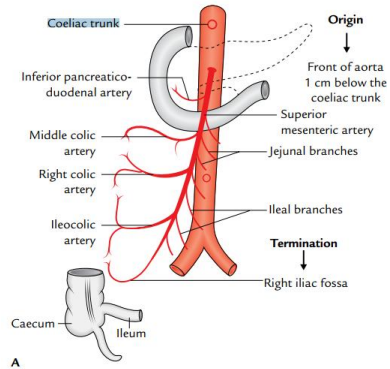
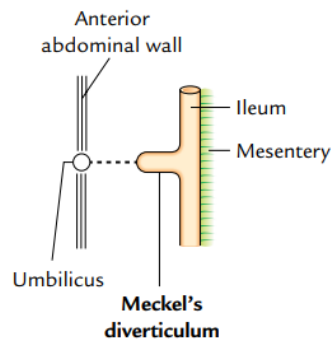
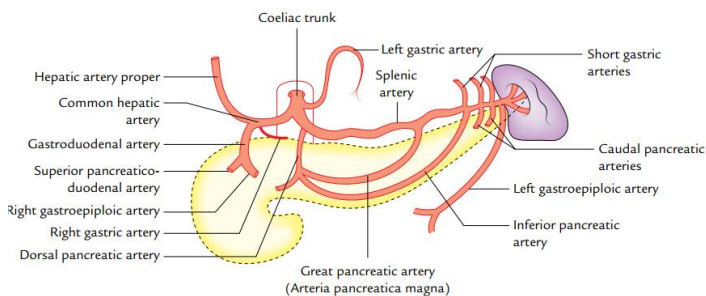


Fig. 10.21 Superior mesenteric artery: A, course and branches;



Coeliac trunk.

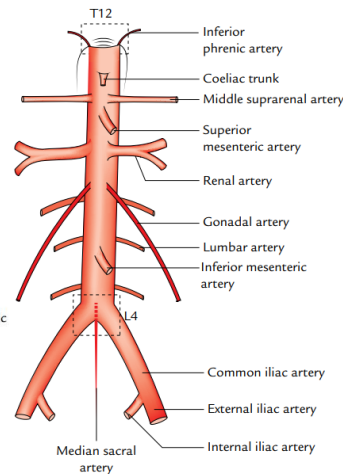


Fig. 12.6 Course and branches of the abdominal aorta.

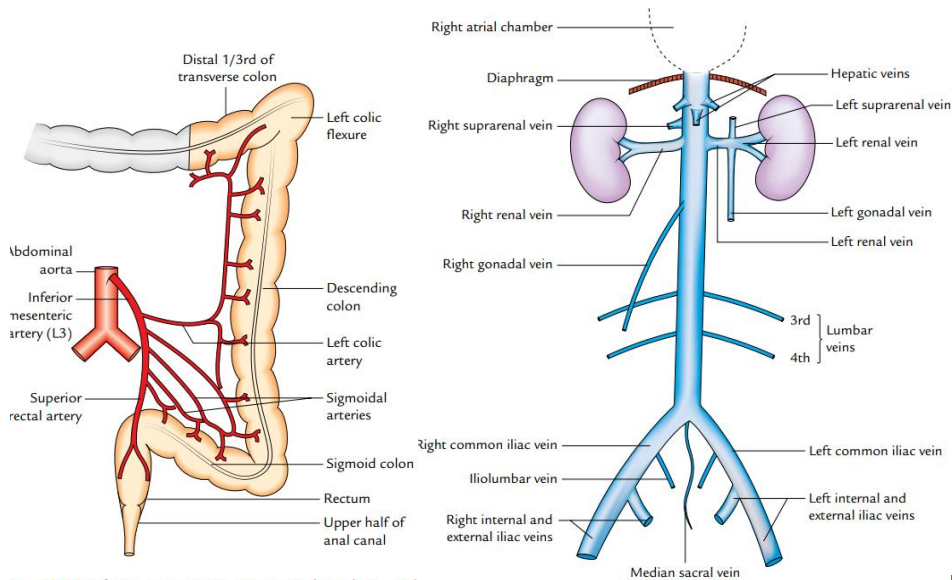


Fig. 10.23 Inferior mesenteric artery, its branches, and distribution.

tributaries of the inferior vena cava.

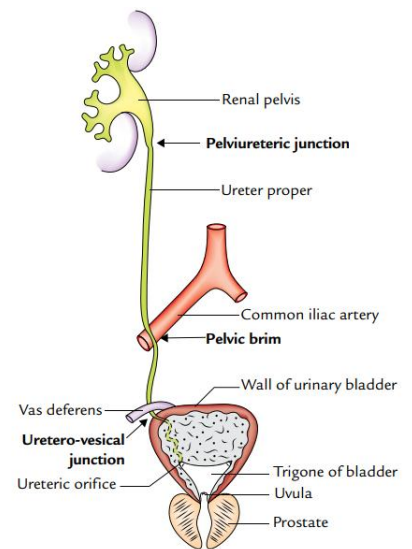


Fig. 11.25 Normal sites of anatomical constrictions in ureter (arrows).

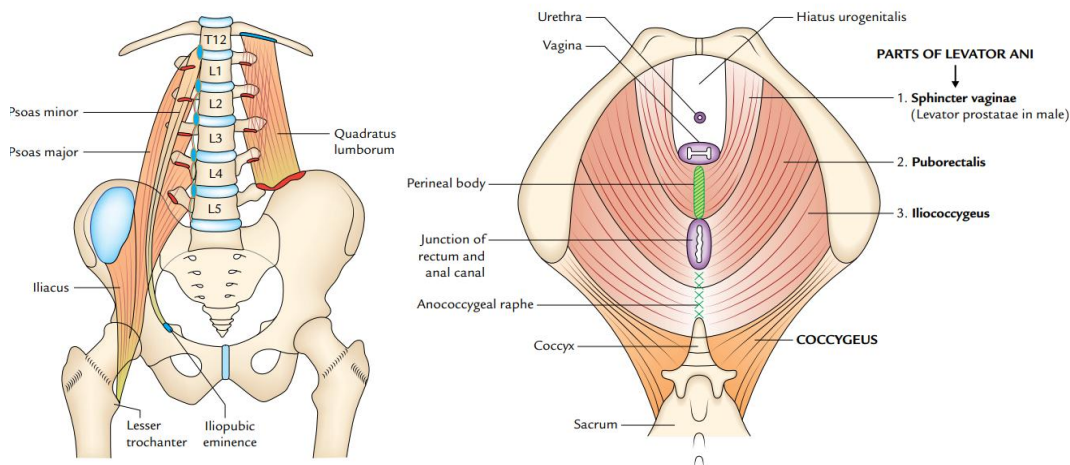


Fig. 12.2 Origin and insertion of the psoas major, psoas minor, iliacus, and quadratus lumborum muscles.

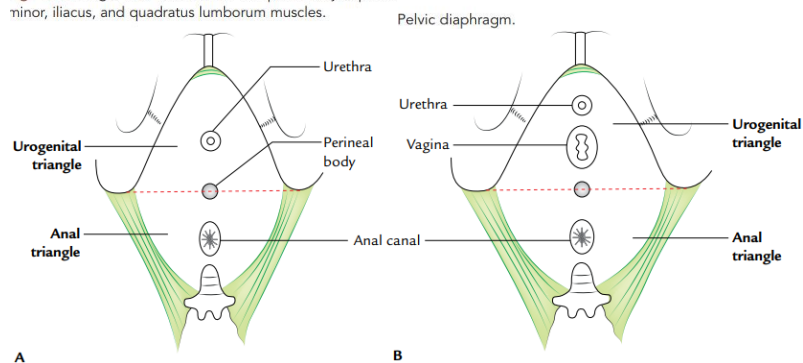


Fig. 12.4 Subdivision of the perineum: A, male perineum; B, female perineum.

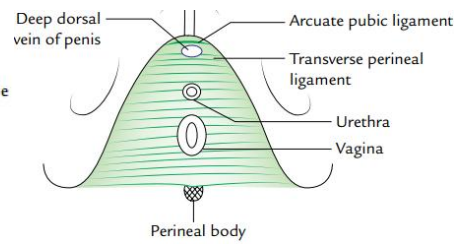
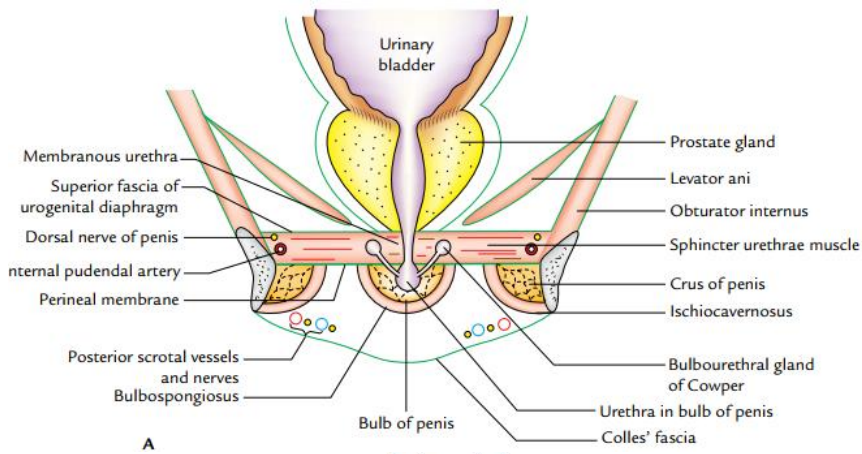


Fig. 15.6 Perineal membrane.

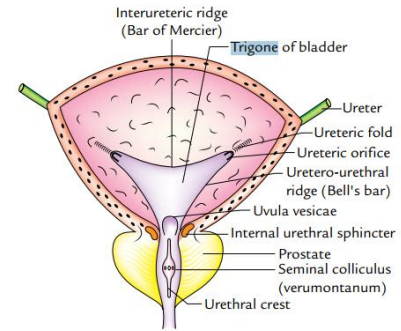
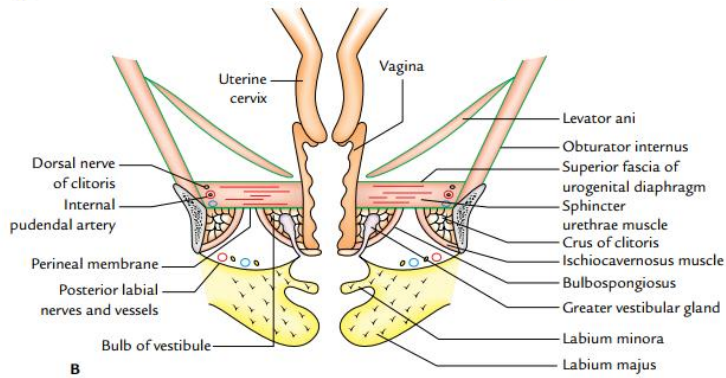
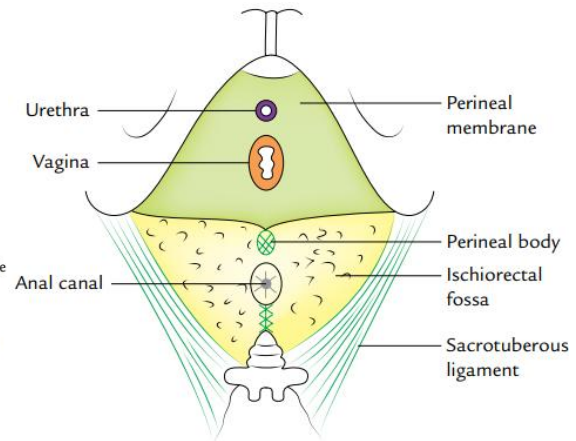
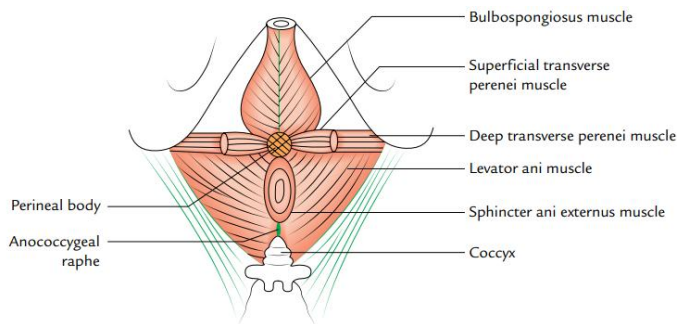


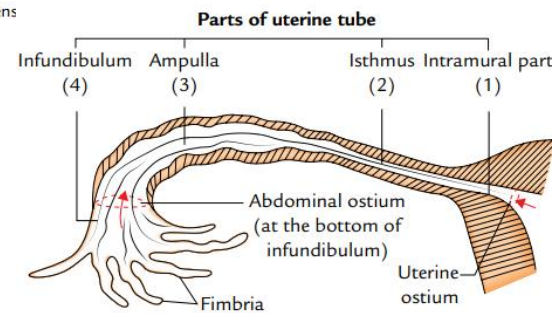
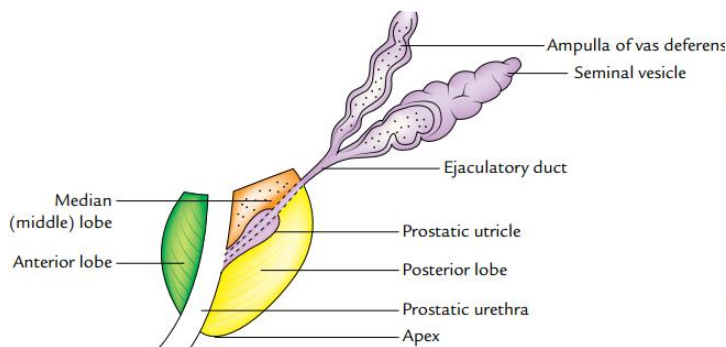
Fig. 15.8 Coronal section of the urogenital region showing contents of the superficial and deep perineal pouches: A, male; B, female.

Fig. 16.8 Interior of the urinary bladder as seen in the coronal section.



Perineal body. Note various muscles attached to it.

Fig. 15.17 Surface view of the ischioanal fossa and perineal body.



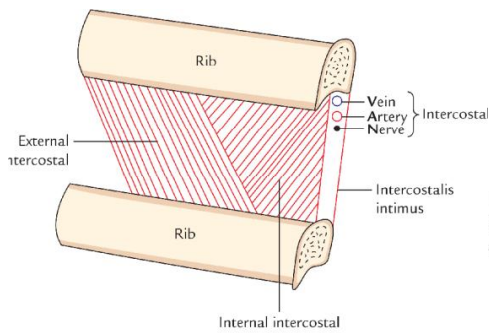


Fig. 16.3 Contents of the intercostal space as seen in dissection.

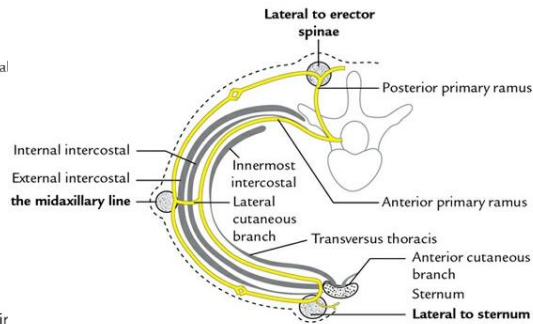
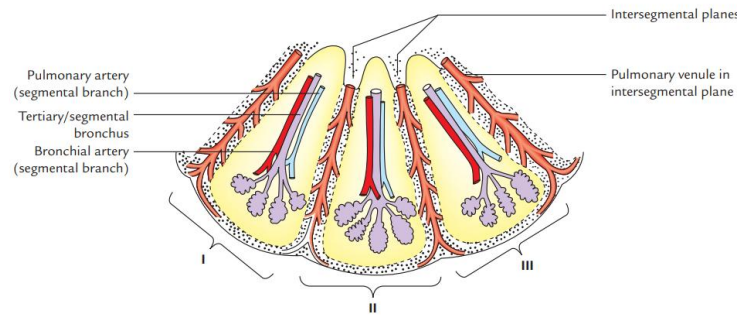
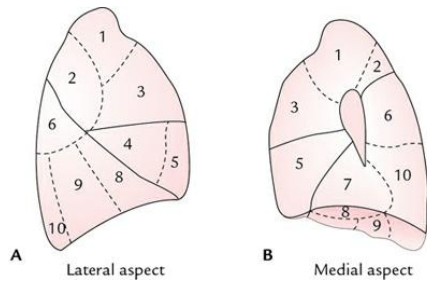
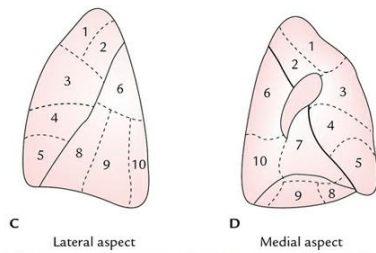


FIG. 1.4 Typical intercostal nerve.



Schematic diagram showing three bronchopulmonary segments (I, II, and III).



A and B, Bronchopulmonary segments of the right lung; C and D, bronchopulmonary segments of the left lung. (Note: Medial basal segment is absent in the left lung.)

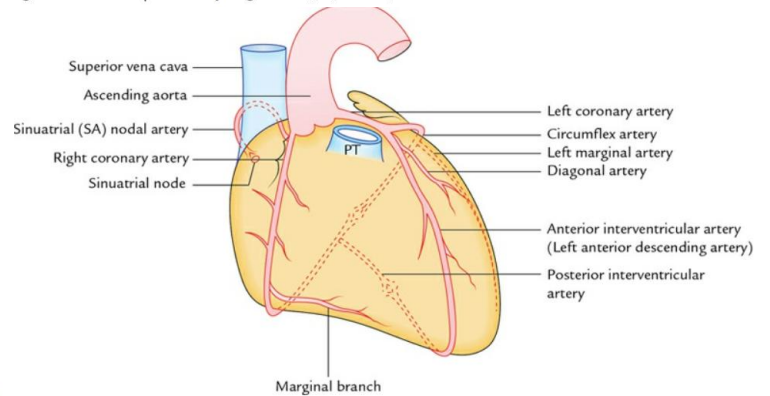


FIG. 4.4 Arterial supply of the heart (PT = pulmonary trunk).

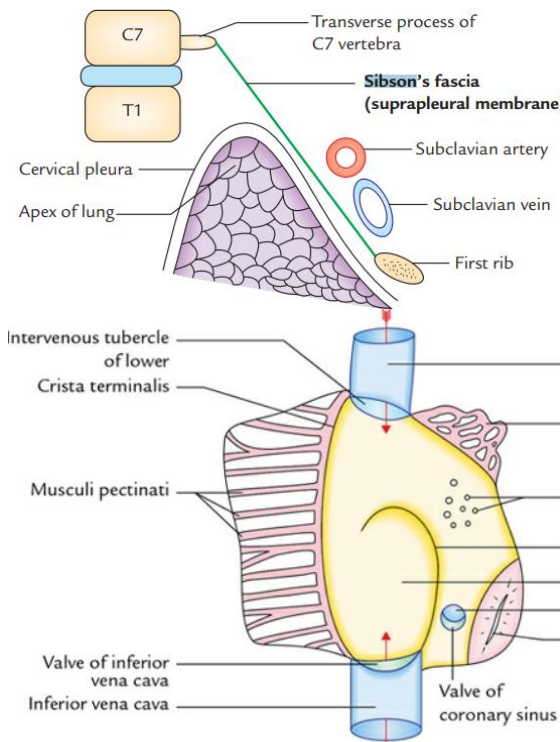


FIG. 4.3 Interior of the right atrium.

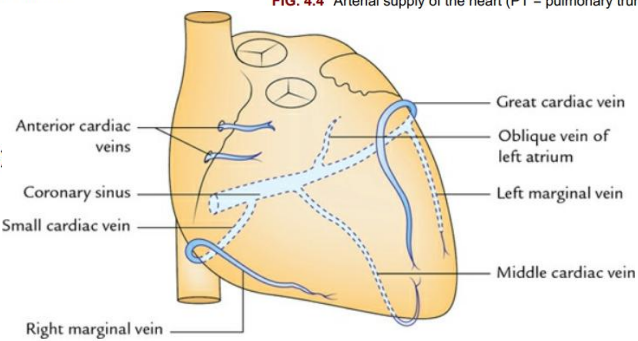


FIG. 4.5 Venous drainage of the heart.

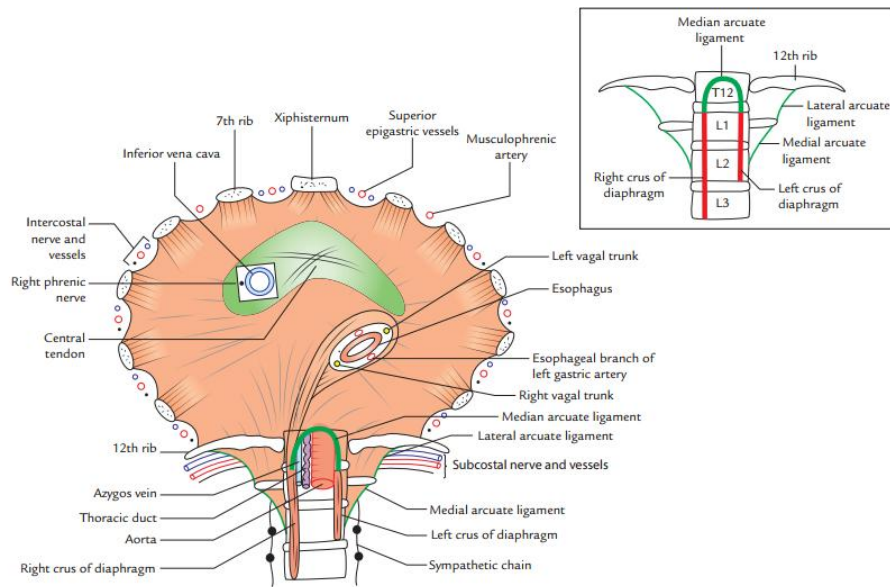
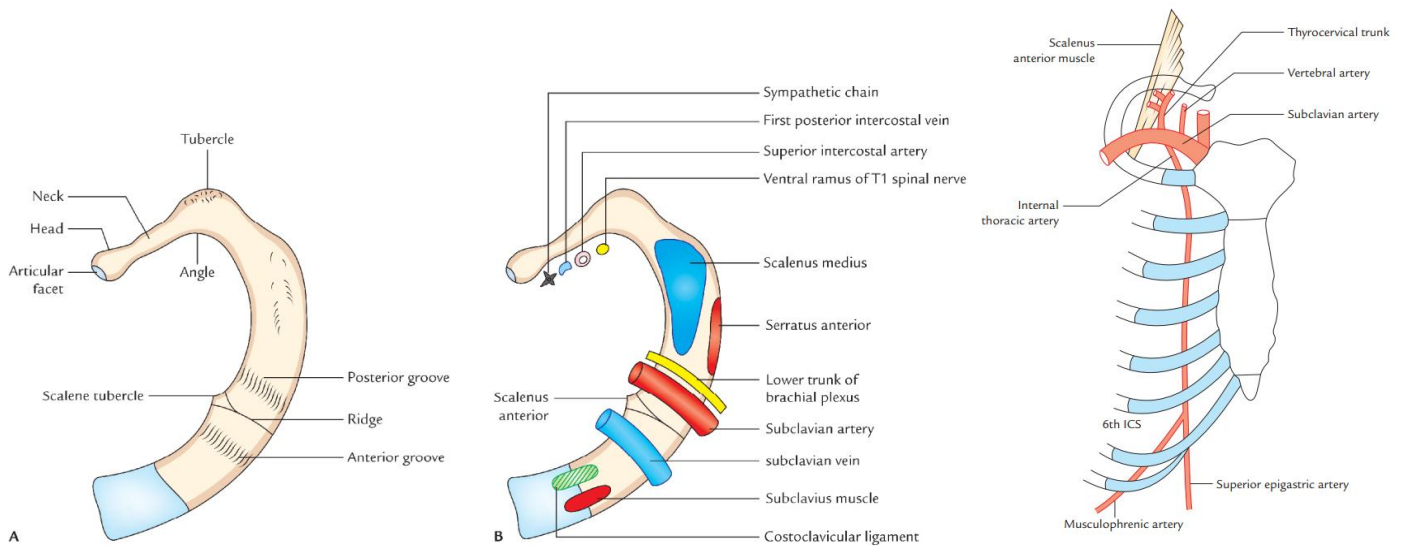
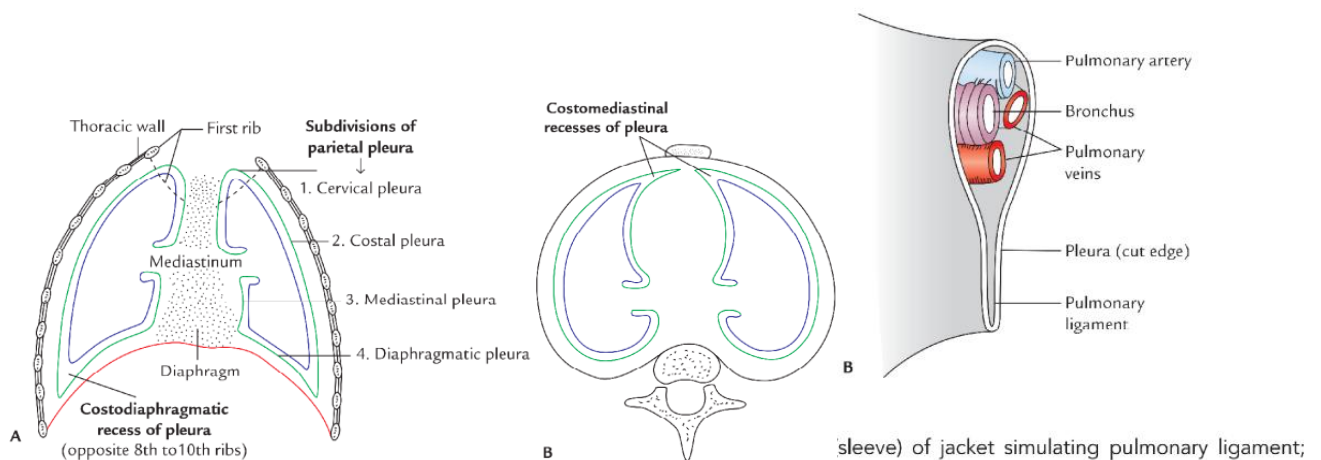


Fig. 14.9 Origin, insertion, and openings of the diaphragm. Figure in the inset shows details of vertebral origin of the diaphragm.



g. 15.5 First rib (left side) seen from above: A, general features; B, attachments and relations.

Fig. 16.9 Origin, course, and termination of internal thoracic artery (ICS = intercostal space).



sleeve) of jacket simulating pulmonary ligament;

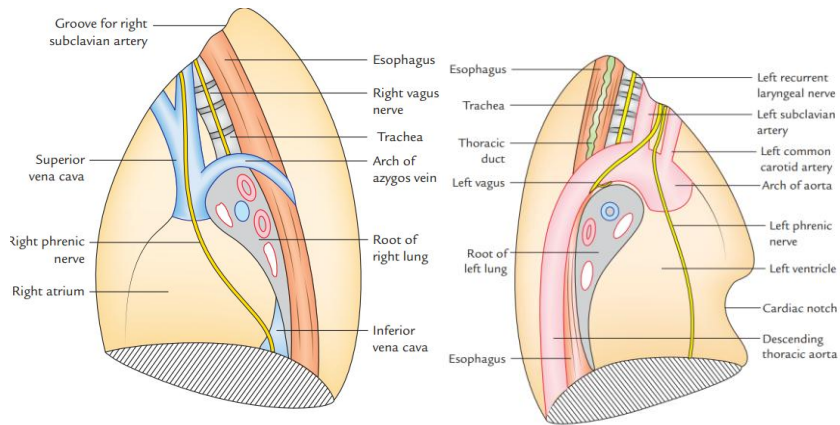


Fig. 18.3 Structures related to the mediastinal surface of the right lung.

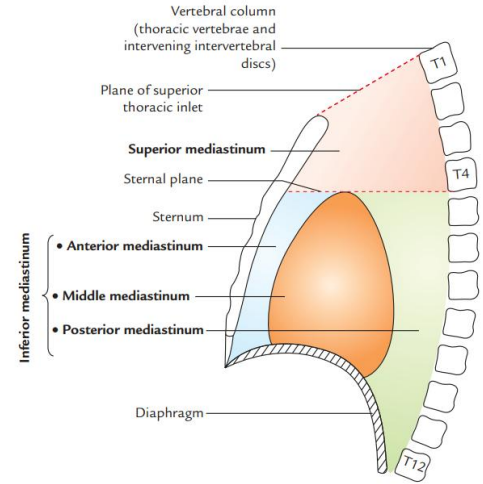


Fig. 18.4 Structures related to the mediastinal surface of the left lung and producing impression on this surface.

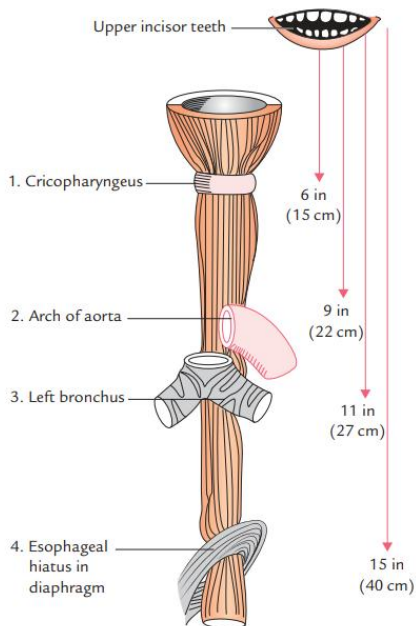
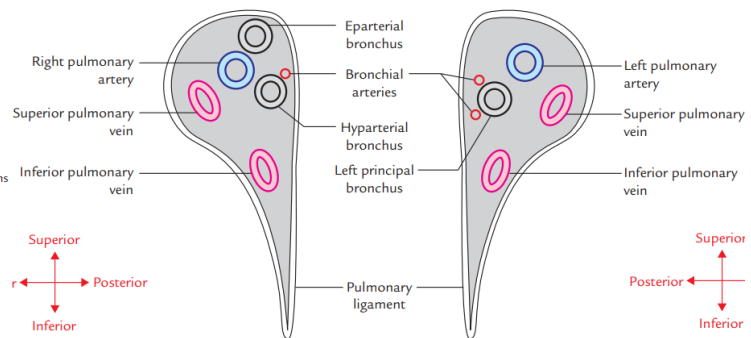
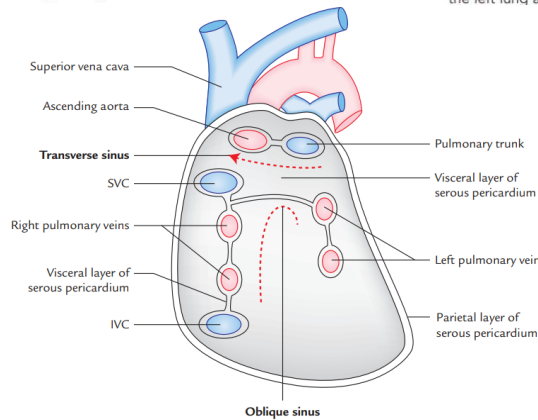
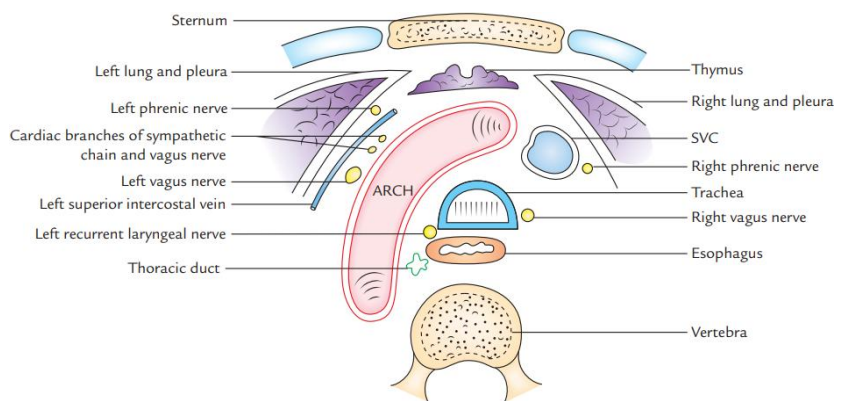


Fig. 22.6 Anatomical sites of constrictions in esophagus. On the right side distances of these sites from the upper incisor teeth are given.



cross section of superior mediastinum showing relations of arch of aorta (SVC = superior vena cava)

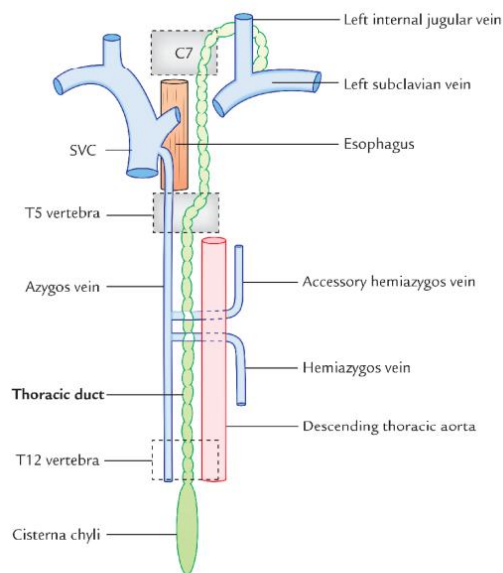


Fig. 23.2 Formation, course, and termination of the thoracic duct.

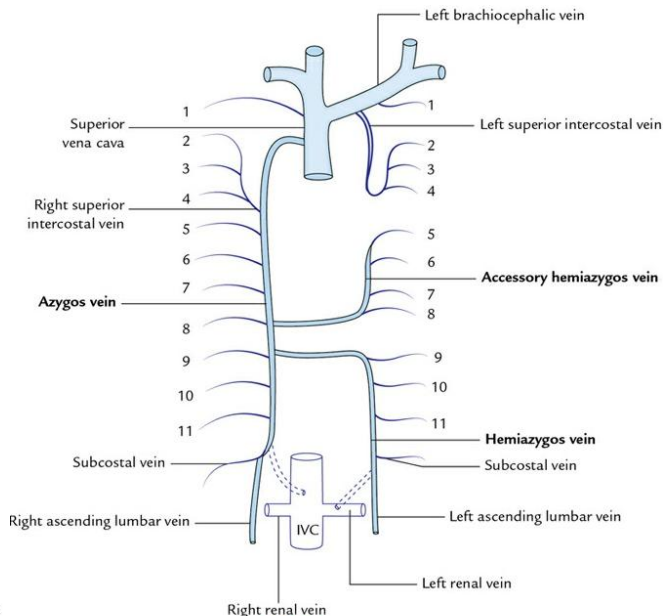


FIG. 6.8 Azygos vein.

LOWER LIMB

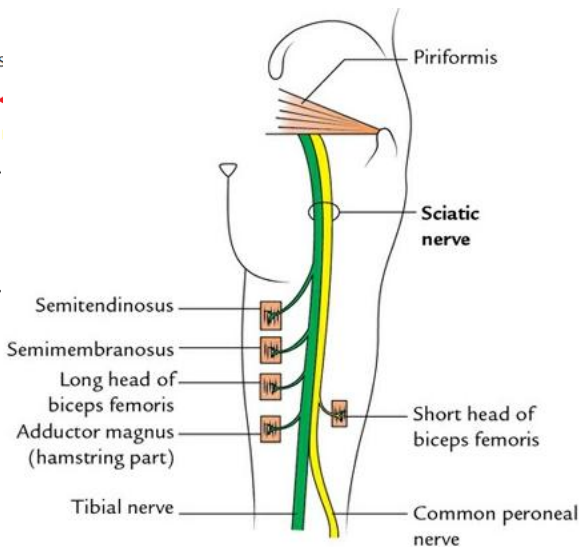
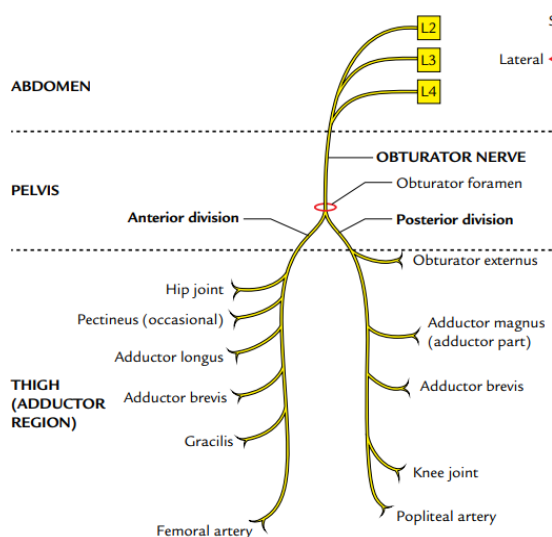
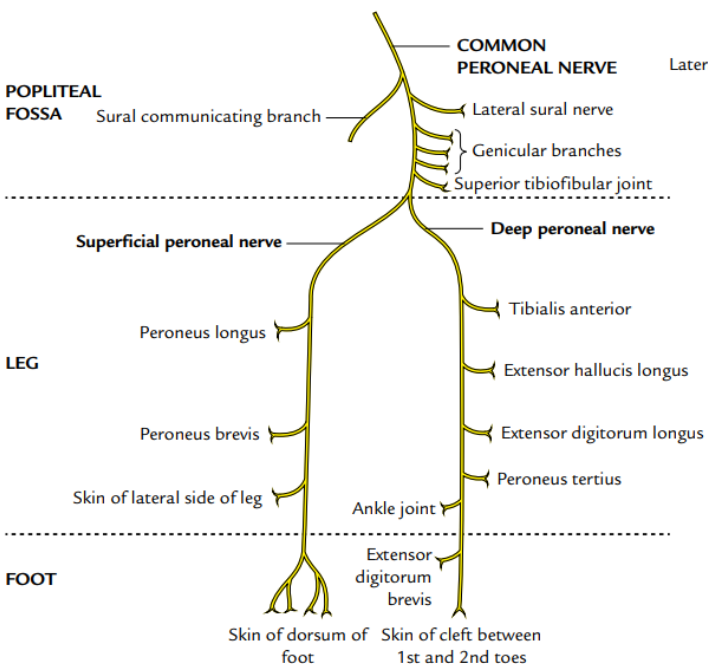
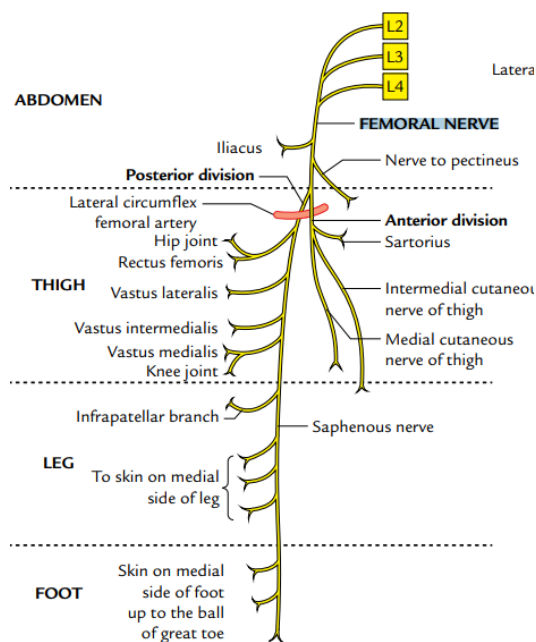


FIG. 21.4 Branches of sciatic nerve.



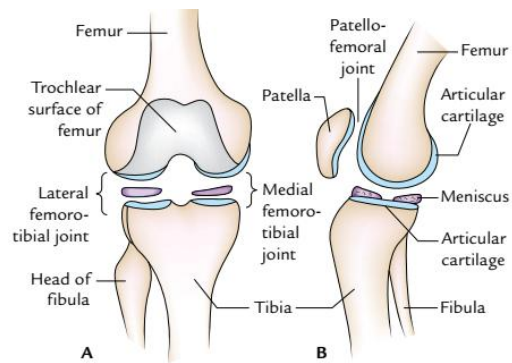


Fig. 32.1 Three primitive joints included in the knee joint: A, condylar type of medial and lateral femorotibial joints; B, saddle type of patellofemoral joint.

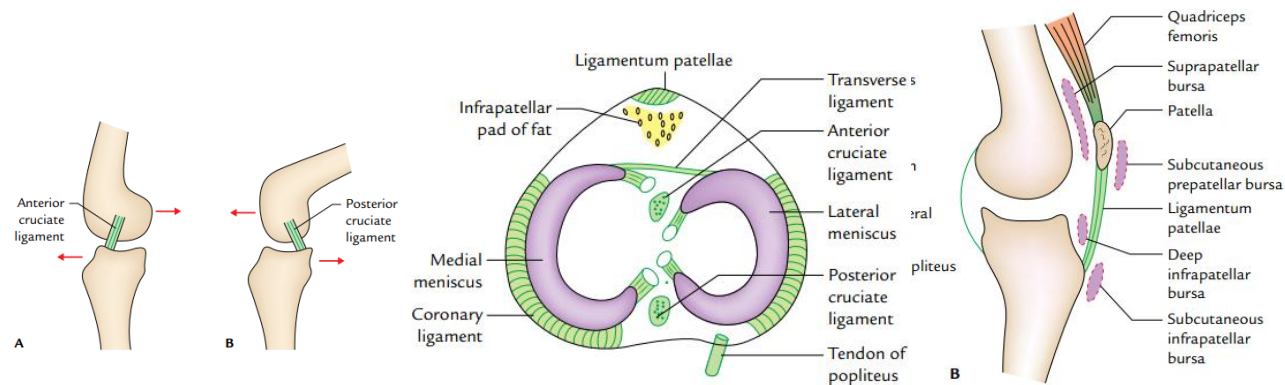
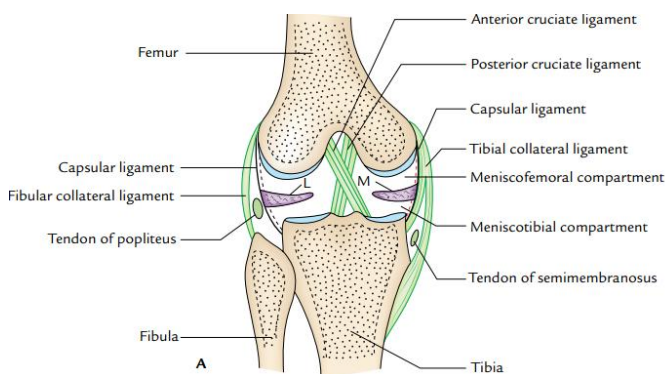


Fig. 32.3 Medial and lateral menisci of the knee joint. A, medial meniscus; B, lateral meniscus.

Fig. 32.4 Medial and lateral menisci of the knee joint.

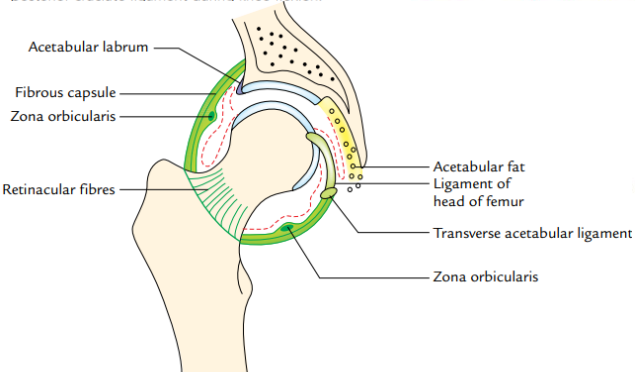


Fig. 32.5 Medial and lateral menisci of the knee joint. A, medial meniscus; B, lateral meniscus.

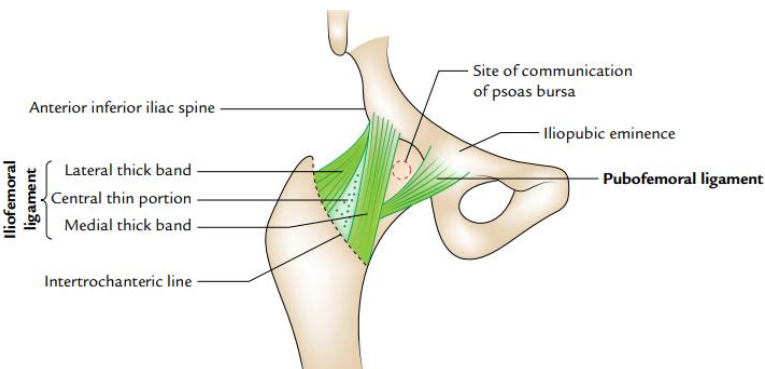
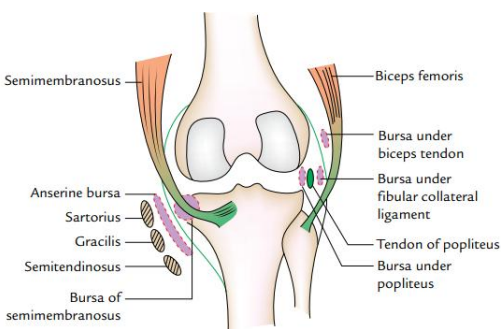


Fig. 32.6 Medial and lateral menisci of the knee joint. A, medial meniscus; B, lateral meniscus.

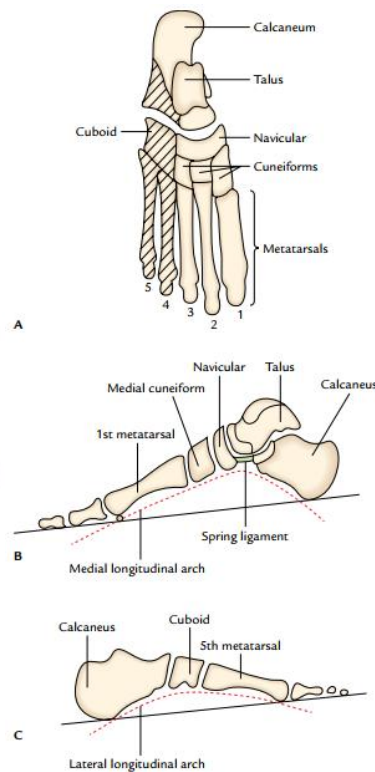
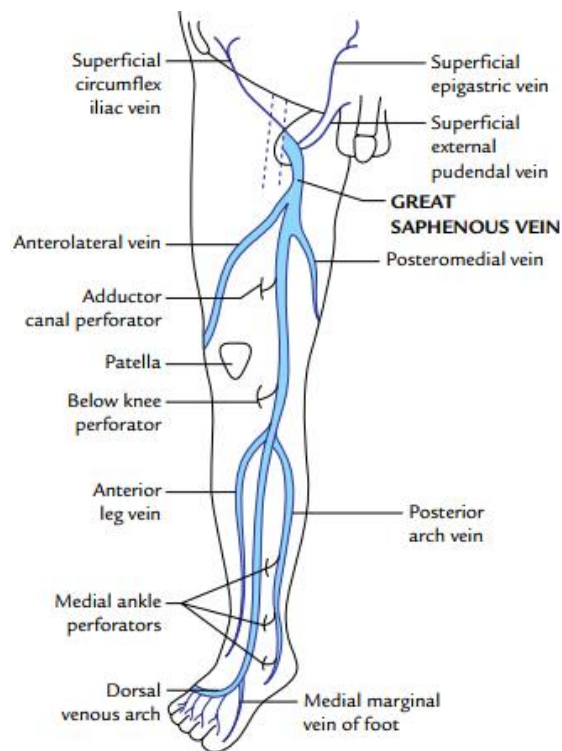
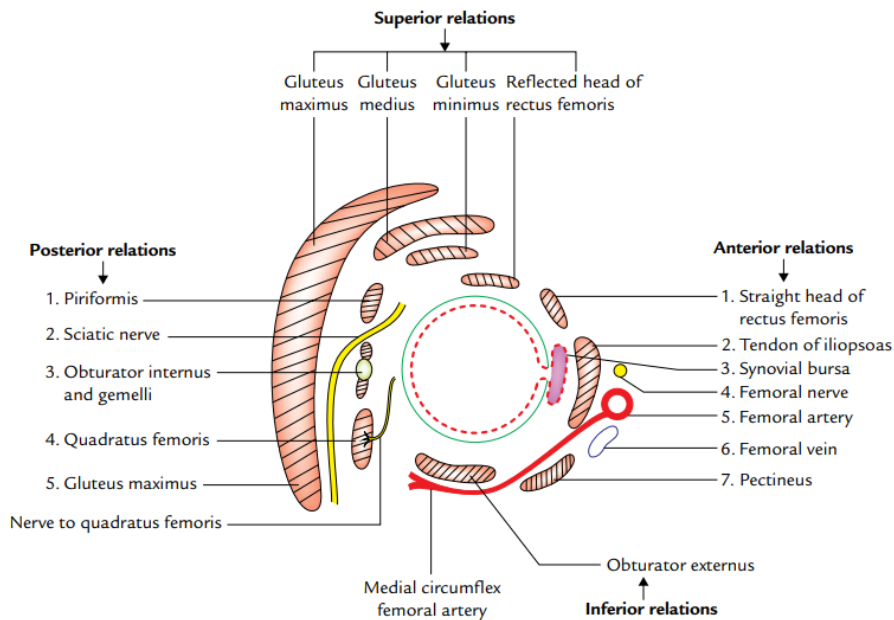


Fig. 31.5 Formation of the longitudinal arches of the foot: A, bones forming medial longitudinal arch are unhatched and bones forming lateral longitudinal arch are hatched. Note calcaneum is common to both but primarily it forms the lateral longitudinal arch; B, medial longitudinal arch—side view; C, lateral longitudinal arch—side view. Note the medial longitudinal arch is higher than the lateral arch. It is more pliant due to the presence of talocalcaneonavicular and subtalar joints. It is more involved in the propulsion, i.e., initiating the next step during walking.

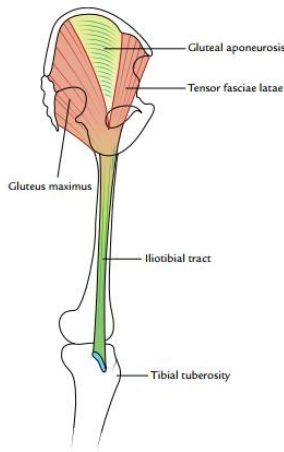
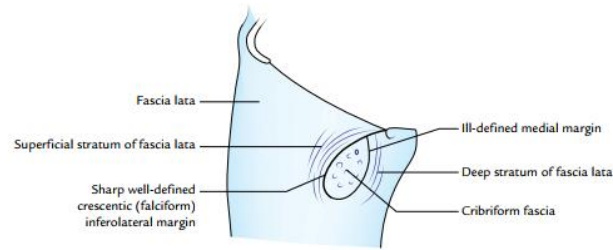


Fig. 22.5 Attachment of the iliotibial tract. Note superficial portion of gluteus maximus and tensor fasciae latae insert into the iliotibial tract from opposite directions.



The saphenous opening. Note the saphenous opening resembles a double-breasted jacket.

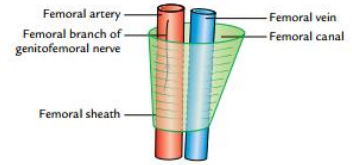


Fig. 22.14 Femoral sheath. Note the asymmetry of the femoral sheath.

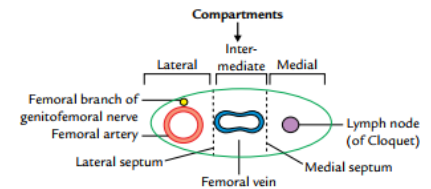
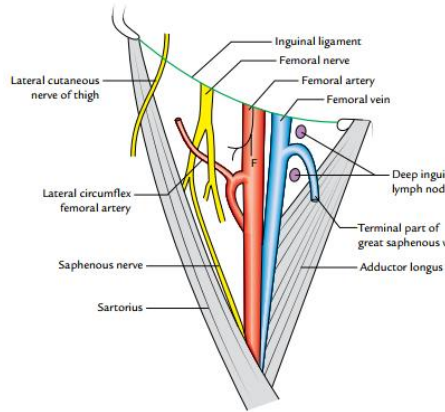
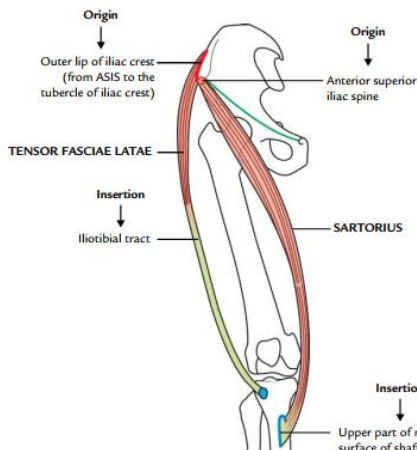
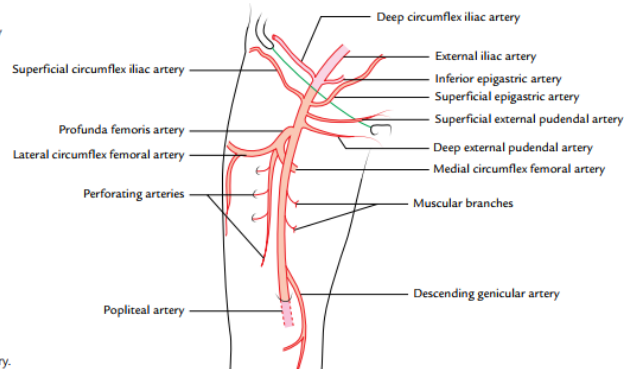
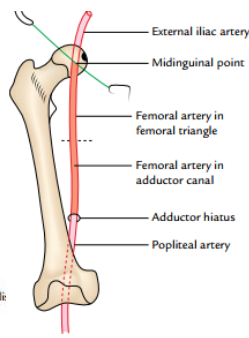
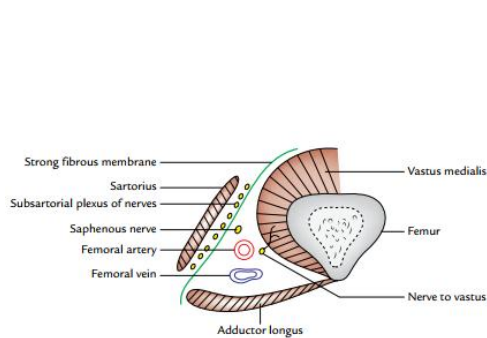
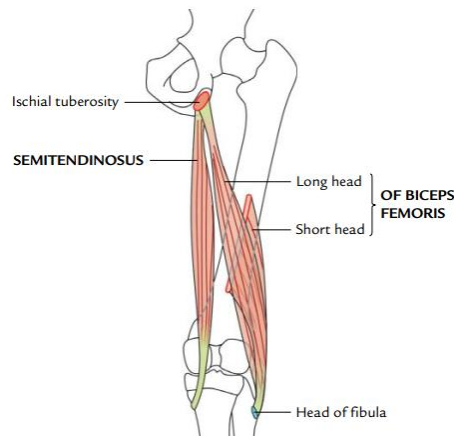
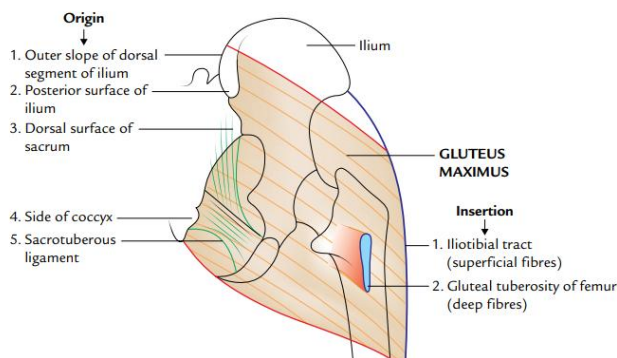
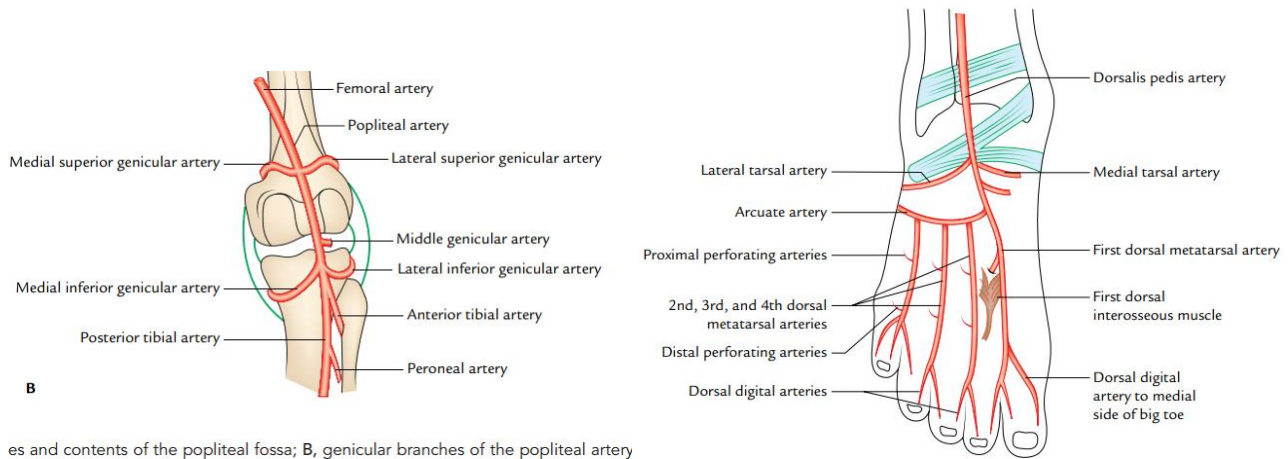
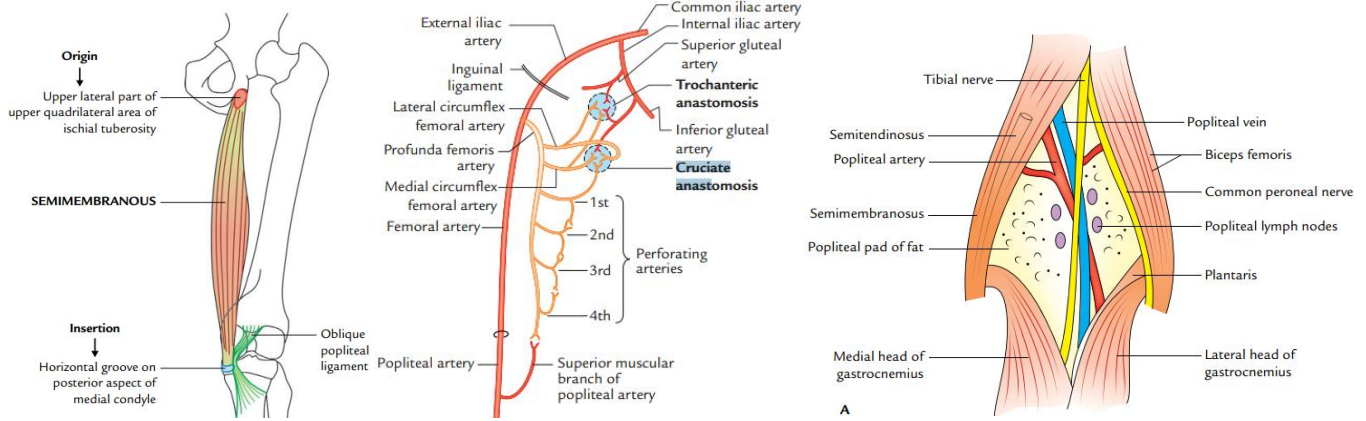


Fig. 22.15 Compartments of the femoral sheath.

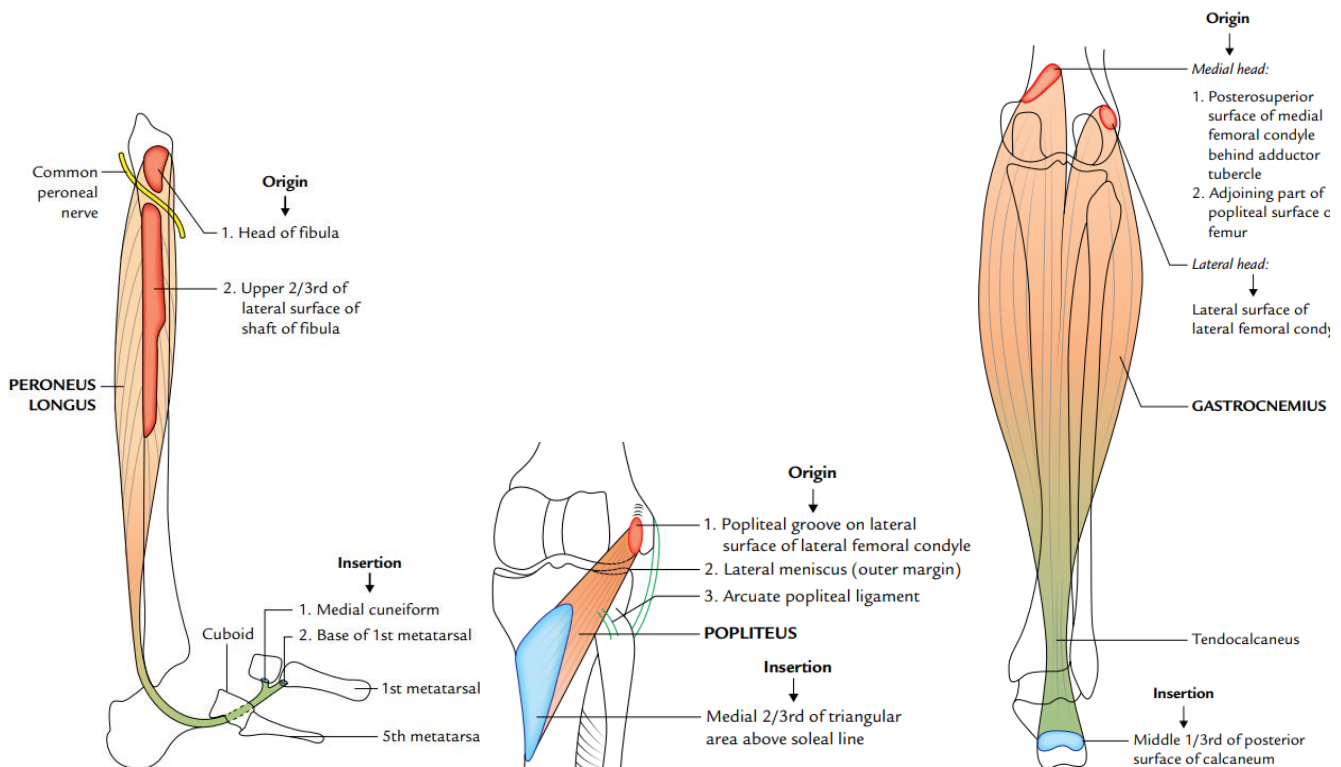


Boundaries and contents of the adductor canal as seen in the transverse section through mid-thigh. Course and extent of the femoral artery.





es and contents of the popliteal fossa; B, genicular branches of the popliteal artery



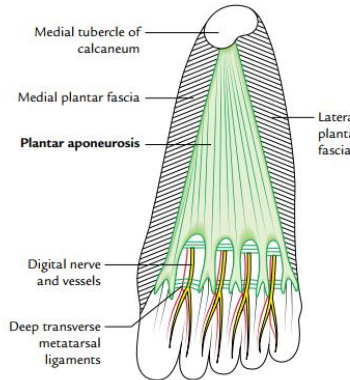
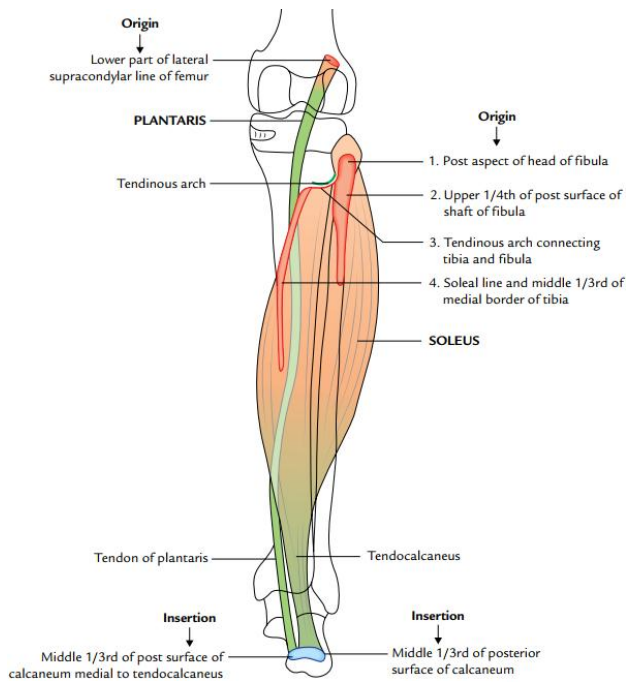


Fig. 30.2 Plantar aponeurosis.

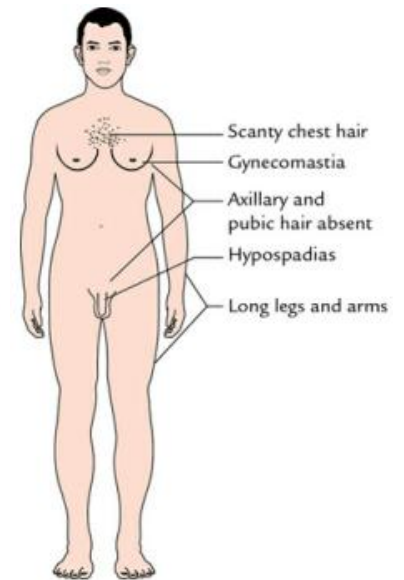


FIG. 25.6 Klinefelter's syndrome.

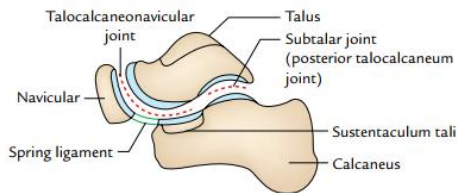


Fig. 32.19 Subtalar and talocalcaneonavicular joints.

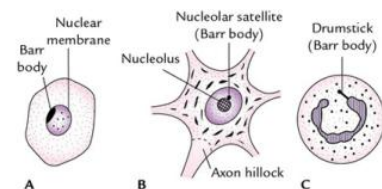


FIG. 25.1 Barr body: A, buccal smear; B, neuron; C, neutrophils.

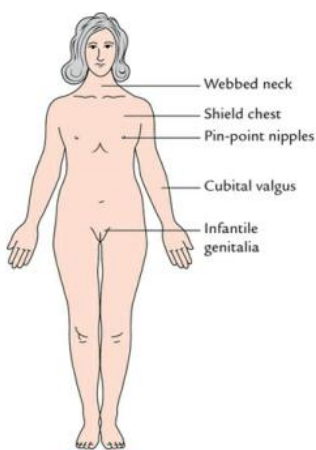


FIG. 25.7 Turner's syndrome.

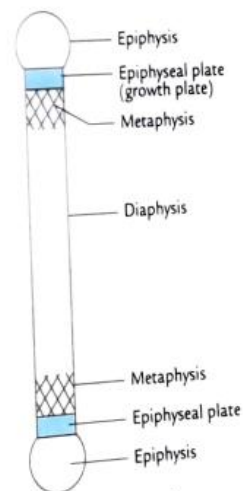


Fig. 31.1 Parts of a growing long bone.

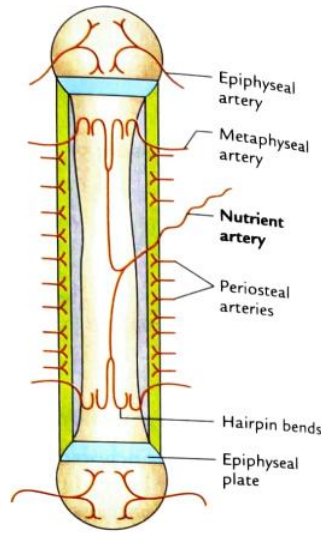


Fig. 31.2 Blood supply to a growing long bone.

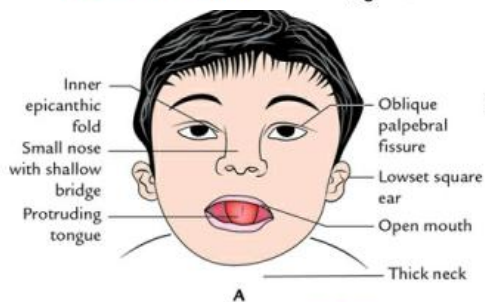
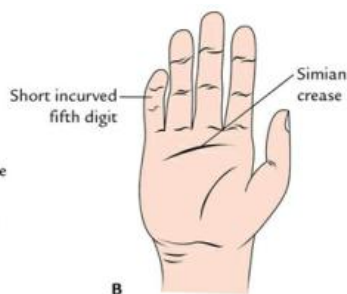


FIG. 25.5 Down's syndrome.



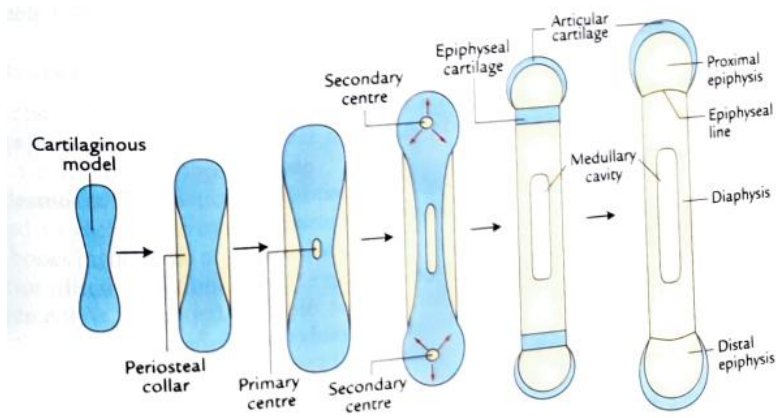


Fig. 31.3 Cartilaginous ossification of a long bone.

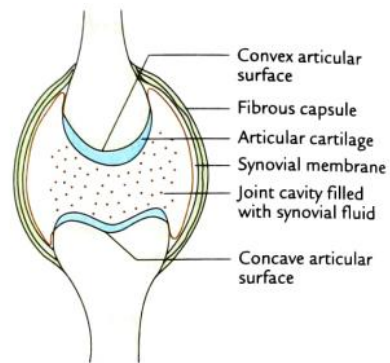
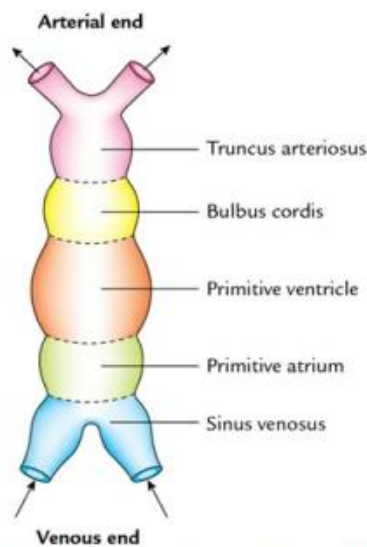


Fig. 32.3 Diagrammatic representation of a typical synovial joint.



G. 4.7 Dilatations of the primitive heart tube.

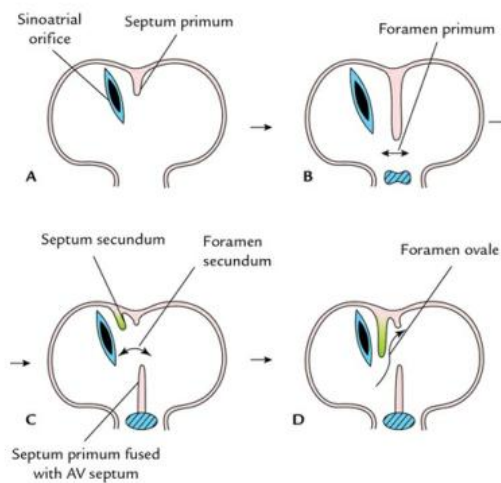


FIG. 4.8 Development of interatrial septum.

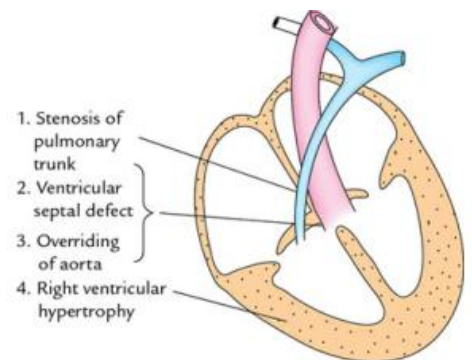


FIG. 4.11 Fallot's tetralogy.

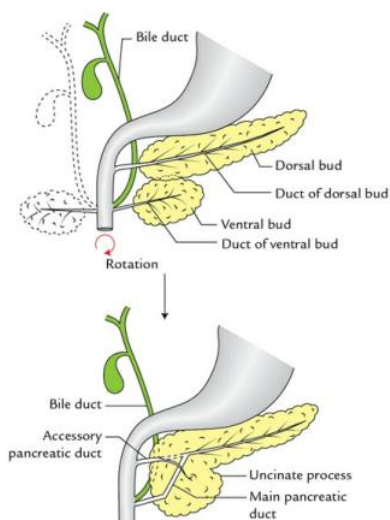


FIG. 12.10 Development of the pancreas.

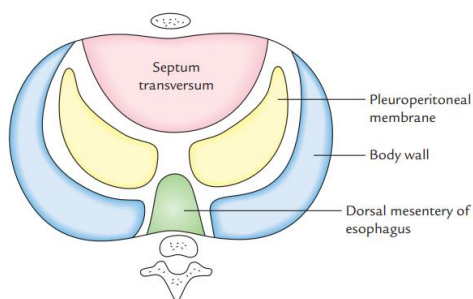


Fig. 14.10 Developmental components of the diaphragm.

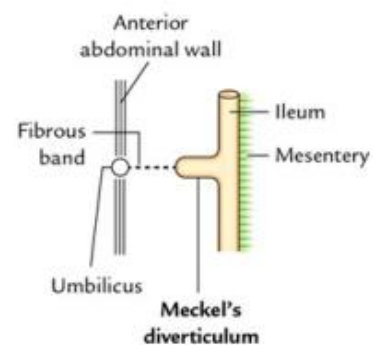


FIG. 13.2 Meckel's diverticulum.

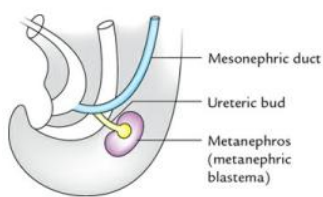


FIG. 14.5 Development of the kidney.

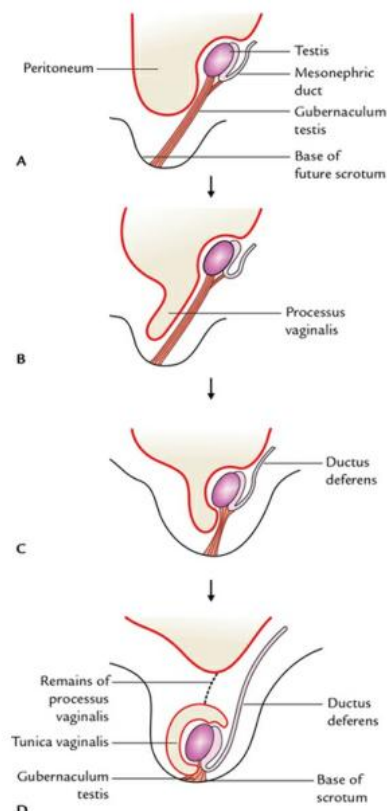


FIG. 8.7 Descent of the testis.

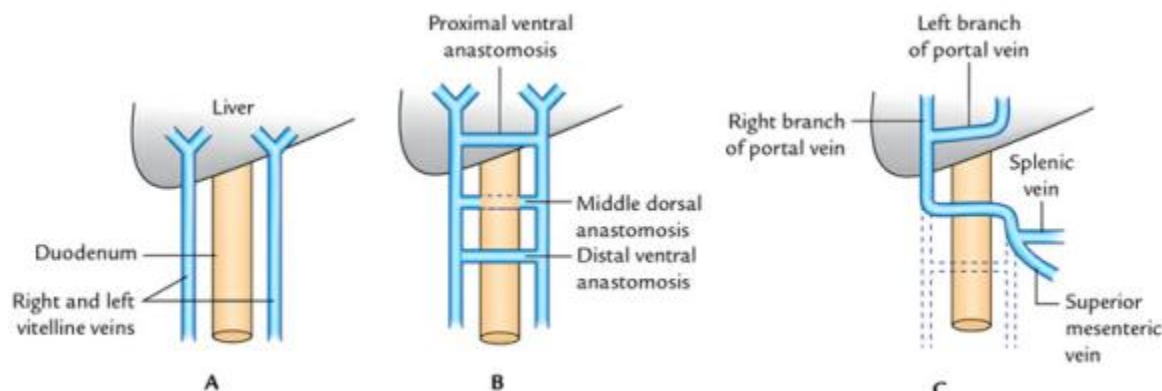
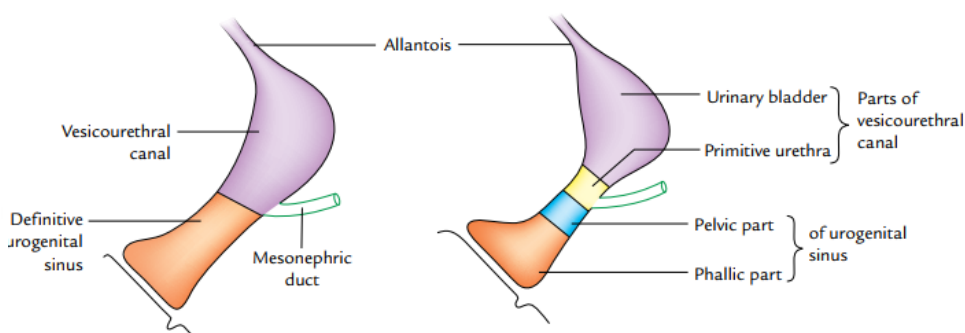


FIG. 12.13 Development of the portal vein.

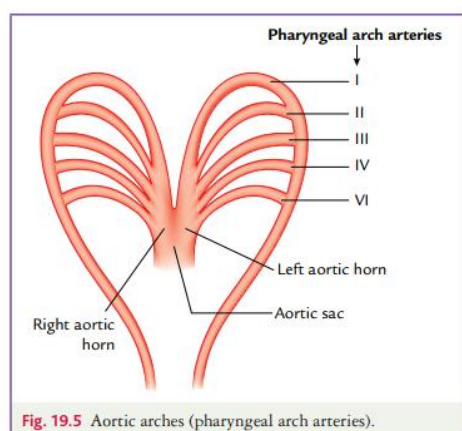


Fig. 19.5 Aortic arches (pharyngeal arch arteries).

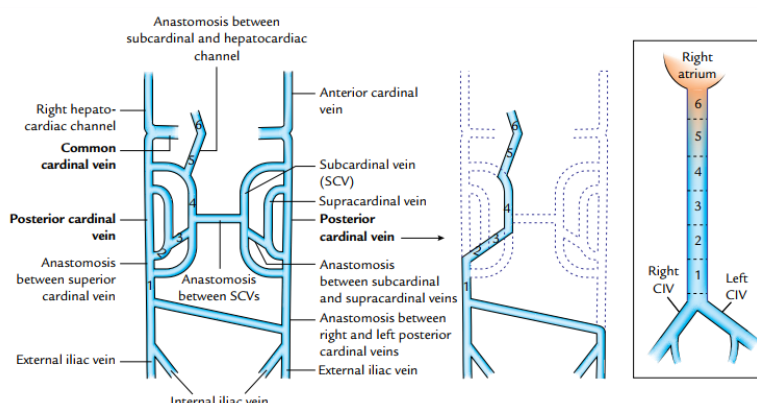
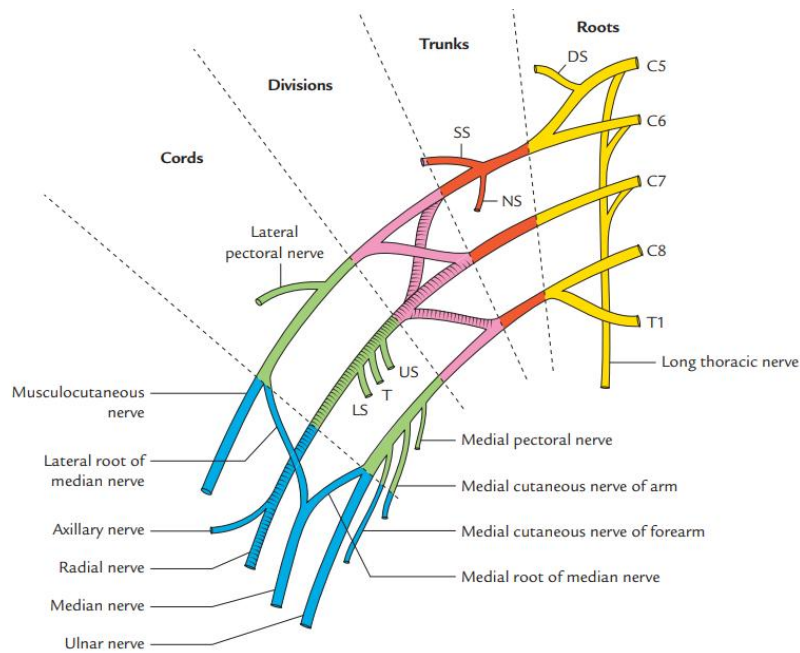


Fig. 19.21 Development of the inferior vena cava. Figure in the inset on the right shows 1 = Right posterior cardinal vein (sacro-cardinal segment). 2 = Supracardinal vein. 3 = Supracardinal-subcardinal anastomosis. 4 = Right subcardinal vein (renal segment). 5 = Anastomotic channel between subcardinal vein and right hepatocardiac channel. 6 = Right hepatocardiac channel (hepatic segment). CIV = common iliac vein.

Development of the arch of aorta 1 - its proximal part develops from left part of distal part of aortic sac (right part of aortic sac forms brachio-cephalic artery). v. Proximal part of aortic sac forms the pulmonary trunk. 2 - its main middle part develops from left 4 th aortic arch. 3 - its distal part develops from the left dorsal aorta between 4 th & 6 th aortic arches.



4.10 Brachial plexus and its branches (SS = suprascapular nerve, NS = nerve to subclavius, US = upper subscapular, LS = lower subscapular nerve, T = thoraco-dorsal nerve, DS = dorsal scapular nerve).

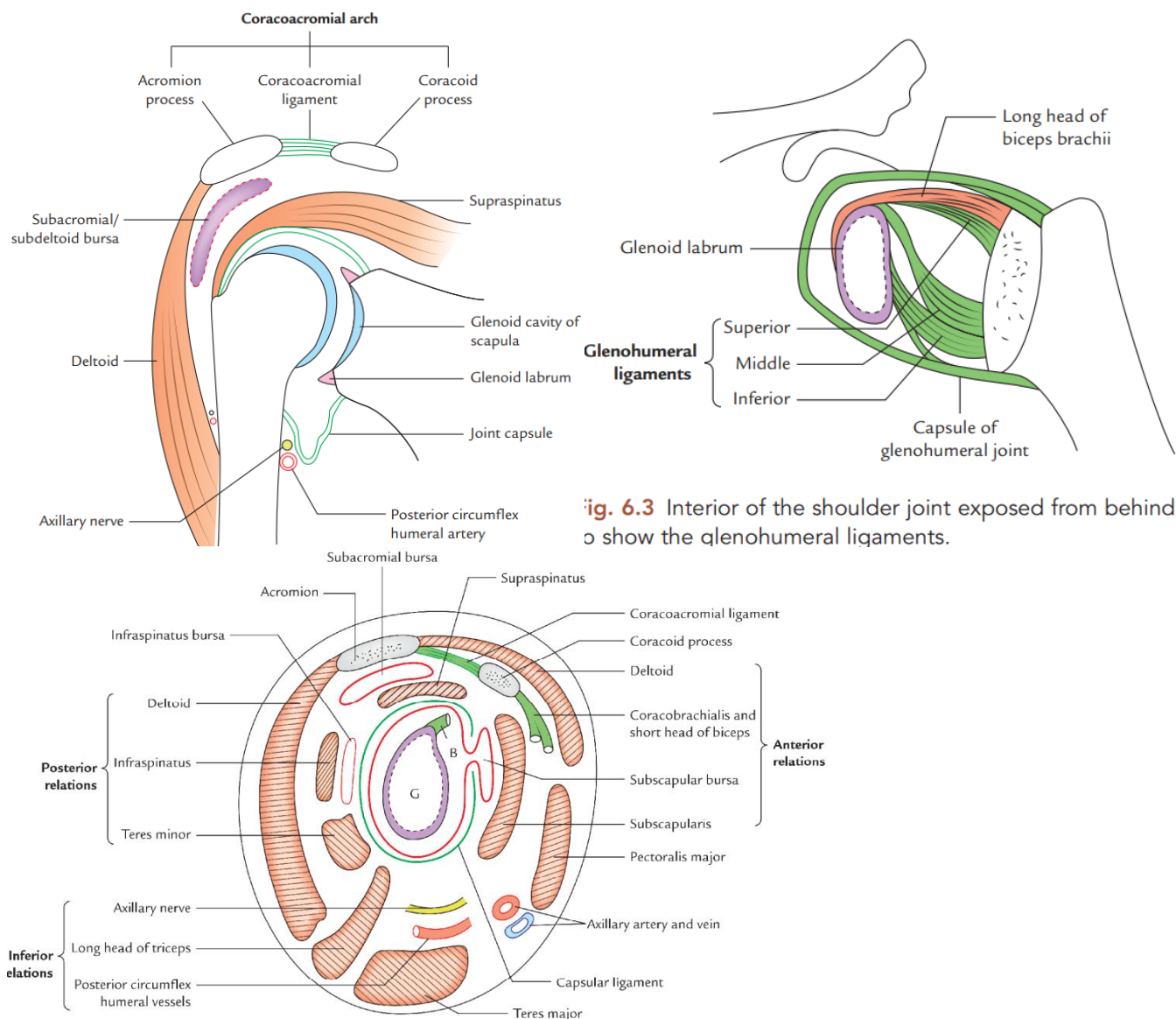


Fig. 6.3 Interior of the shoulder joint exposed from behind to show the glenohumeral ligaments.

Relations of the shoulder joint as seen in sagittal section (G = glenoid fossa, B = long head of biceps).

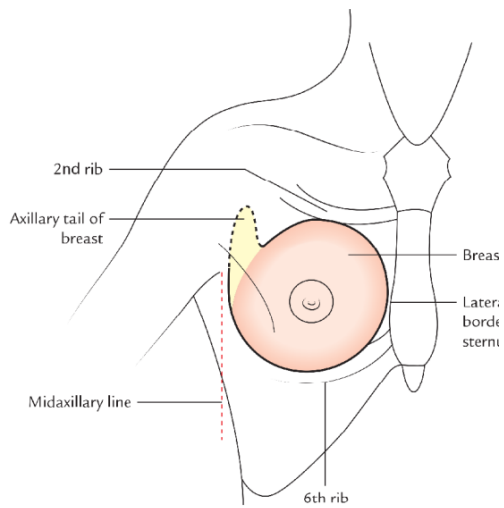


Fig. 3.12 Location and extent of the breast.

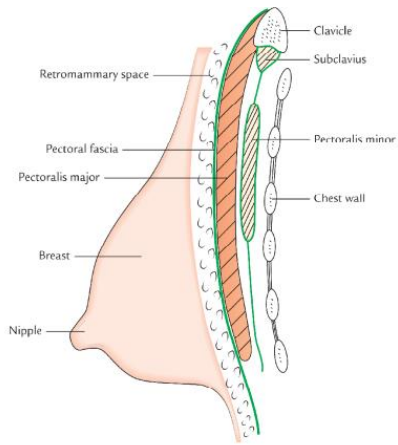
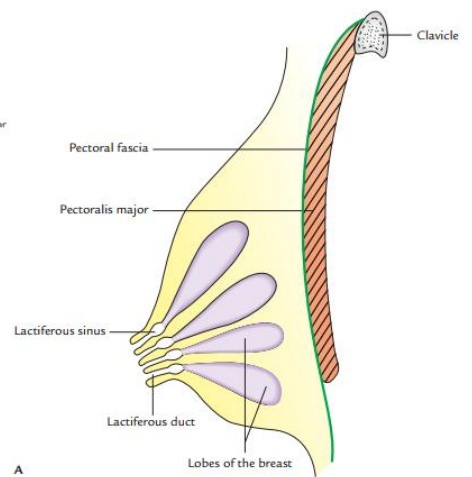
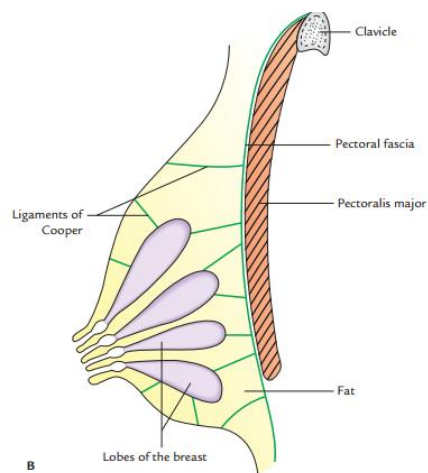


Fig. 3.14 Deep relations of the breast.



A



B

e breast); B, stroma of the breast (suspensory ligaments of

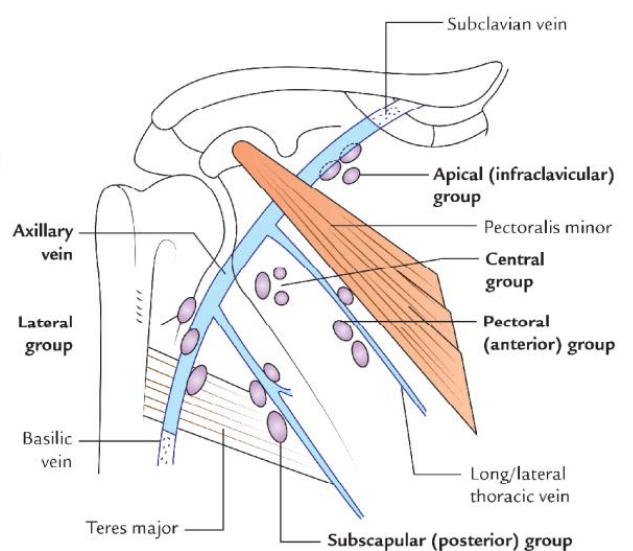
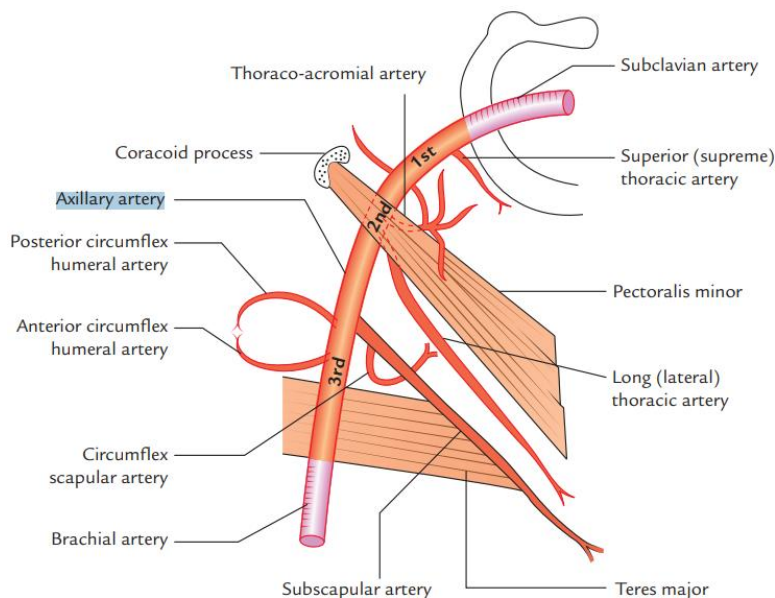
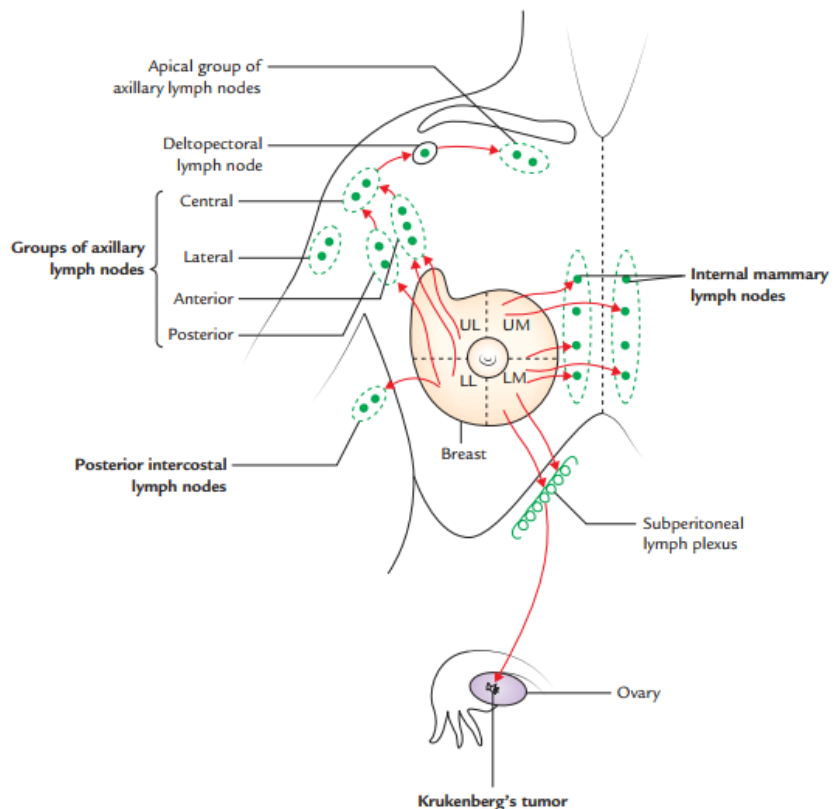
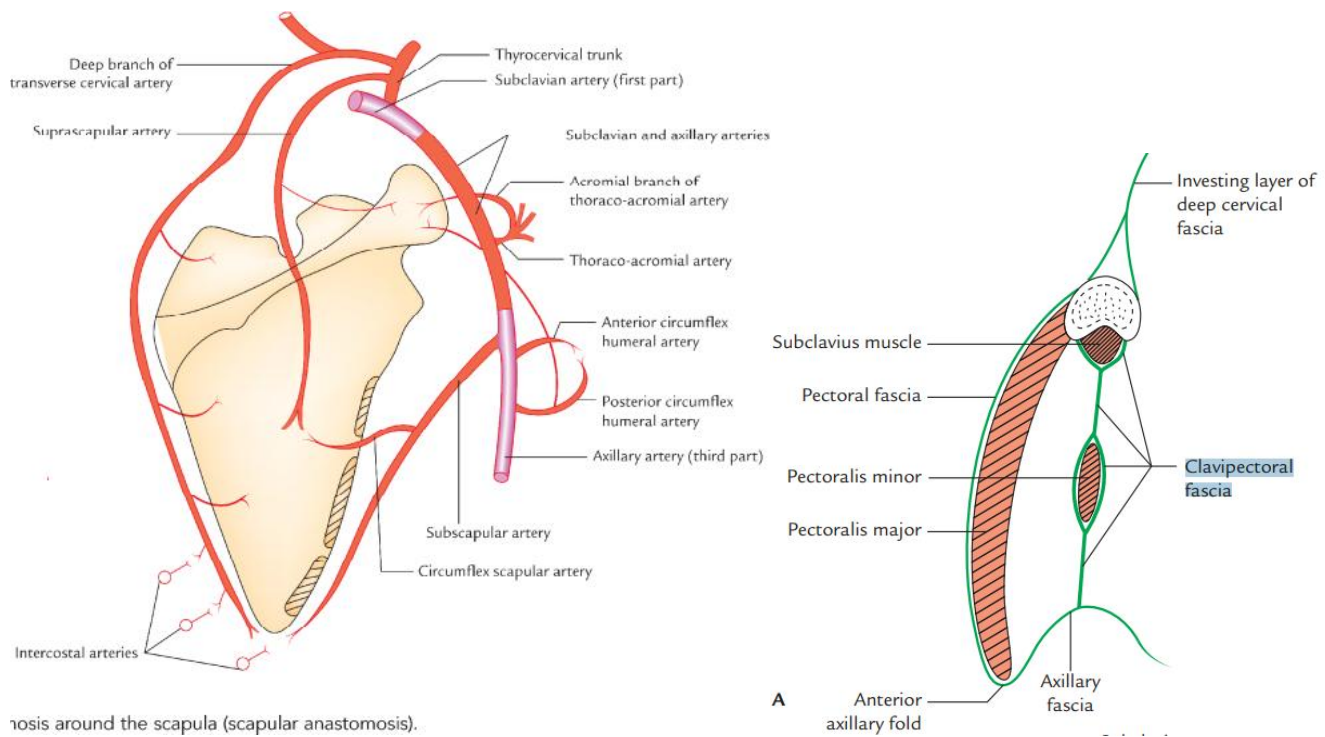
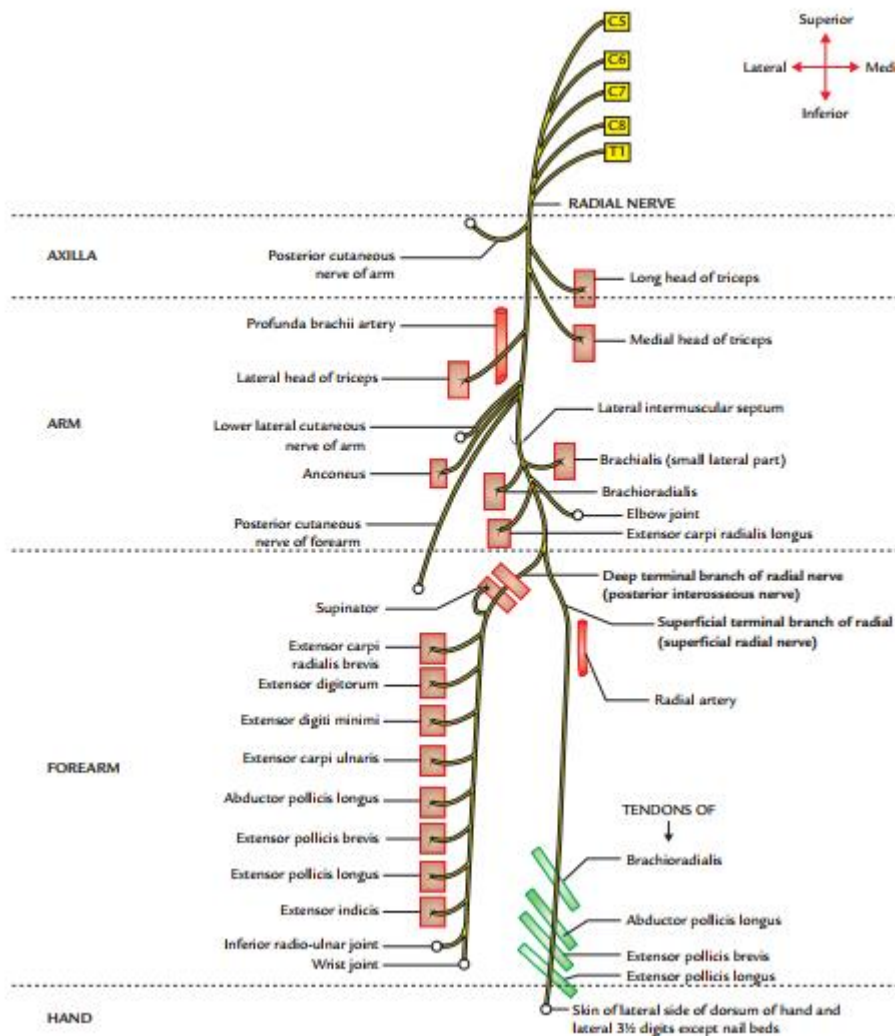


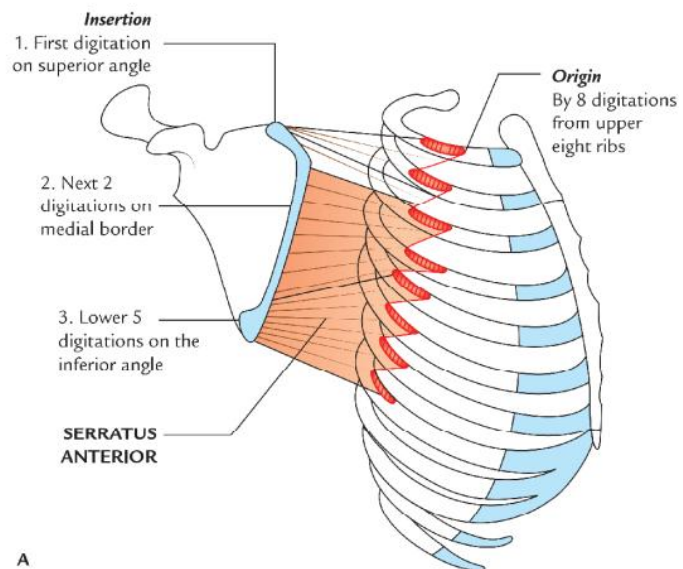
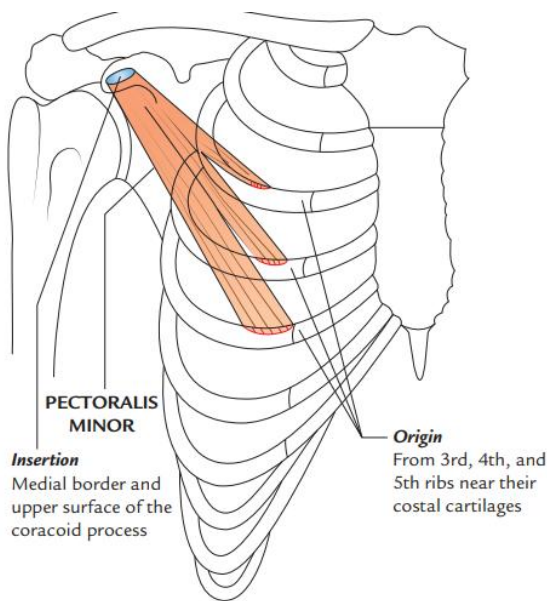
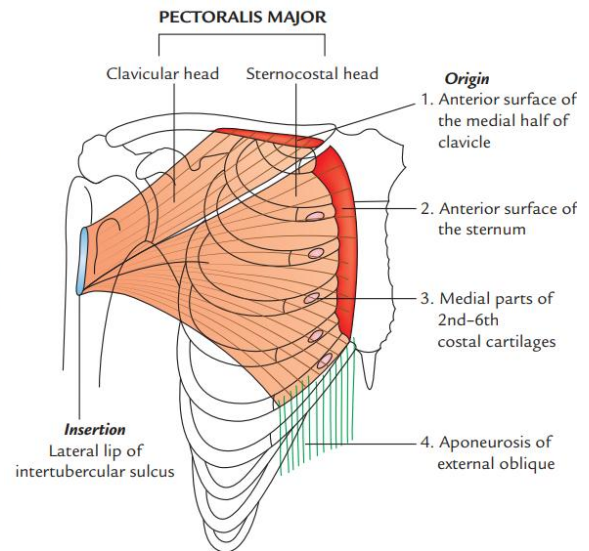
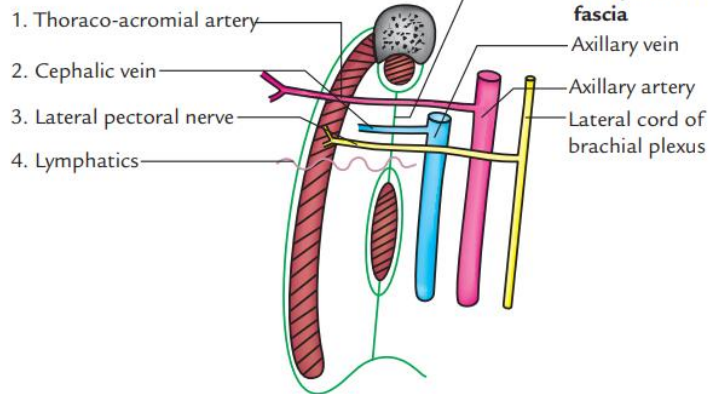
Fig. 4.8 Axillary lymph nodes.



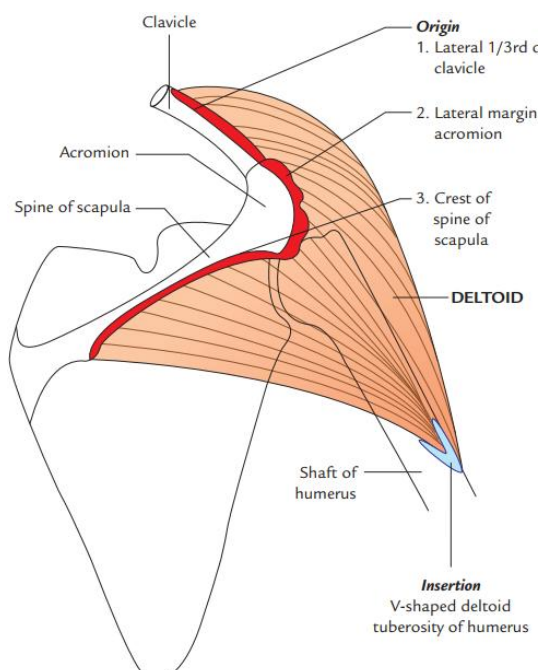
osis around the scapula (scapular anastomosis).



Structures piercing clavipectoral fascia



A



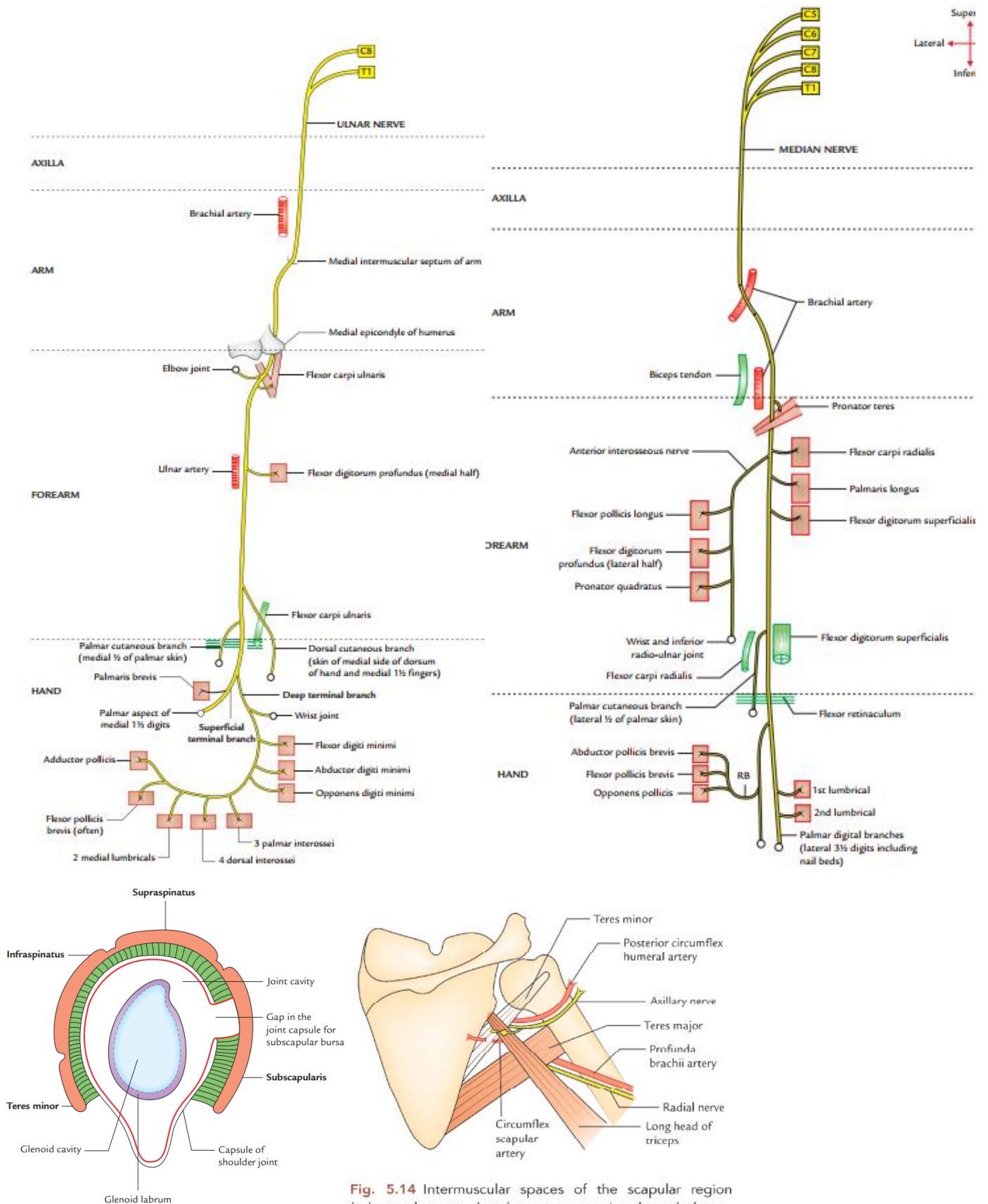
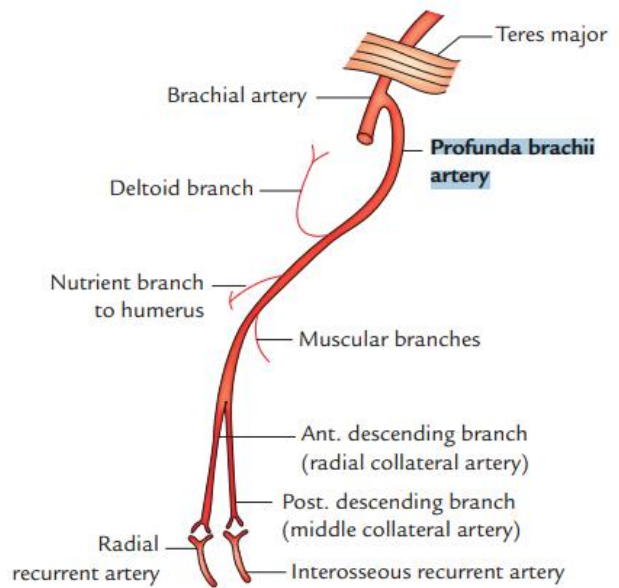
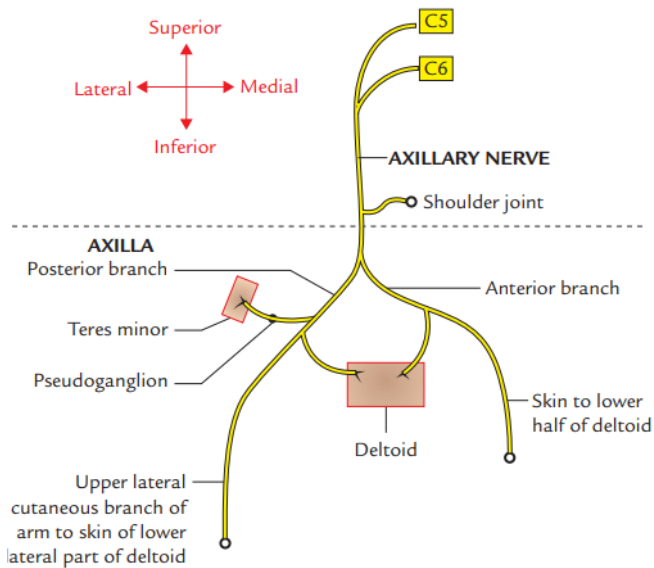
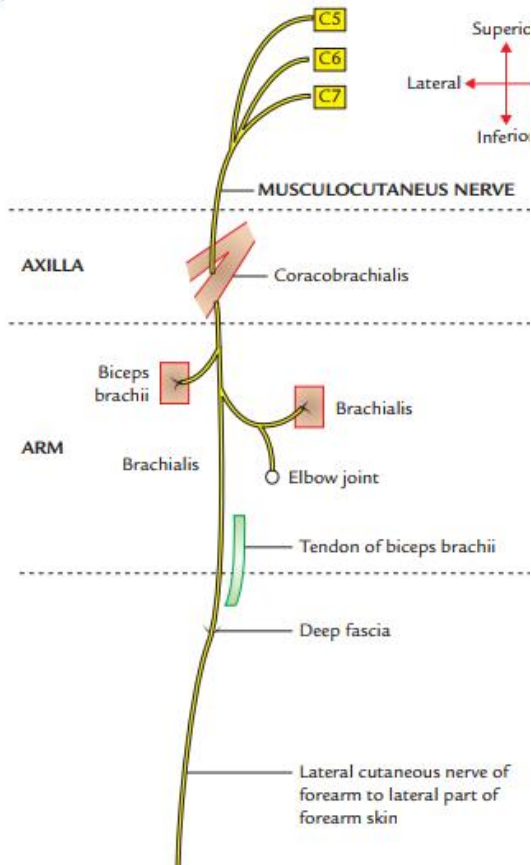
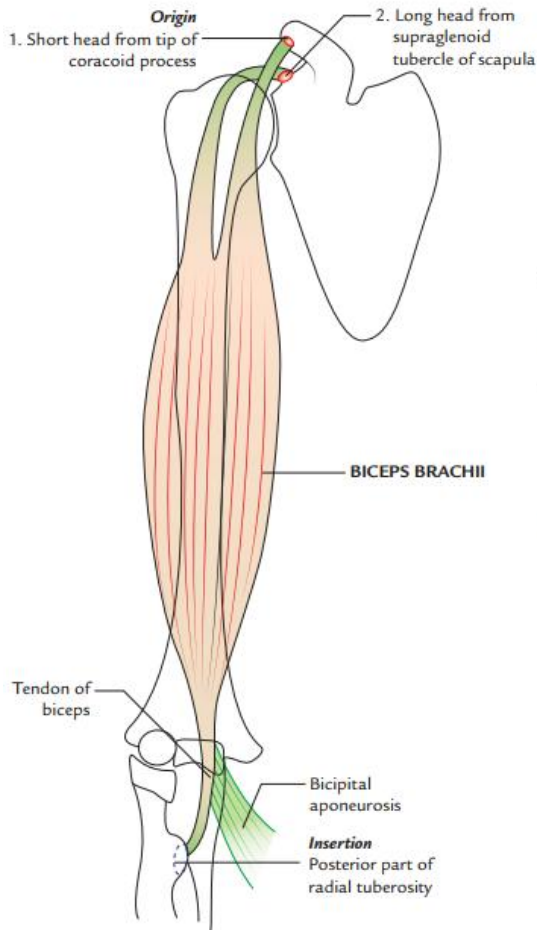


Fig. 5.14 Intermuscular spaces of the scapular region (subscapular spaces) and structures passing through them.



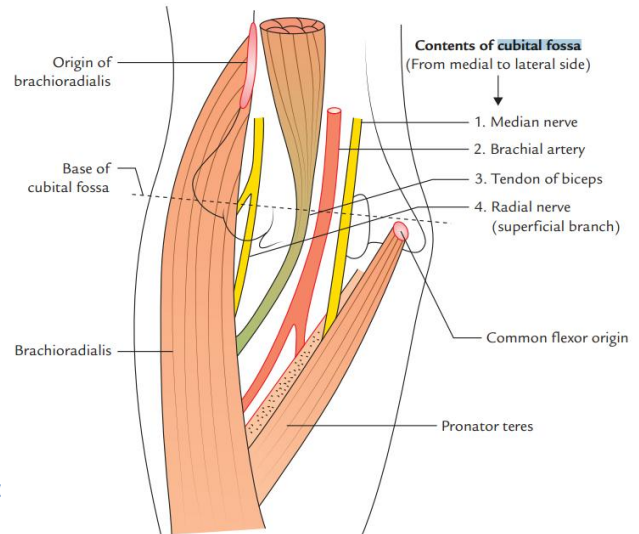
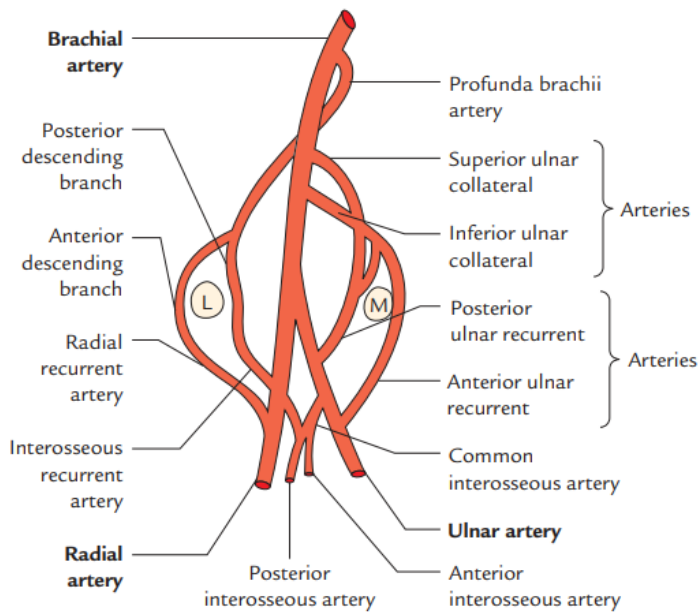
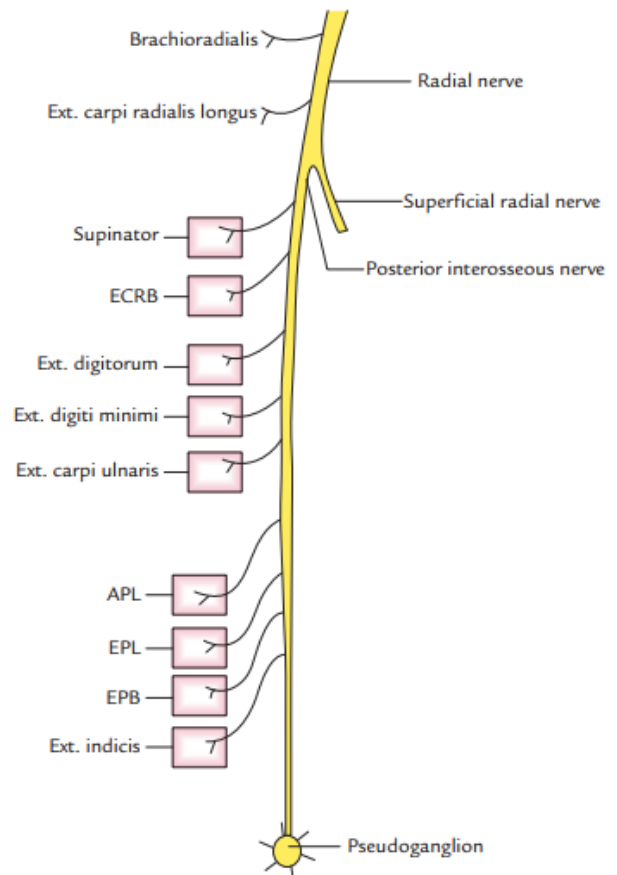
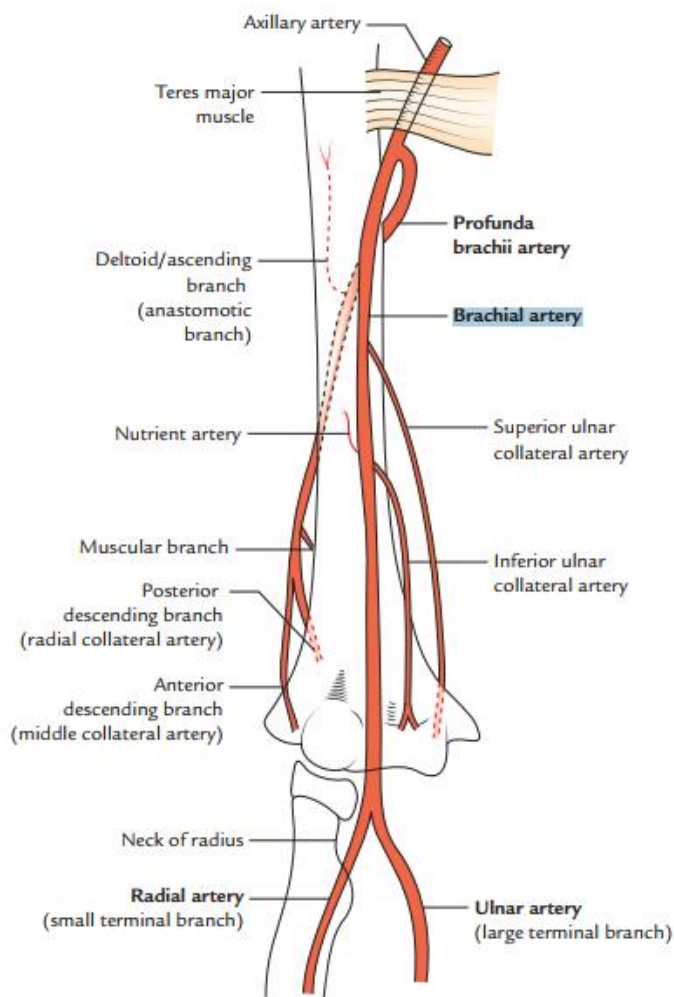
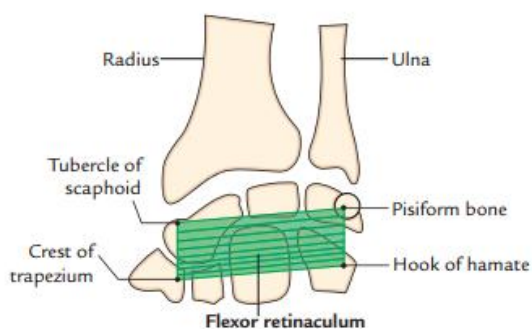
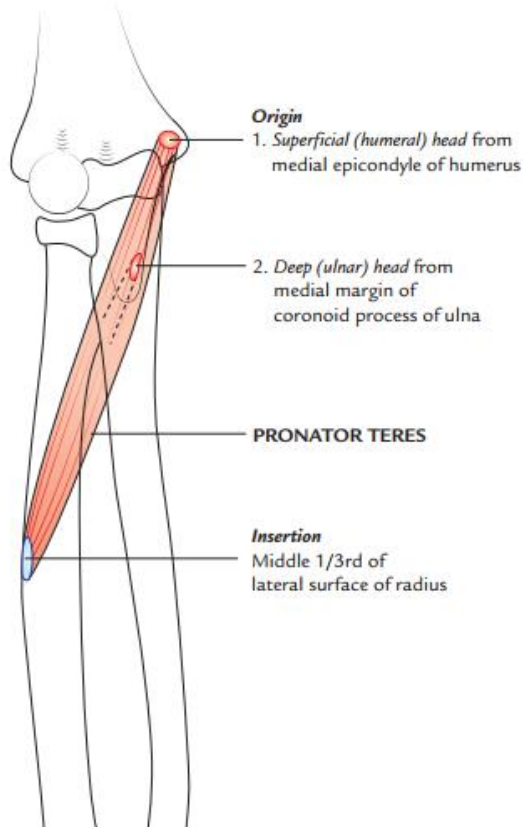
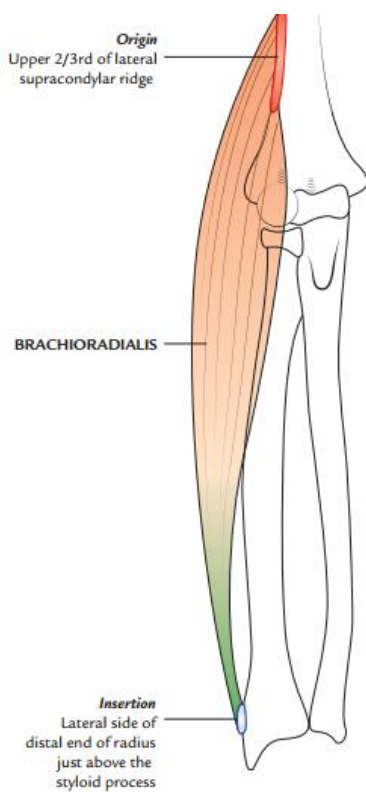
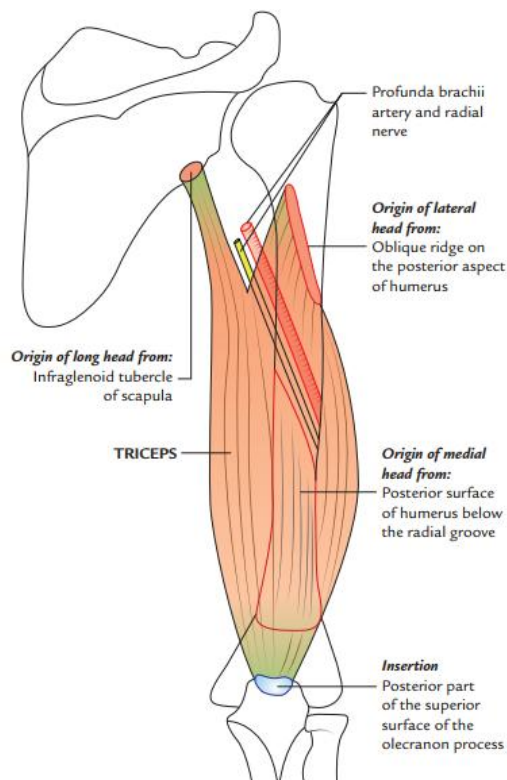
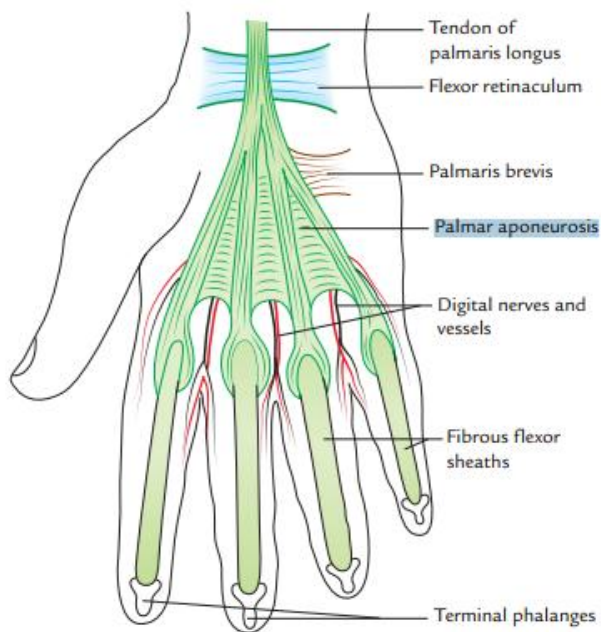


Fig. 8.10 Arterial anastomosis around the elbow joint (L = lateral epicondyle, M = medial epicondyle).





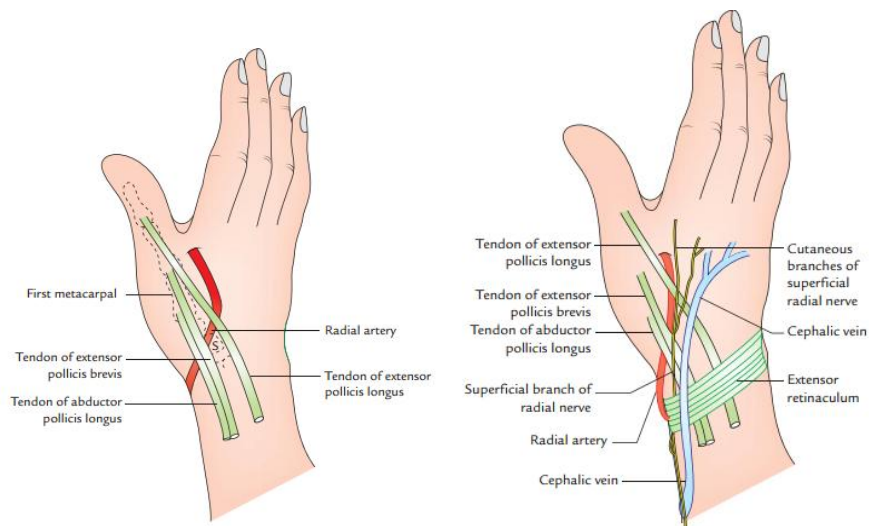


Fig. 11.28 Boundaries and contents of the anatomical snuffbox (S = scaphoid). Fig. 11.29 Structures crossing the roof of anatomical snuffbox.

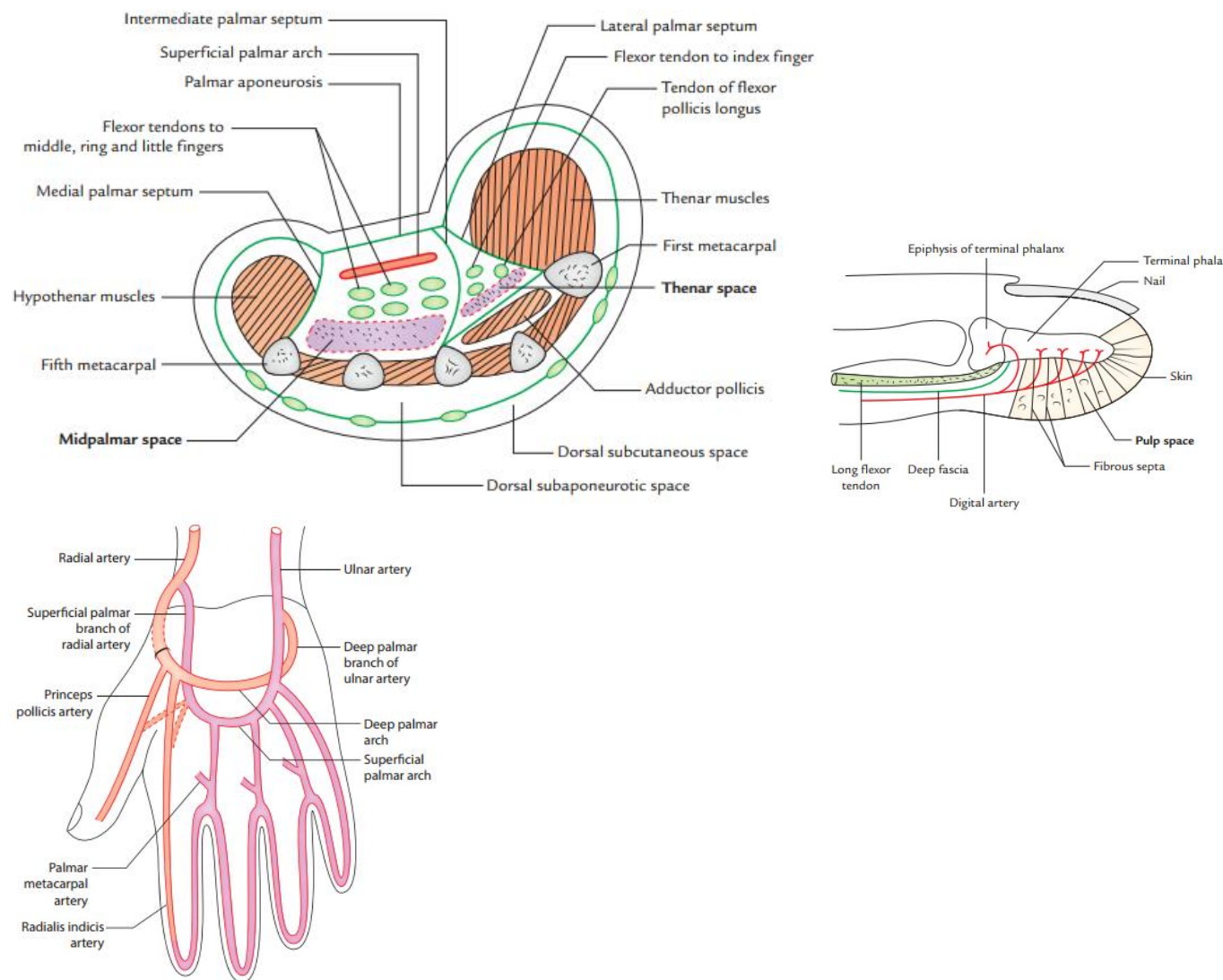


Fig. 11.17 Superficial and deep palmar arterial arches.

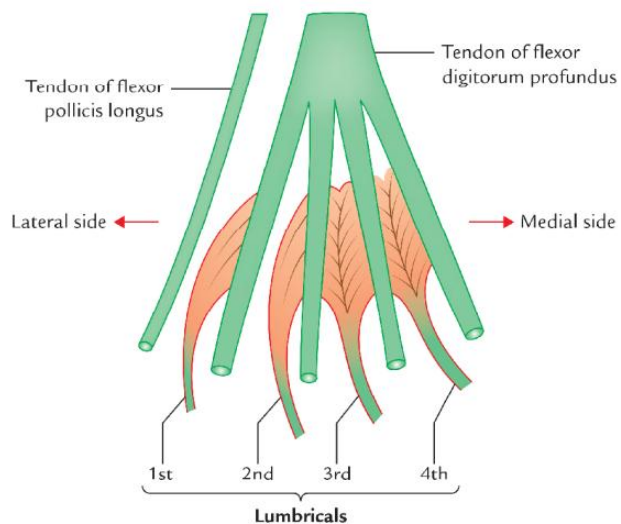
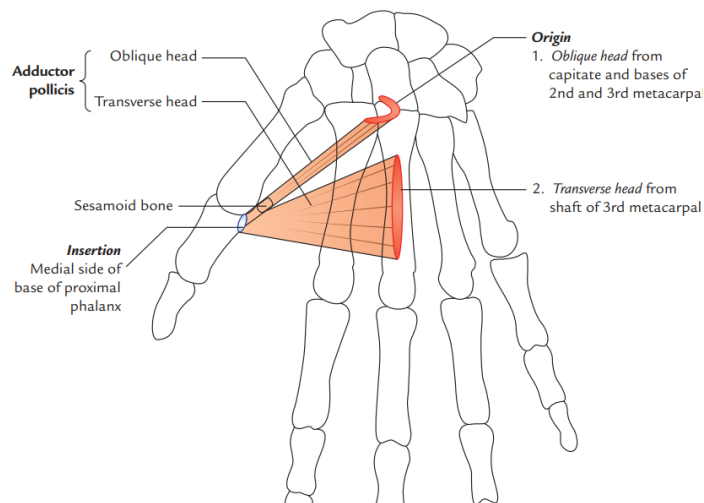


Fig. 11.14 Lumbrical muscles.

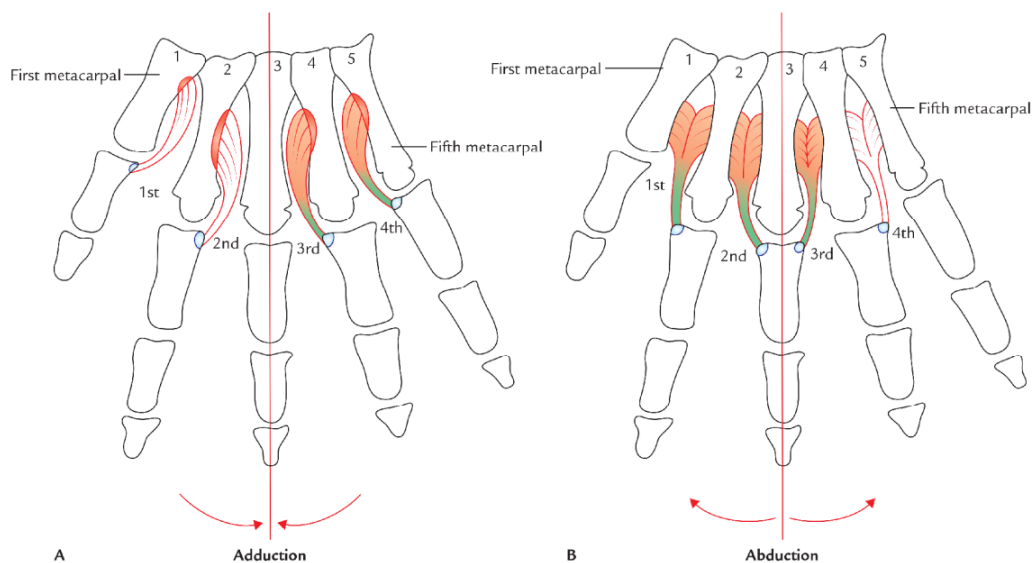
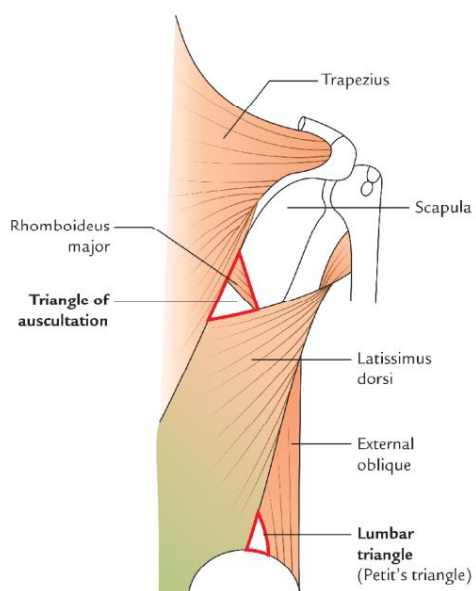


Fig. 11.15 Interosseous muscles: A, palmar interossei; B, dorsal interossei.

HEAD AND NECK

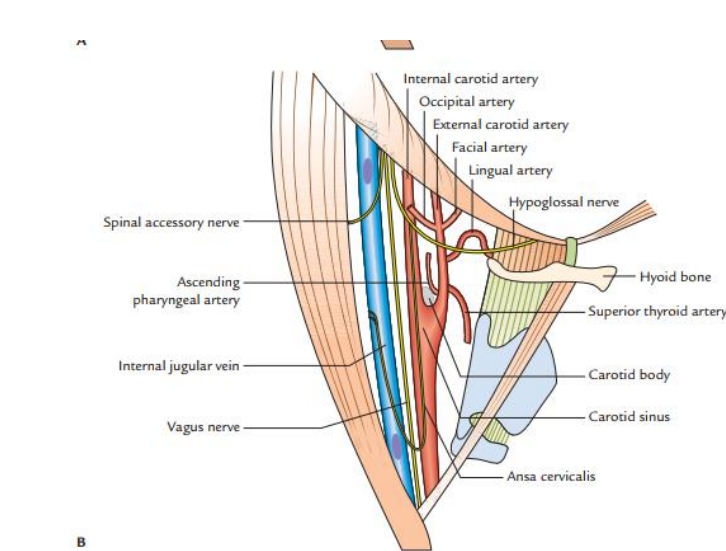


Fig. 6.5 Digastric (submandibular) triangle: A, boundaries and floor; B, boundaries and contents.

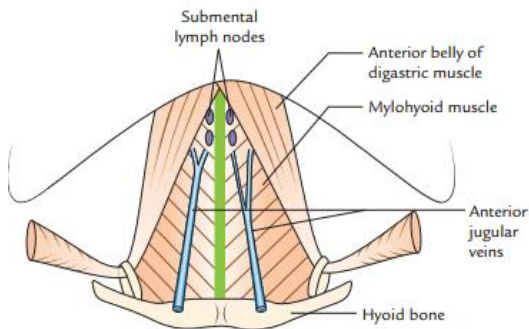


Fig. 6.4 Boundaries and contents of the submental triangle.

Boundaries and contents of the suboccipital triangle.

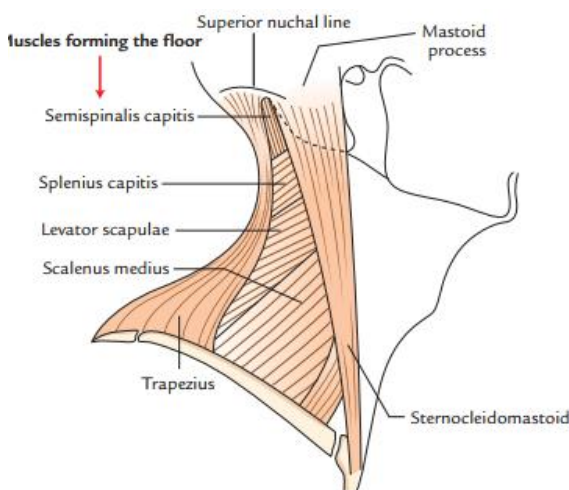
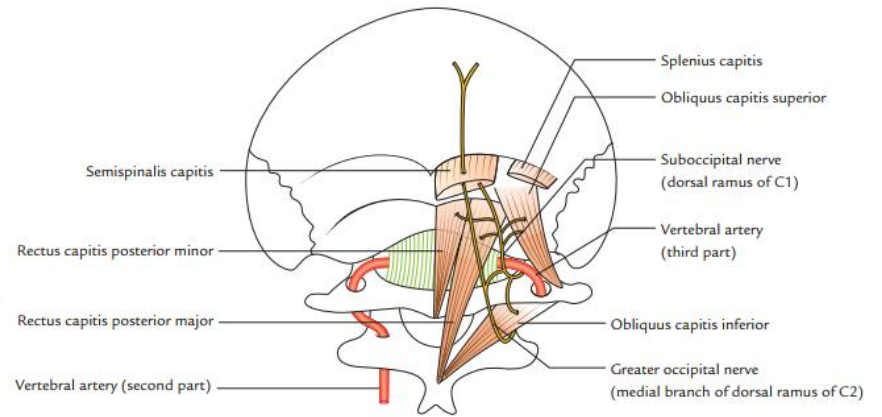
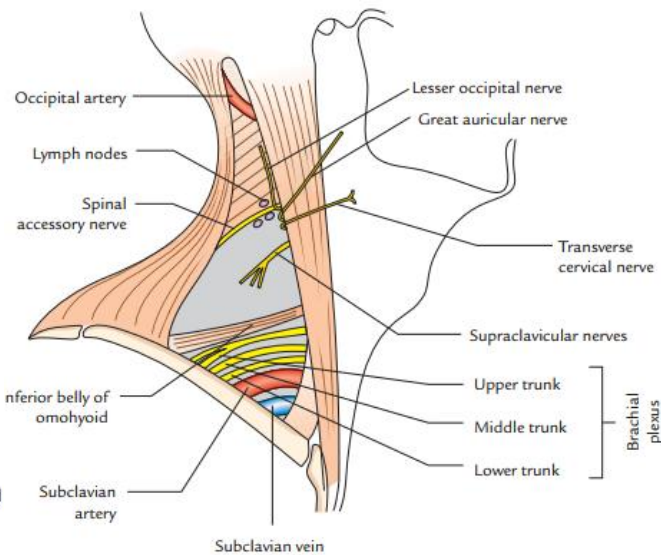
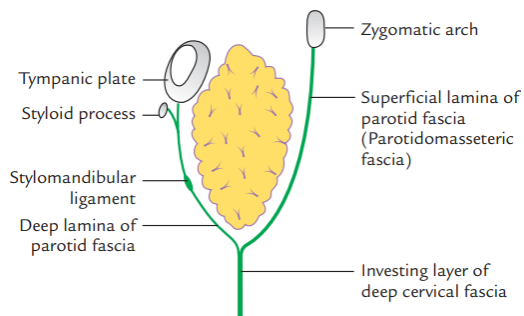


Fig. 5.6 Boundaries and floor of the posterior triangle.

Main contents of the posterior triangle.





8.3 Parotid capsule.

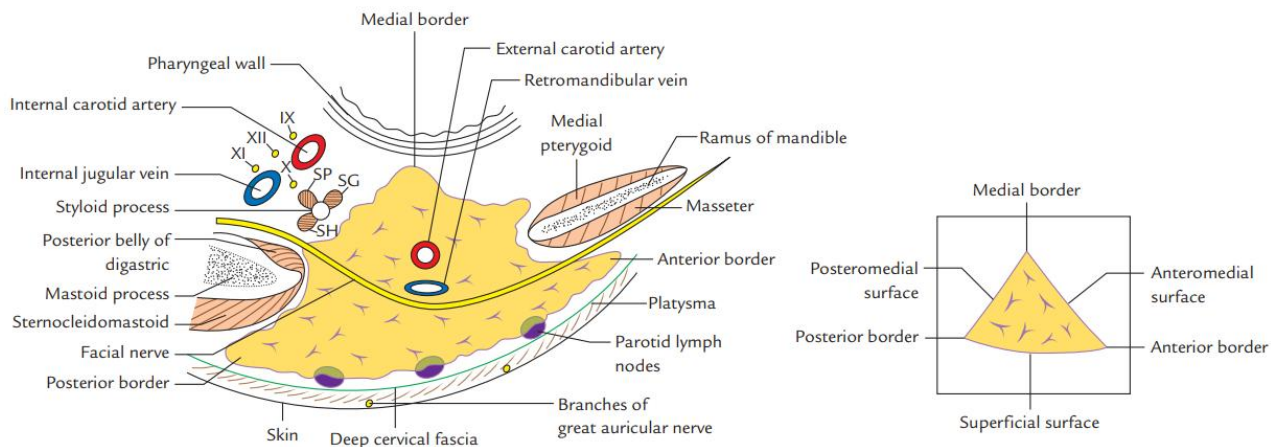


Fig. 8.4 Horizontal section through parotid gland showing its relations and the structures passing through it. The inset figure shows borders and surfaces of the parotid gland (SG = styloglossus muscle, SH = stylohyoid muscle, SP = stylopharyngeus muscle).

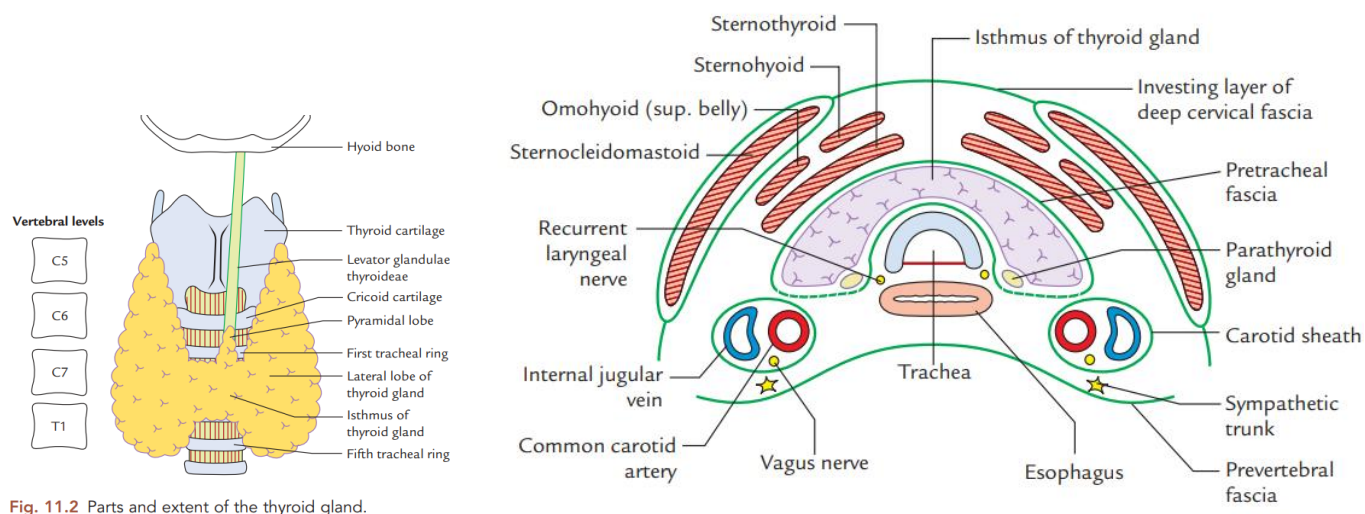
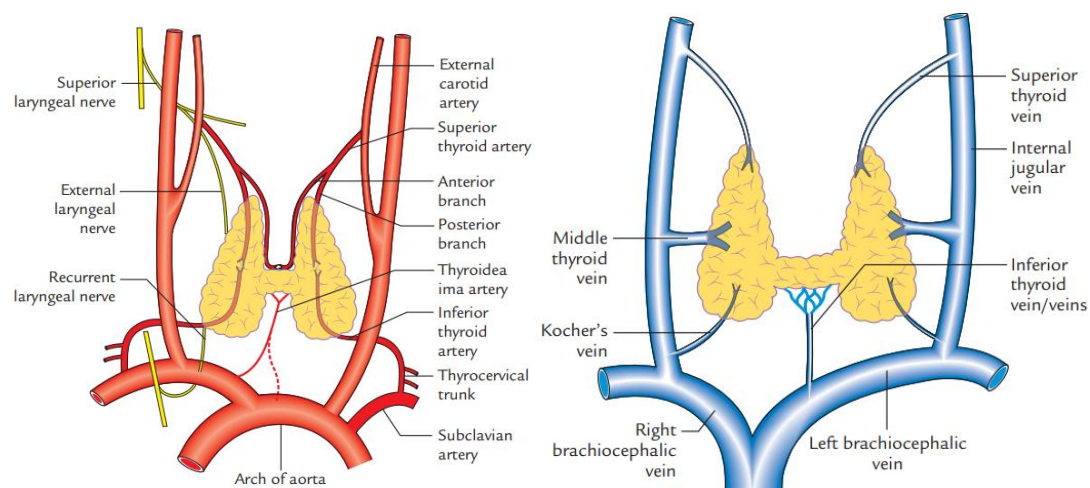
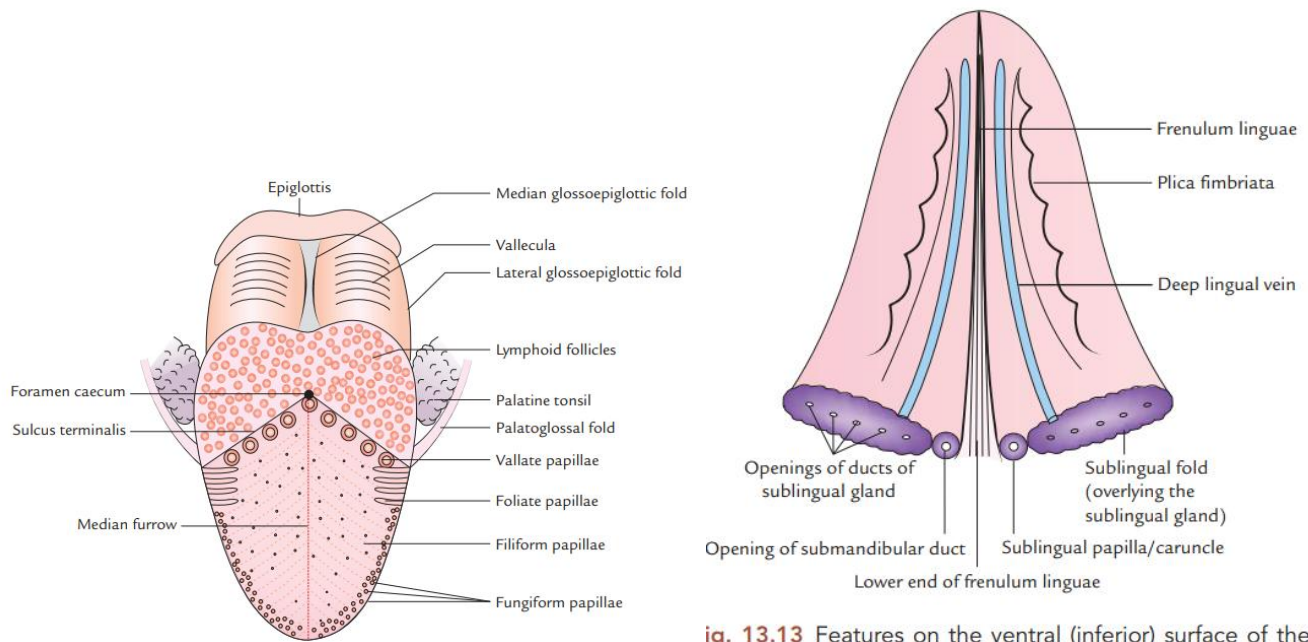


Fig. 11.2 Parts and extent of the thyroid gland.





ig. 13.13 Features on the ventral (inferior) surface of the

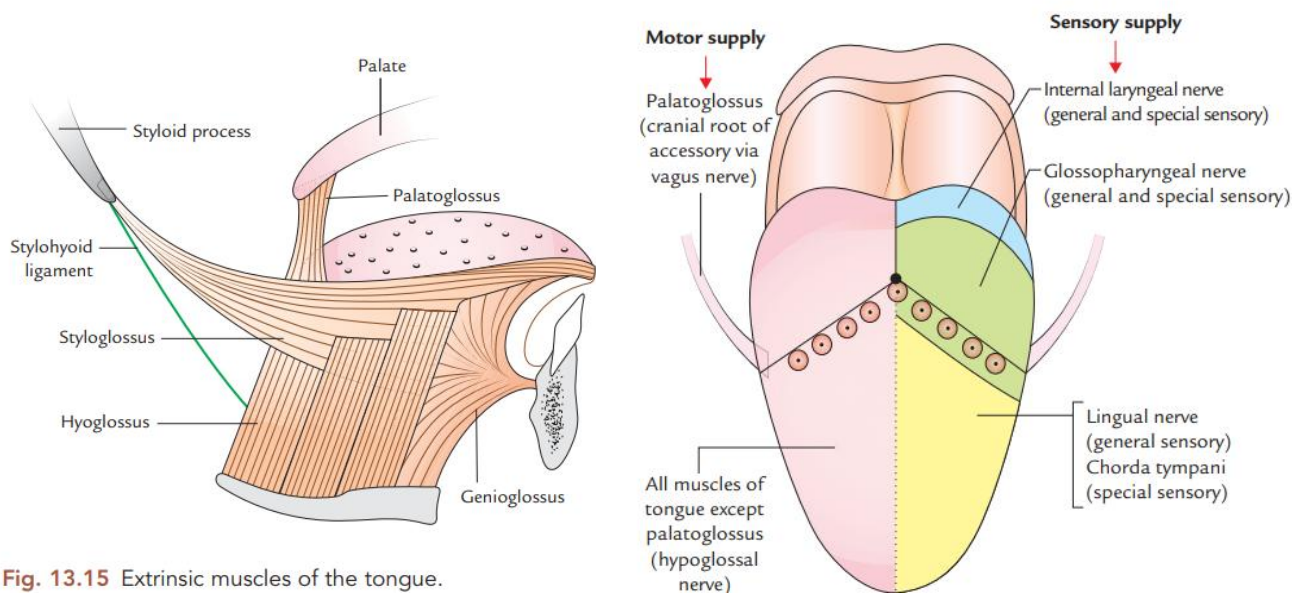


Fig. 13.15 Extrinsic muscles of the tongue.

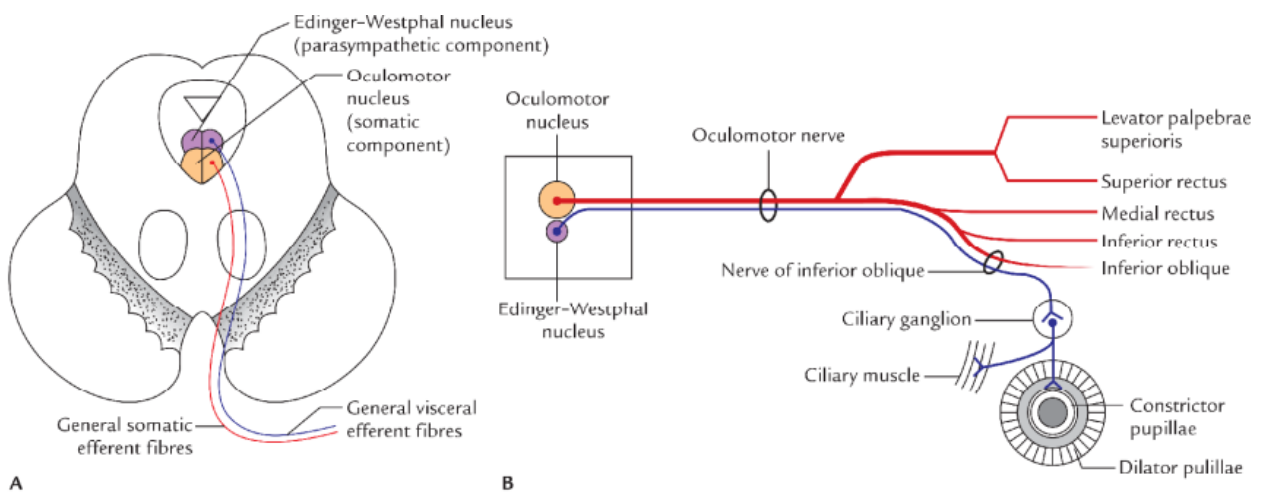


Fig. 22.3 Oculomotor nerve: A, the functional components and nuclei of the oculomotor nerve; B, the distribution of the constitutional fibres of the oculomotor nerve.

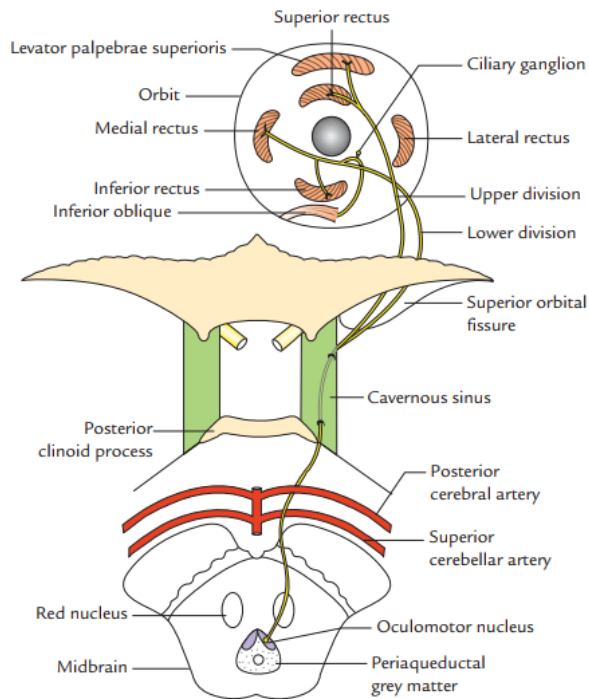


Fig. 22.4 Origin, course, and distribution of the oculomotor nerve.

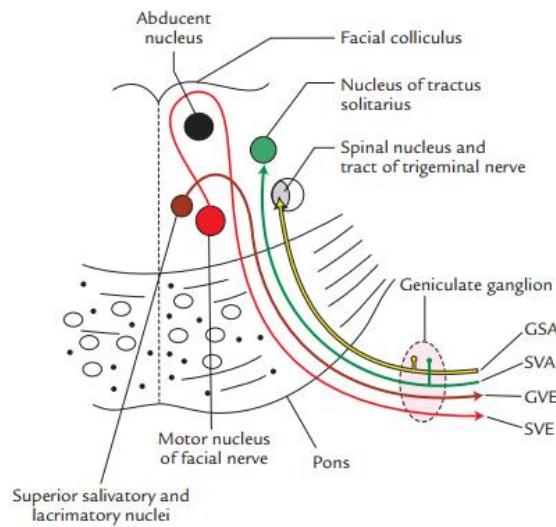
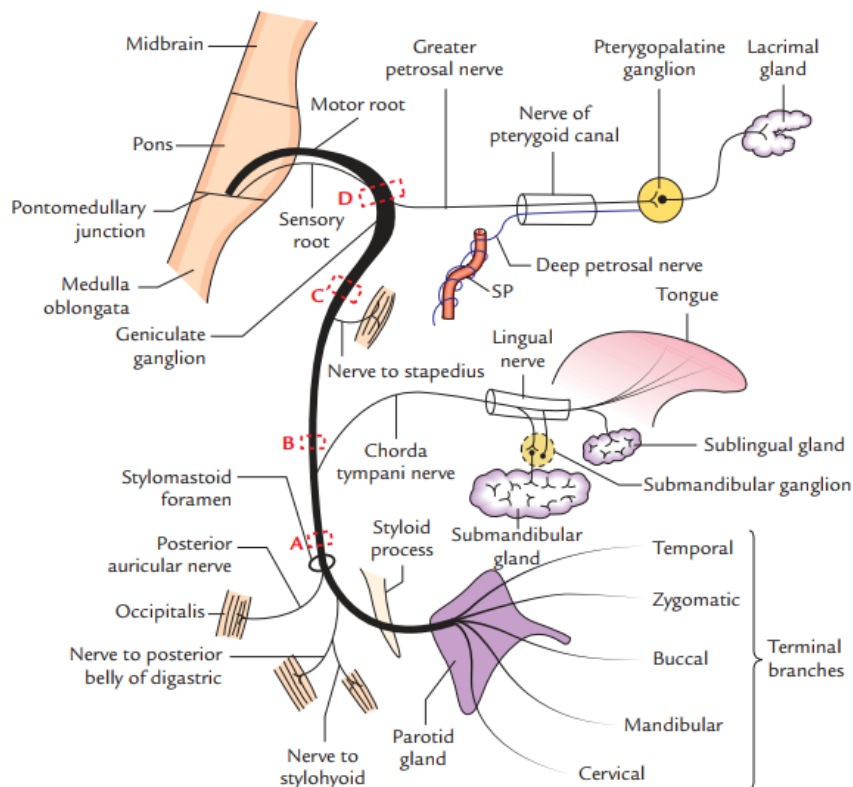
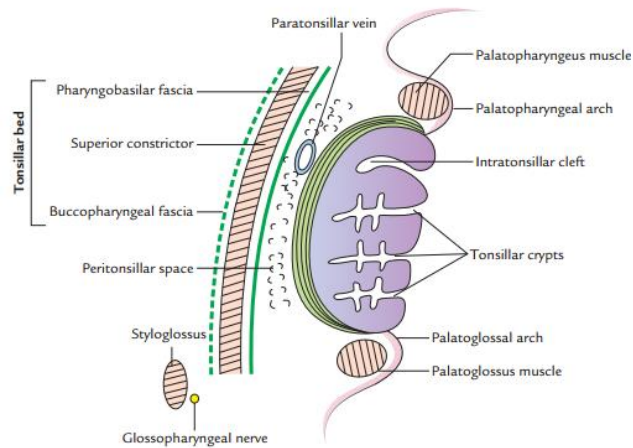


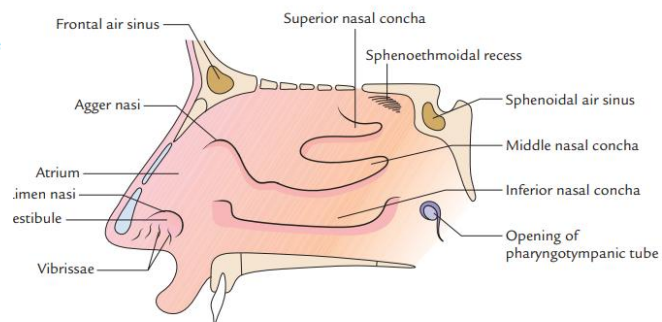
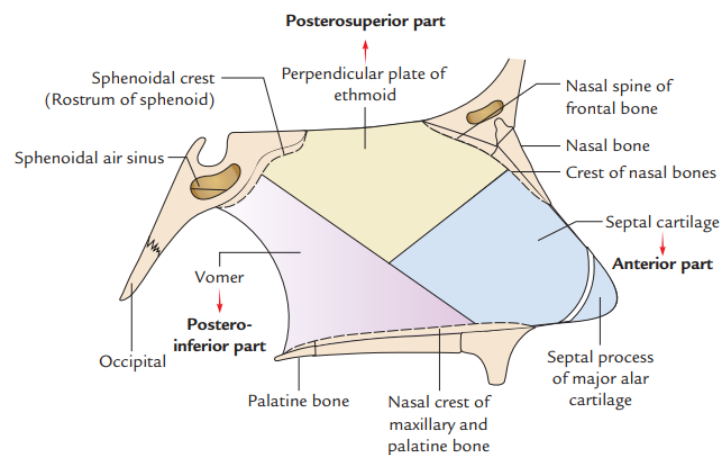
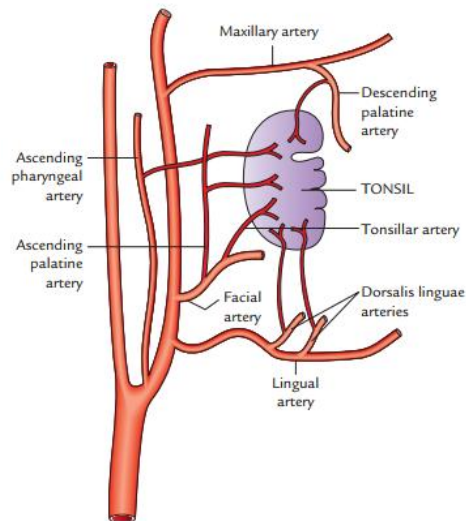
Fig. 22.10 Functional components and nuclei of the facial nerve.



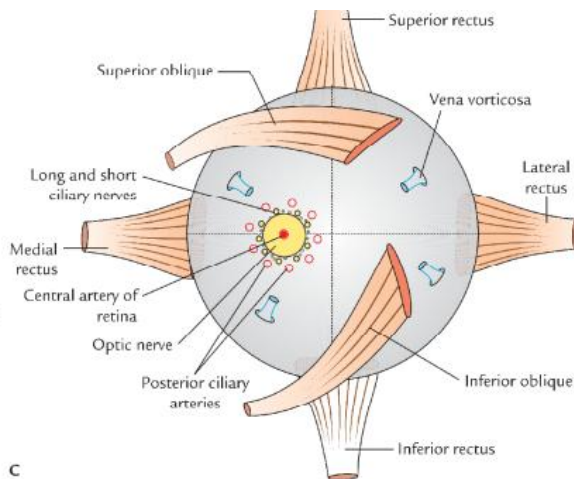
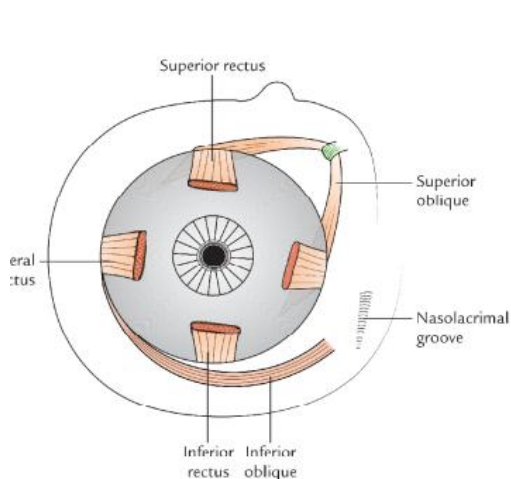
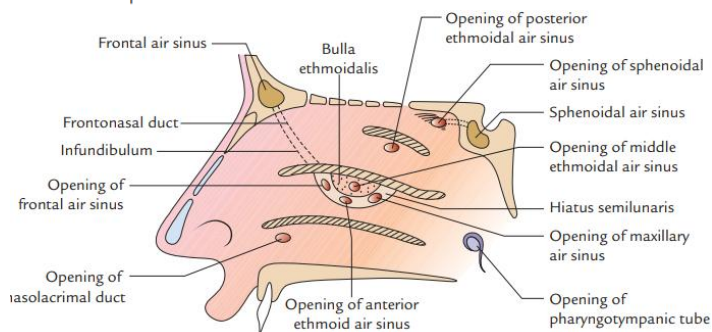
urse, and distribution of the facial nerve (SP = sympathetic plexus around internal



il section through tonsillar fossa showing medial and lateral surfaces of the tonsil



the nasal septum.



19.6 Extraocular muscles: A, origin; B, insertion of the recti muscles; C, insertion of the oblique muscles.

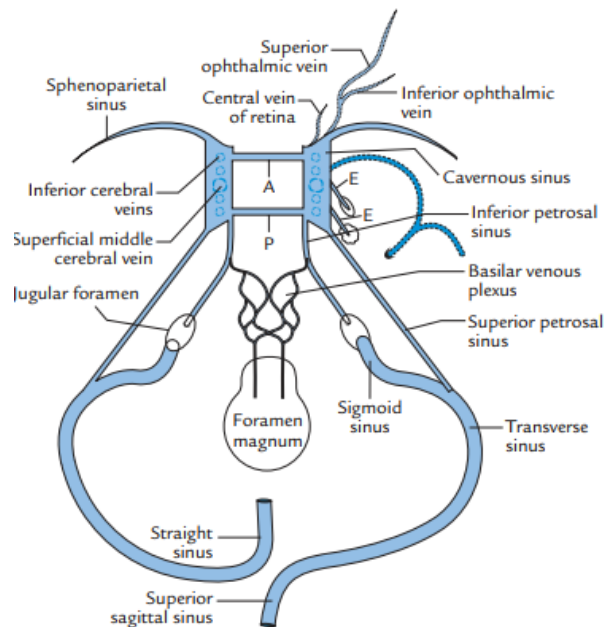
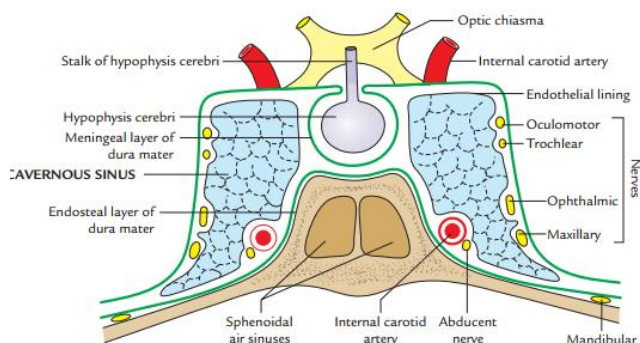


Fig. 21.17 Tributaries and communications of cavernous sinus (superior view) (A = anterior intercavernous sinus, P = posterior intercavernous sinus, E = emissary vein).

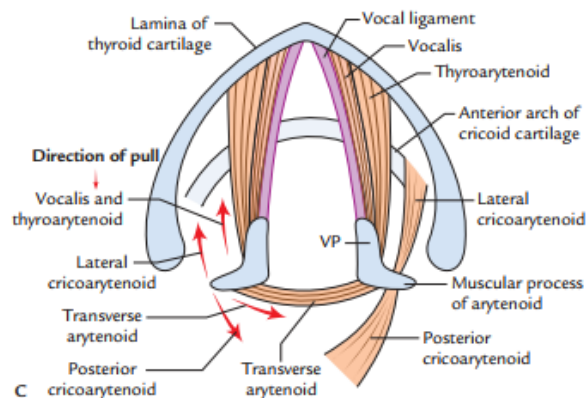
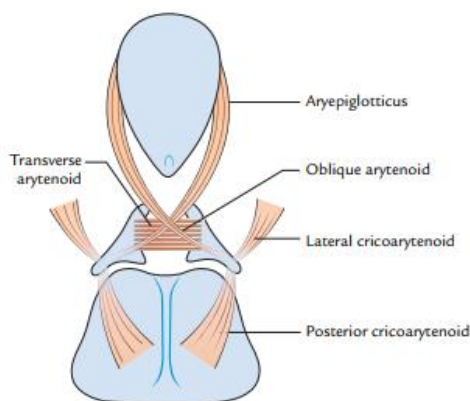


Fig. 15.6 Intrinsic muscles of the larynx: A, lateral view; B, posterior view; C, direction of pull of some intrinsic muscles (VP = vocal process of arytenoid).

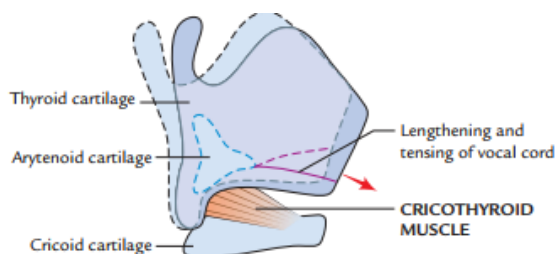


Fig. 15.7 Action of the cricothyroid muscle.

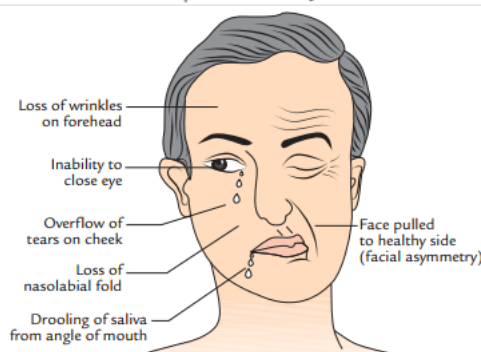
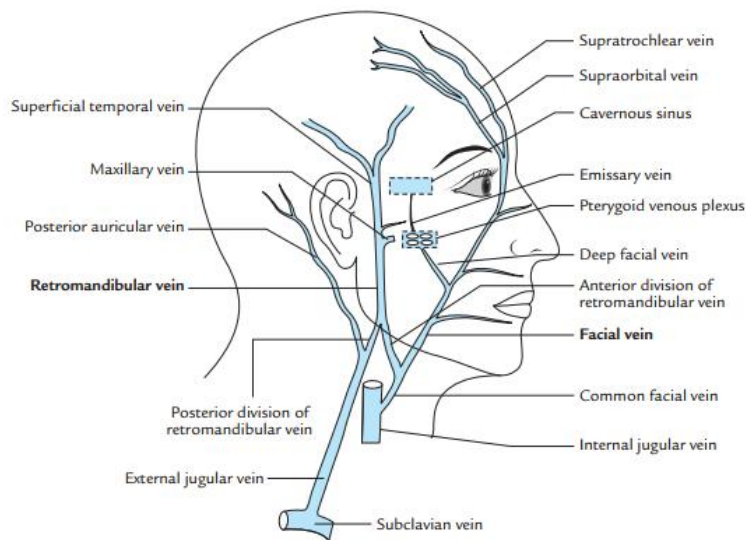


Fig. 3.14 Bell's palsy on the right side. Note the facial asymmetry (right side appears to be pulled on the left side).



Venous drainage of the face.

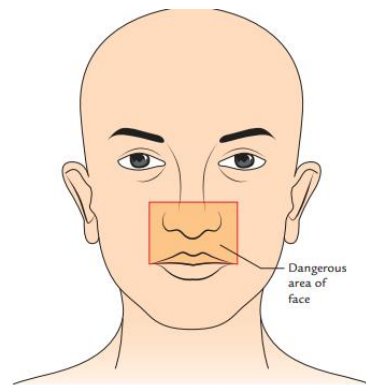


Fig. 3.19 Dangerous area of the face.

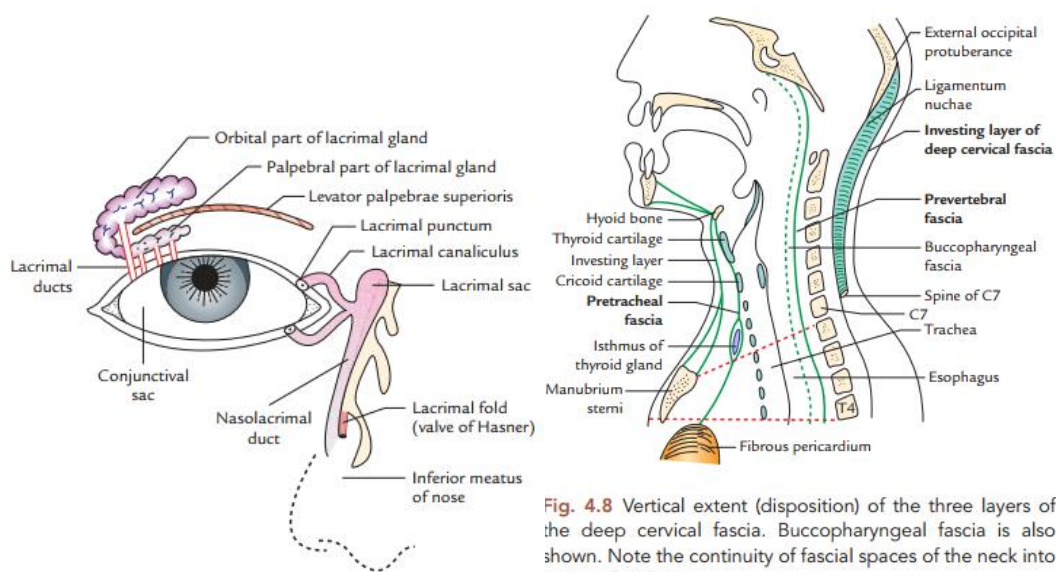


Fig. 4.8 Vertical extent (disposition) of the three layers of the deep cervical fascia. Buccopharyngeal fascia is also shown. Note the continuity of fascial spaces of the neck into the mediastinum.

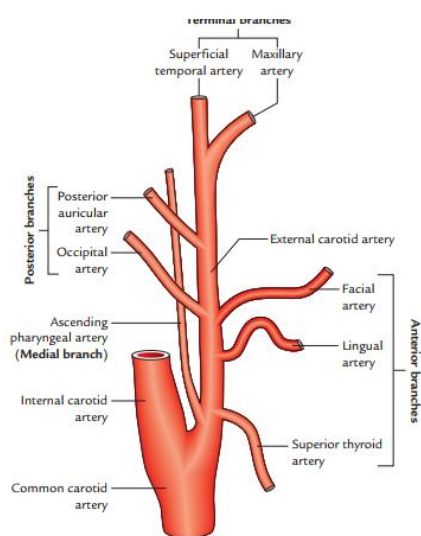


Fig. 16.10 Branches of the external carotid artery.

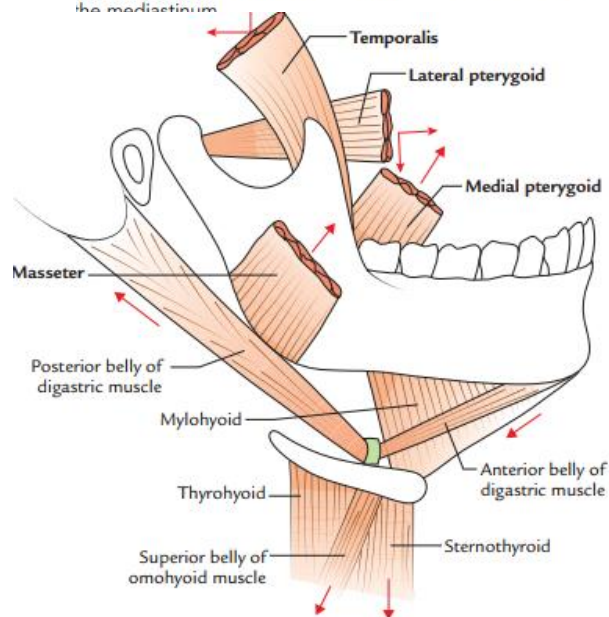


Fig. 10.20 Muscles of mastication producing movements of

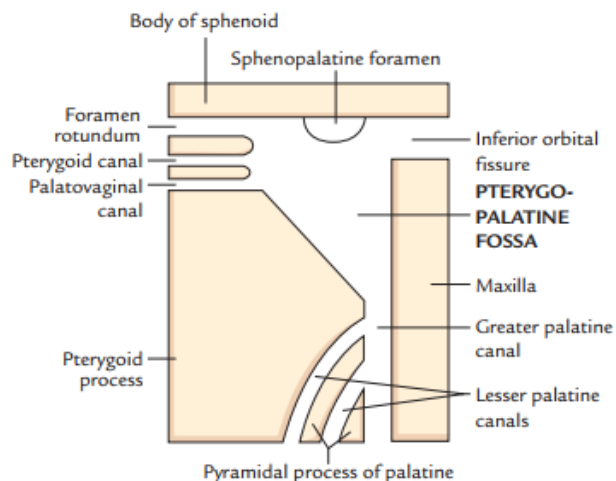


Fig. 10.23 Boundaries of the pterygopalatine fossa.

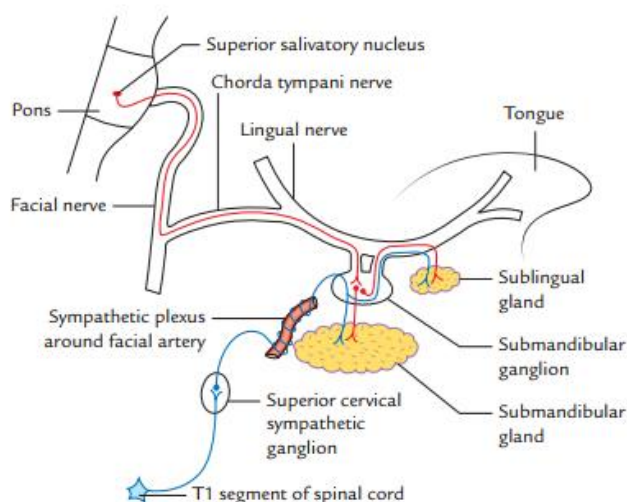


Fig. 9.21 Submandibular ganglion and its connections.

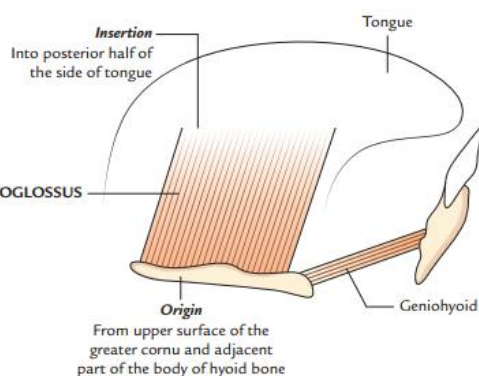
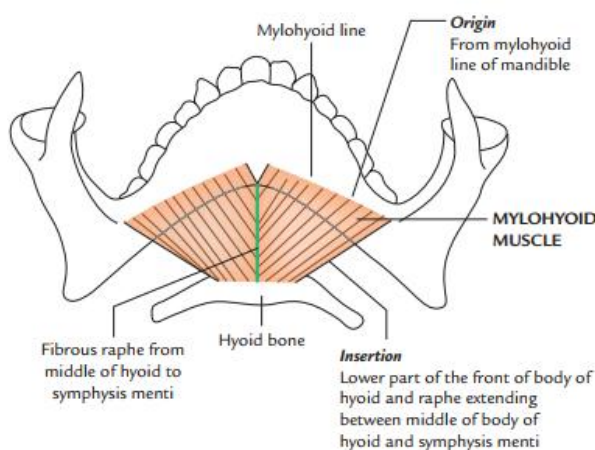
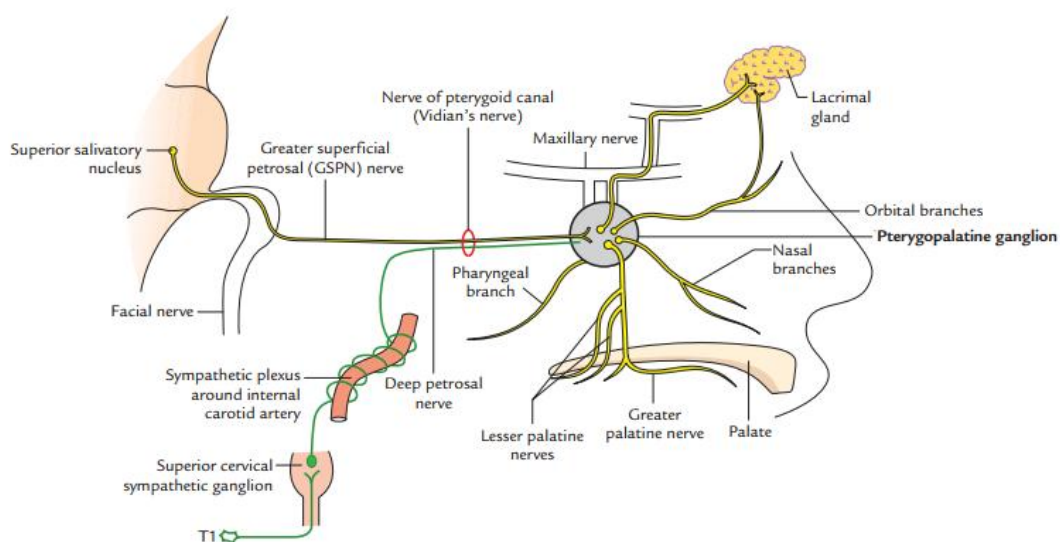


Fig. 9.7 Origin and insertion of the hyoglossus muscle.

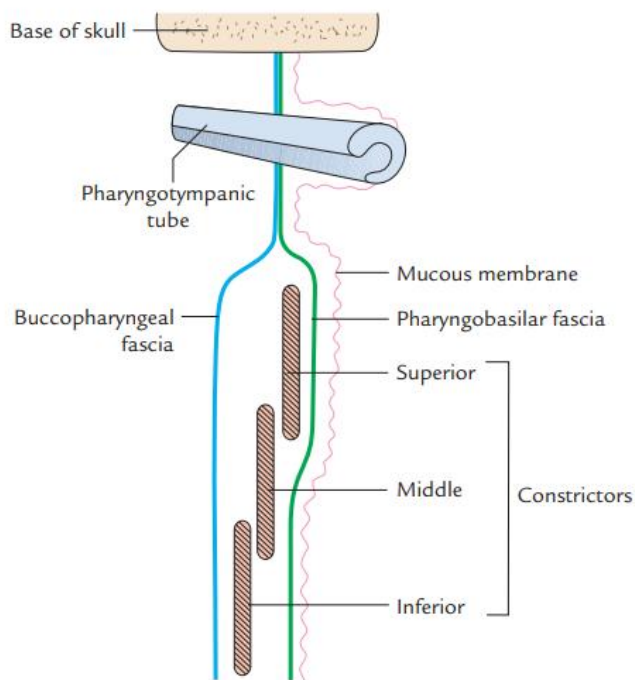
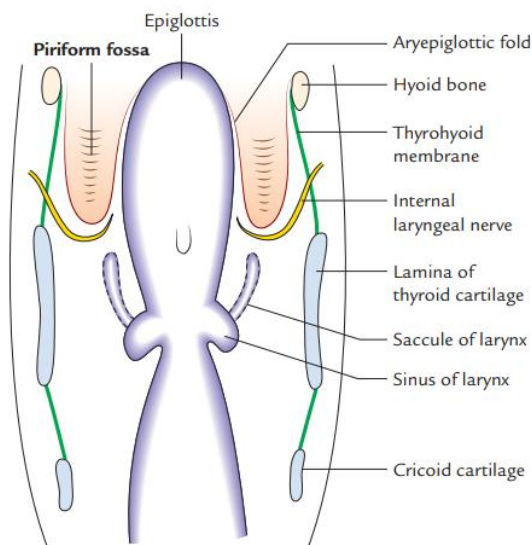


Fig. 14.7 Structure of the pharyngeal wall.

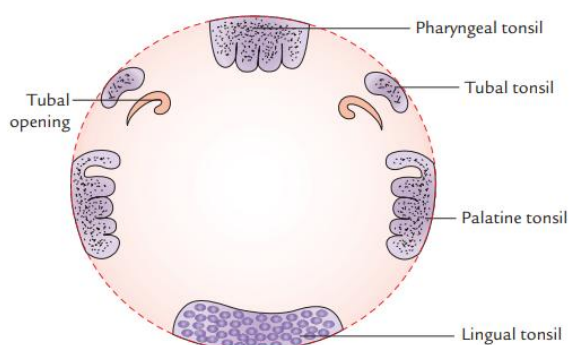


Fig. 14.8 Waldeyer's ring (an interrupted 'circle of tonsils' at the upper end of the respiratory and alimentary tracts).

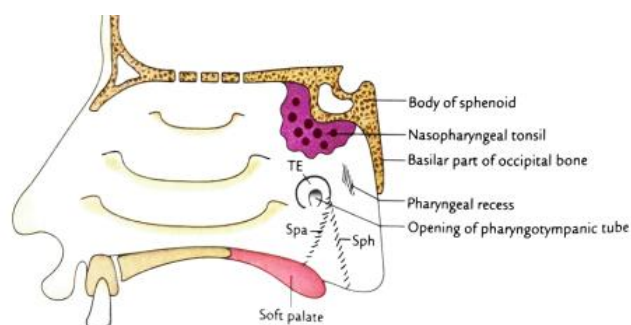


Fig. 14.2 Nasopharynx. TE, tubal elevation; Spa, salpingopalatine fold; Sph, salpingopharyngeal fold

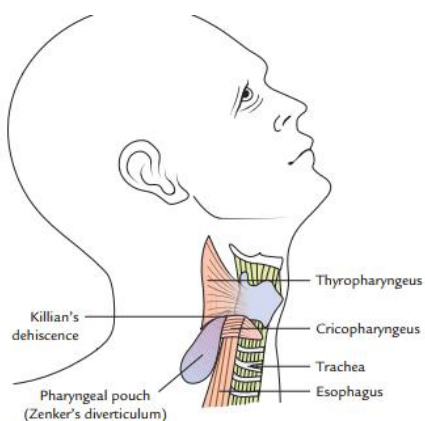
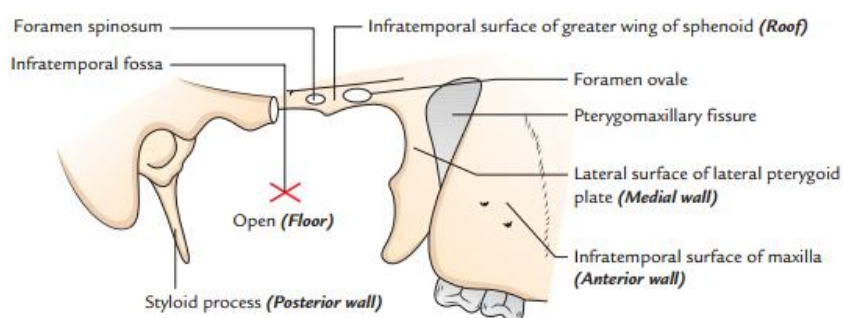


Fig. 14.12 Pharyngeal diverticulum.



of the infratemporal fossa.

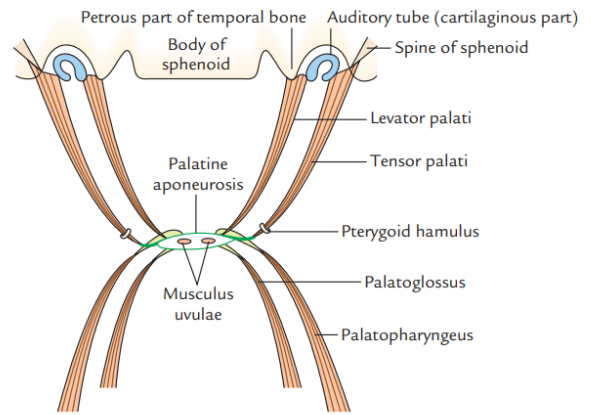
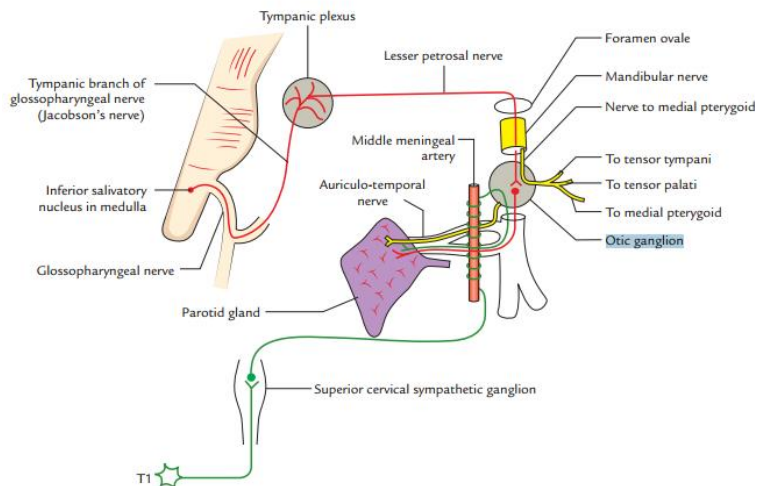


Fig. 14.21 Muscles of the soft palate.

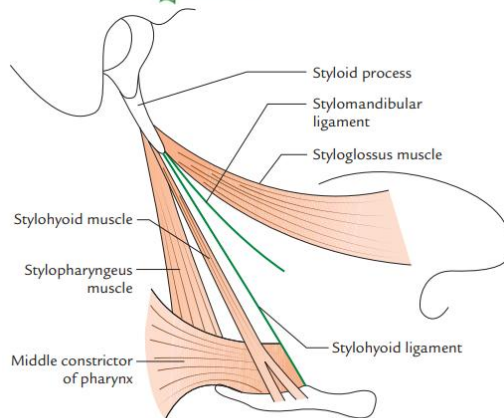


Fig. 16.19 Styloid apparatus.

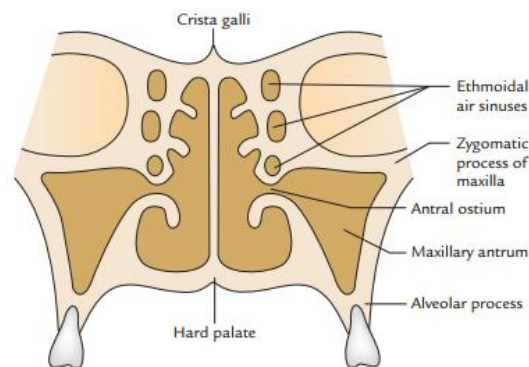
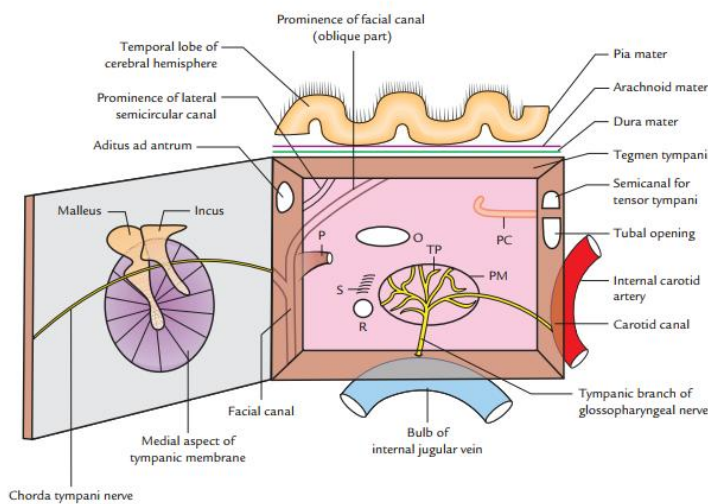


Fig. 17.17 Relations of maxillary sinus.



Schematic diagram to show the boundaries (and their relations) of the middle ear. The middle ear is

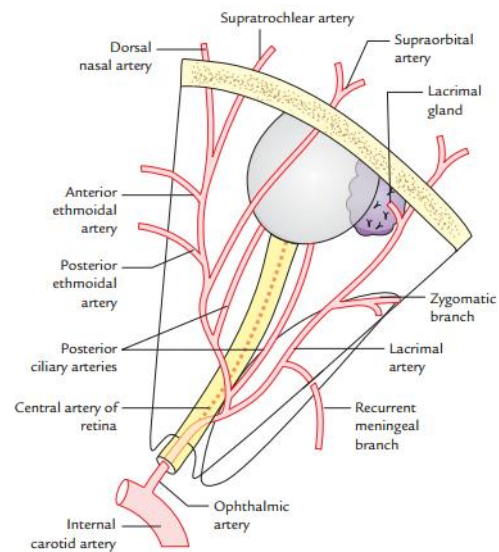


Fig. 19.17 Ophthalmic artery.

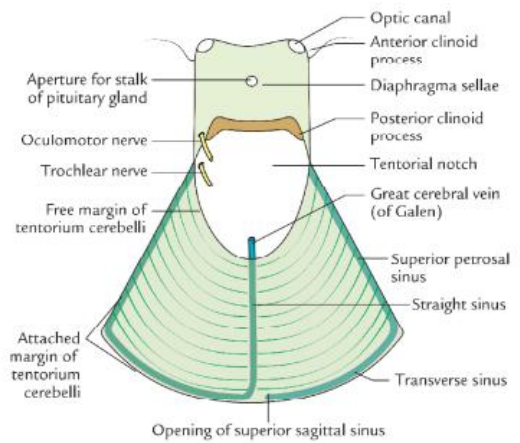
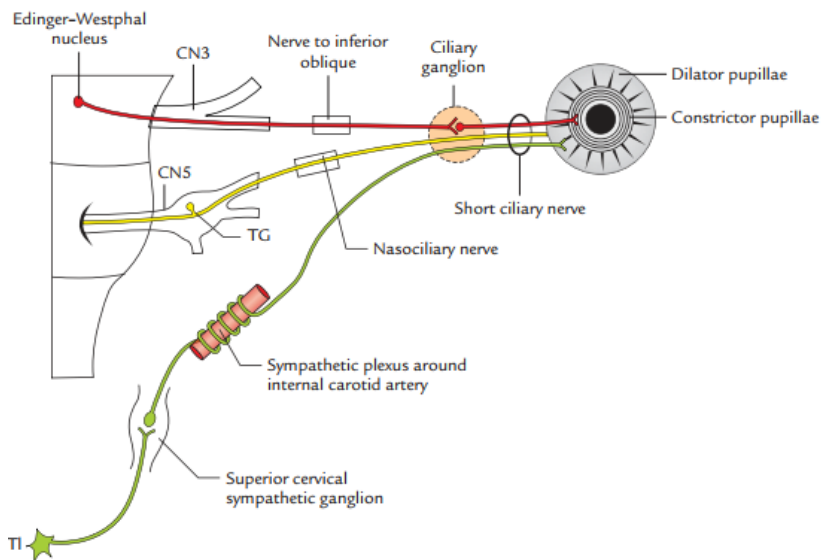
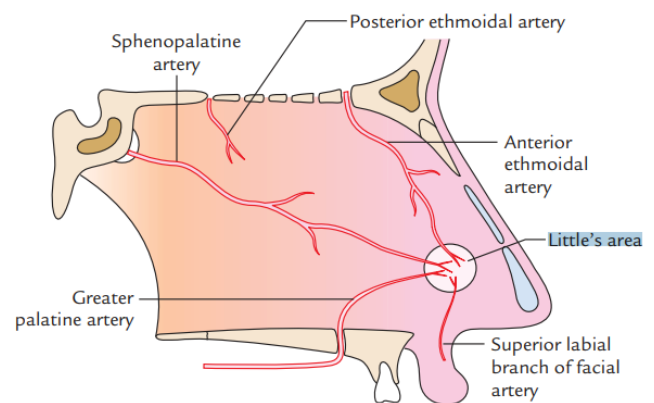
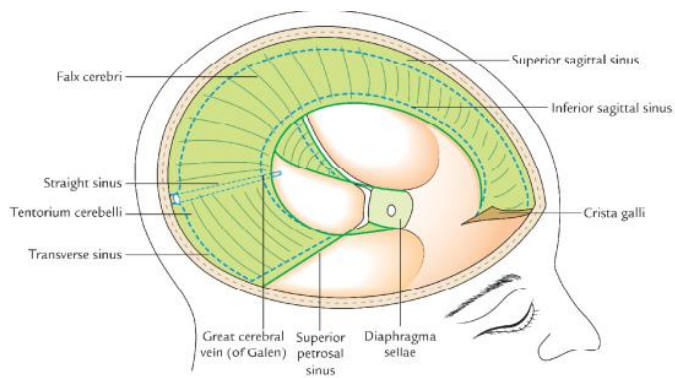


Fig. 21.13 Tentorium cerebelli as seen from above.



I folds and dural venous sinuses enclosed within them, viewed from superolateral aspect.

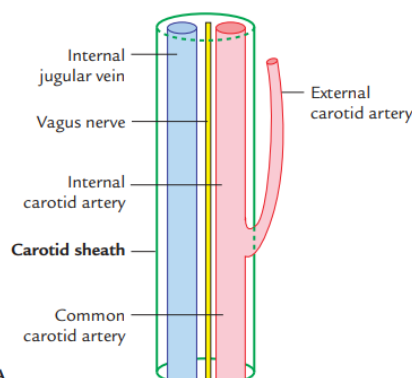
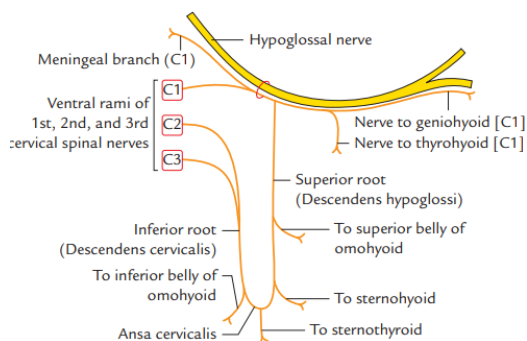
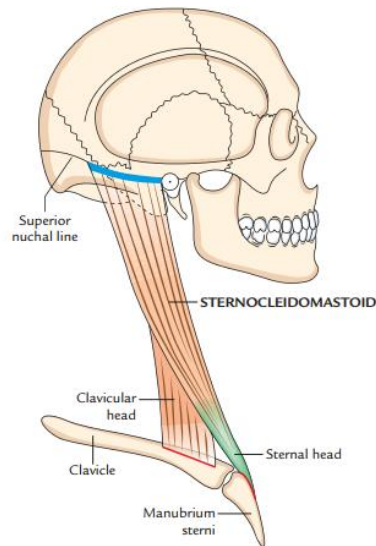
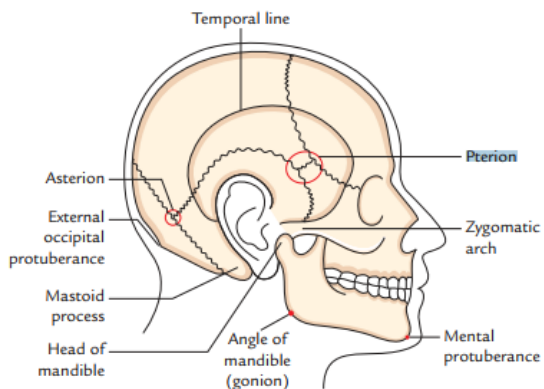


Fig. 6.7 Formation and distribution of the ansa cervicalis.

NEUROANATOMY

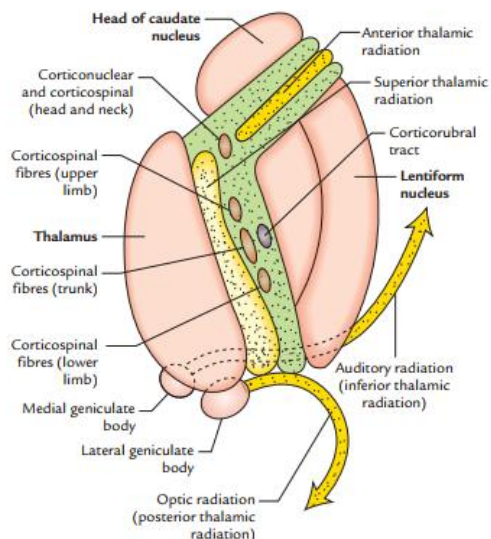
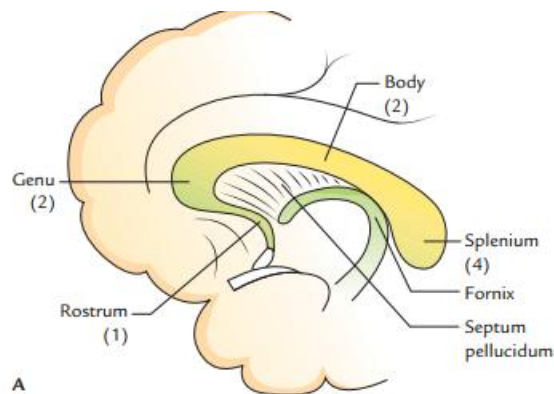
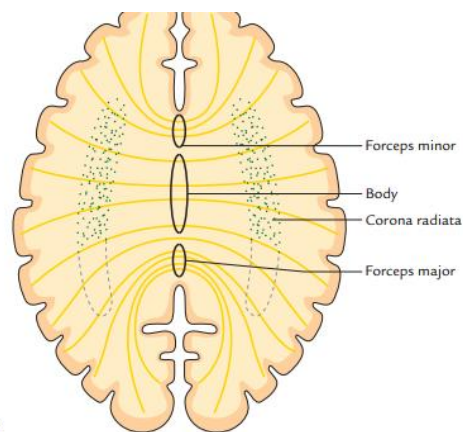


Fig. 27.4 Main components of internal capsule and their constituent fibres (yellow = thalamocortical, green = cortico-pontine fibres, red = corticonuclear and corticospinal fibres, violet = corticorubral fibres).



A



B

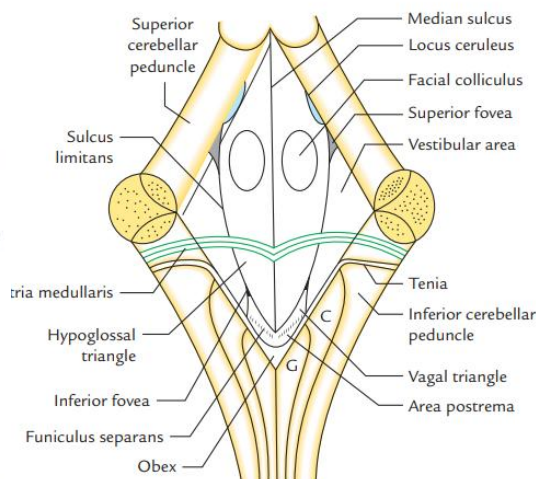


Fig. 27.3 Corpus callosum: A, parts of corpus callosum as seen in median sagittal section of the cerebrum; B, fibres of corpus callosum as seen in transverse section of cerebrum through corpus callosum.

g. 25.5 Rhomboid fossa (floor of the 4th ventricle) = cuneate tubercle, G = gracile tubercle).

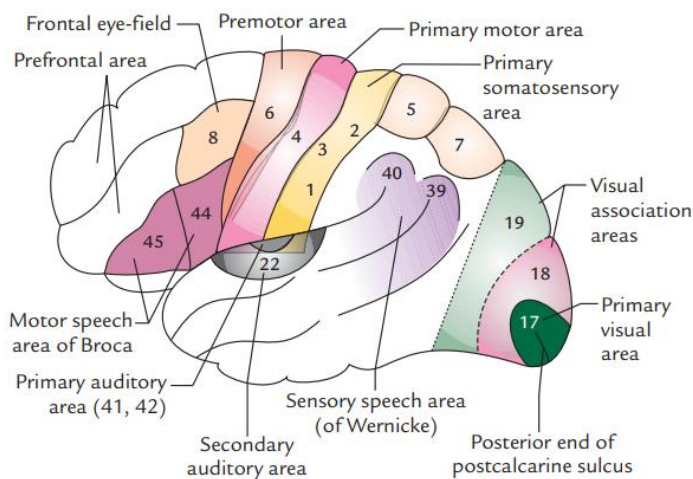


Fig. 27.1 Functional cortical areas on the superolateral surface of the cerebral hemisphere.

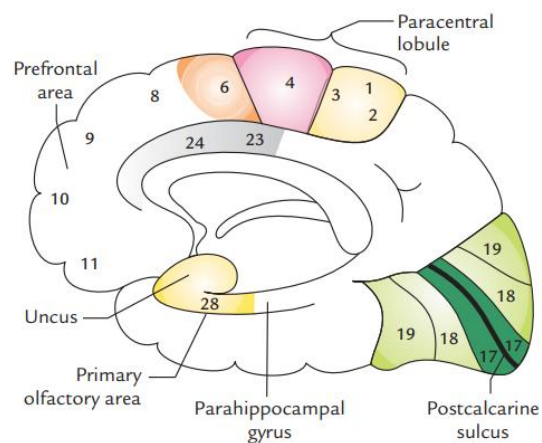


Fig. 27.2 Functional cortical areas on the inferomedial surface of the cerebral hemisphere.

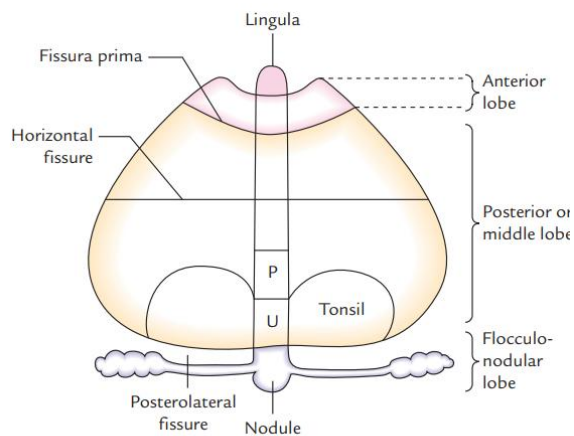


Fig. 25.3 Anatomical lobes of the cerebellum.

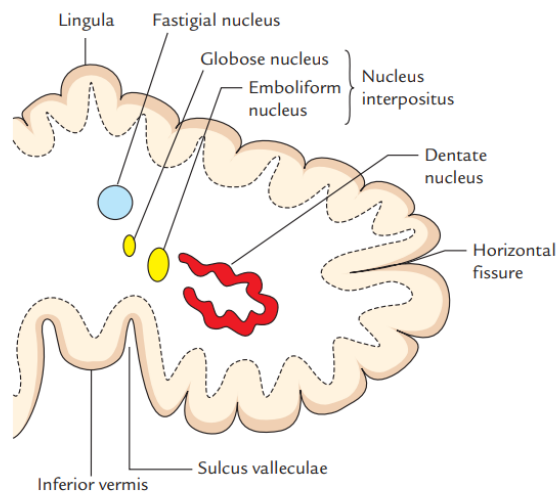


Fig. 25.4 Transverse section of the cerebellum showing intracerebellar nuclei.

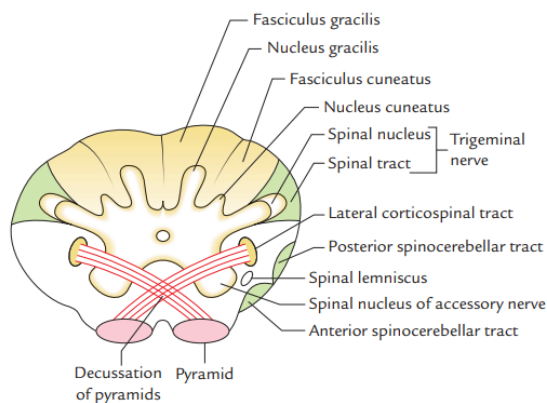


Fig. 24.3 Transverse section of medulla at the level of sensory decussation.

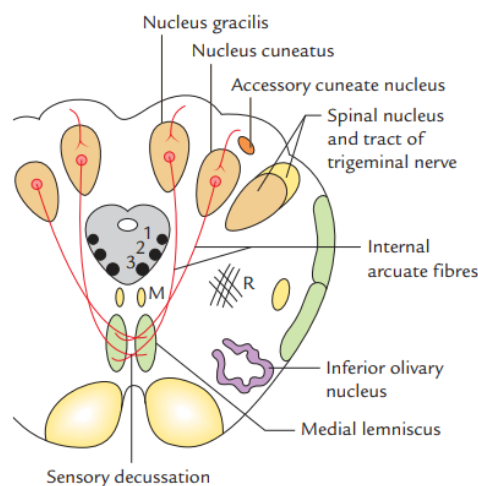


Fig. 24.4 Transverse section of medulla at the level of sensory decussation (1 = nucleus tractus solitarius, 2 = dorsal nucleus of vagus, 3 = hypoglossal nucleus; M = medial longitudinal fasciculus, R = reticular formation).

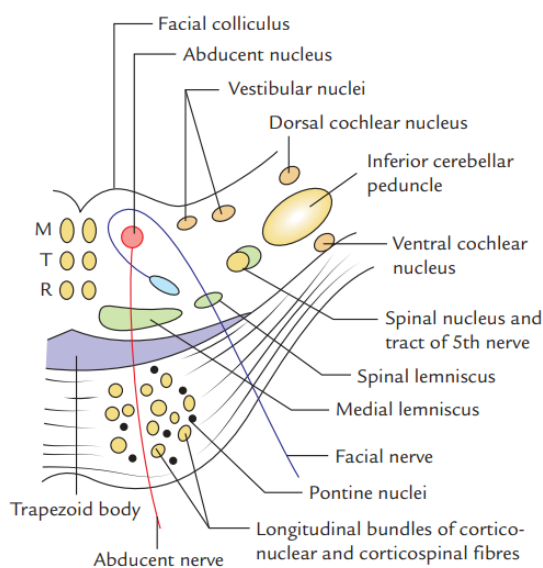


Fig. 24.6 Transverse section through the lower part of the pons (M = medial longitudinal bundle, R = rubrospinal tract, T = tectospinal tract).

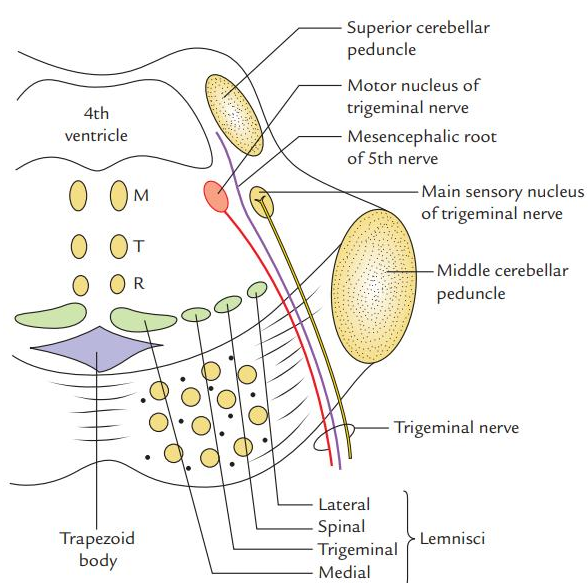


Fig. 24.7 Transverse section through the upper part of the pons (M = medial longitudinal bundle, R = rubrospinal tract, T = tectospinal tract).

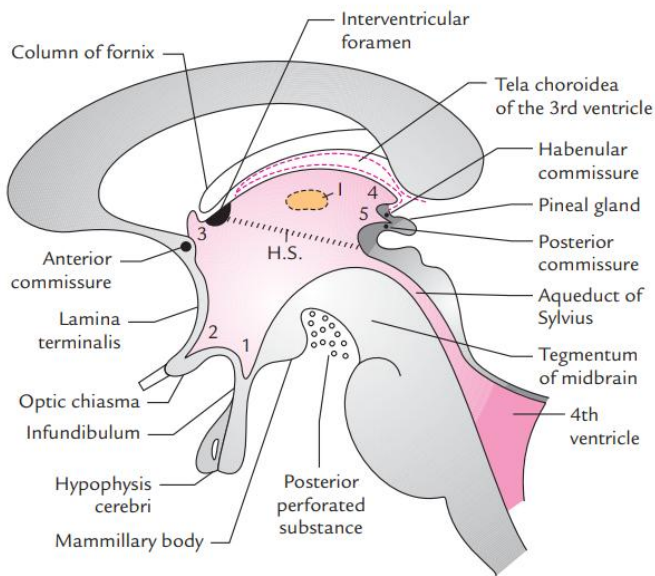
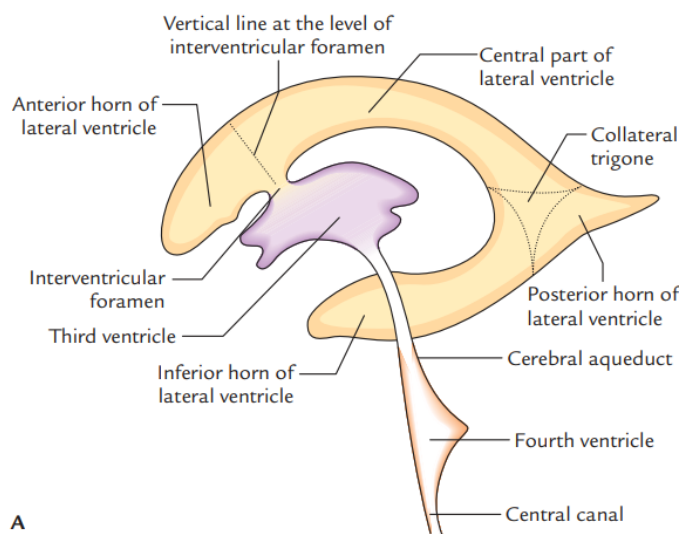
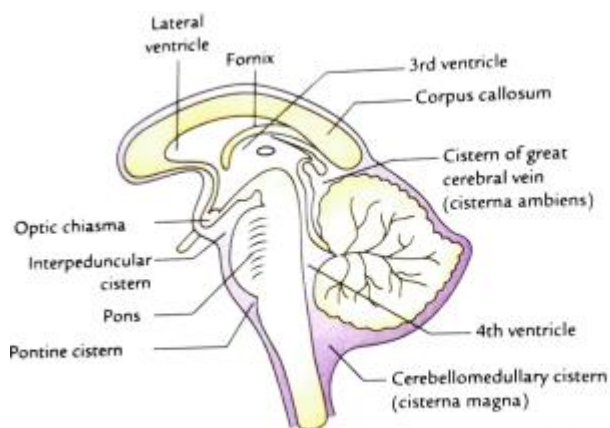


Fig. 26.4 Boundaries and recesses of the 3rd ventricle as seen in sagittal section (HS = hypothalamic sulcus, I = interthalamic adhesion, 1 = infundibular recess, 2 = optic recess, 3 = anterior recess, 4 = suprapineal recess, 5 = pineal recess).



A



al section of the brain showing location of principal subarachnoid cisterns.

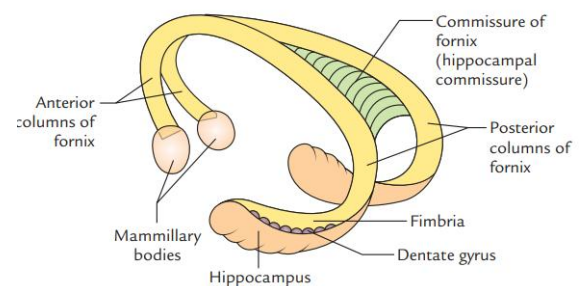


Fig. 28.5 Main parts of the fornix.

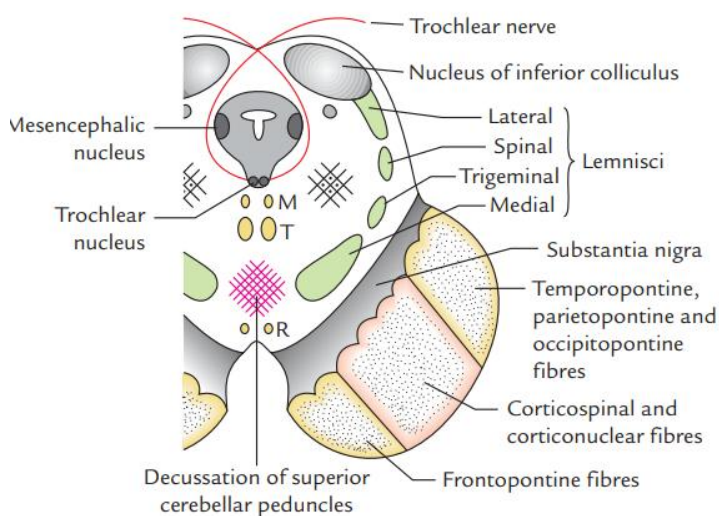


Fig. 24.8 Transverse section of the midbrain at the level of inferior colliculi (M = medial longitudinal bundle, R = rubrospinal tract. T = tectospinal tract).

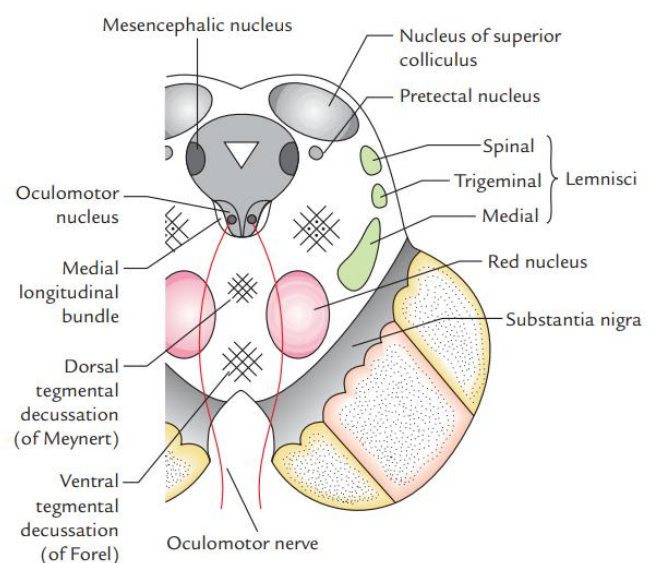


Fig. 24.9 Transverse section of the midbrain at the level of superior colliculi.

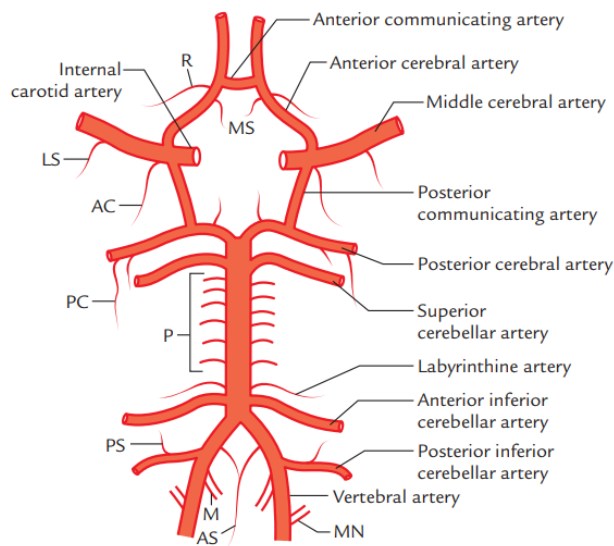


Fig. 29.2 Circle of Willis (AS = anterior spinal artery, LS = lateral striate artery, M = medullary branches, MN = meningeal branches, MS = medial striate arteries, P = pontine branches, PC = posterior choroidal artery, PS = posterior spinal artery, R = recurrent artery of Heubner, AC = anterior choroidal artery).

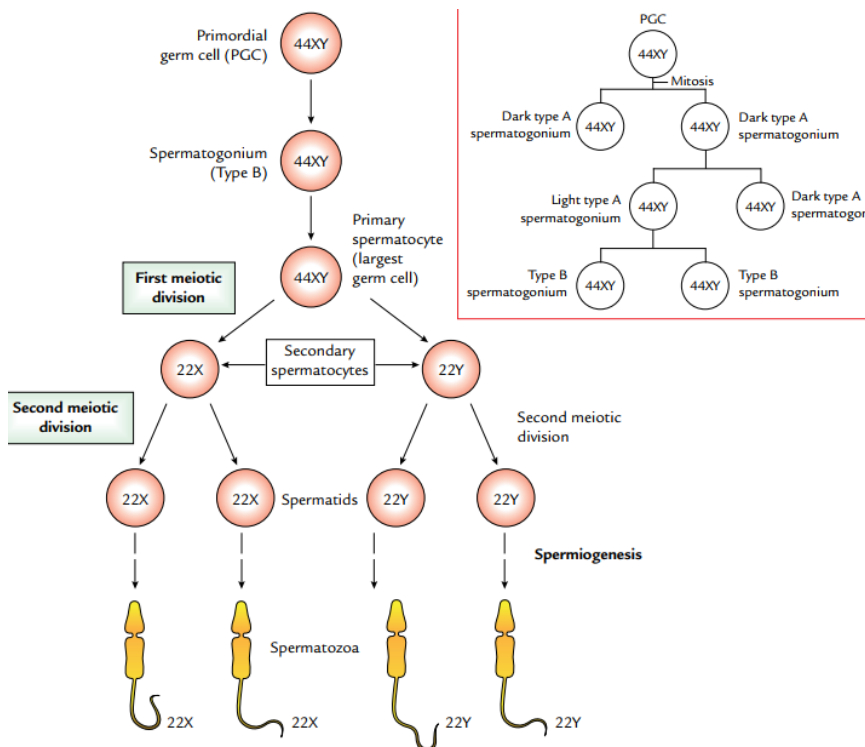
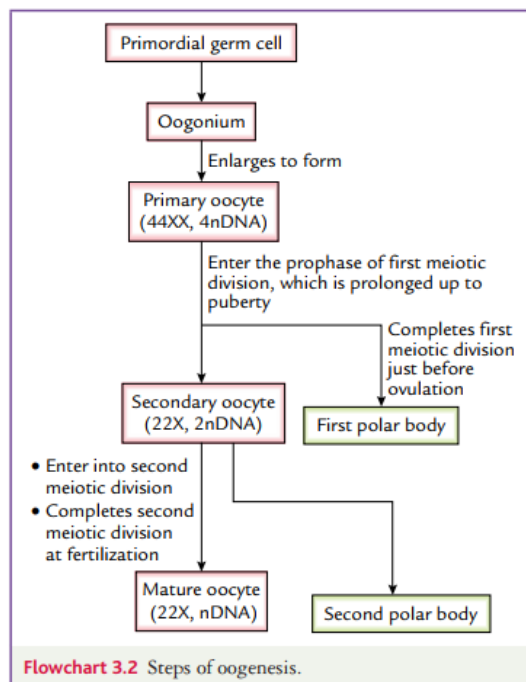
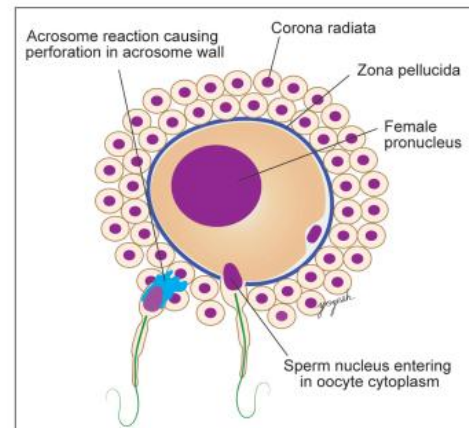
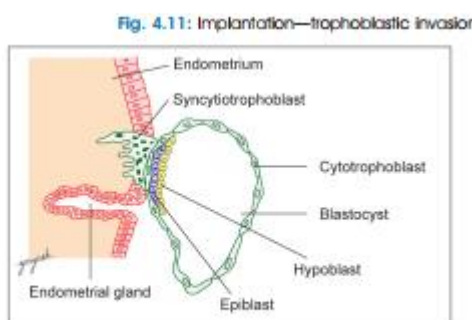


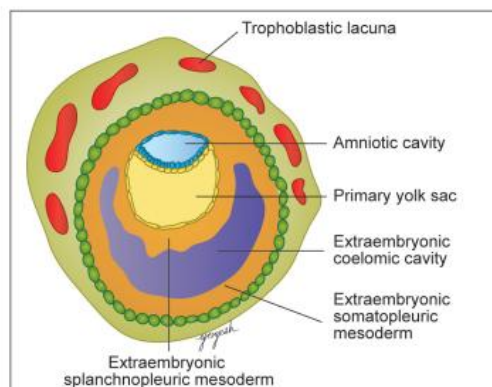
Fig. 3.4 Spermatogenesis (simplified form). Figure in the inset shows spermatocytogenesis.



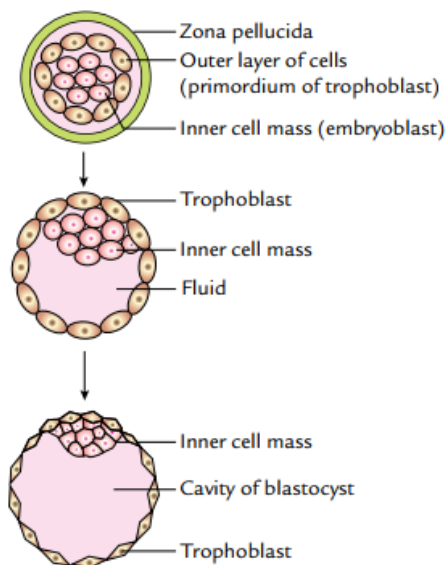
Practice Fig. 4.1: Fertilisation



Practice Fig. 4.4: Implantation



Practice Fig. 9.4: Yolk sac, amniotic cavity and extraembryonic coelomic cavity



5 Formation of blastocysts.

Flowchart 9.1: Structure of the placenta

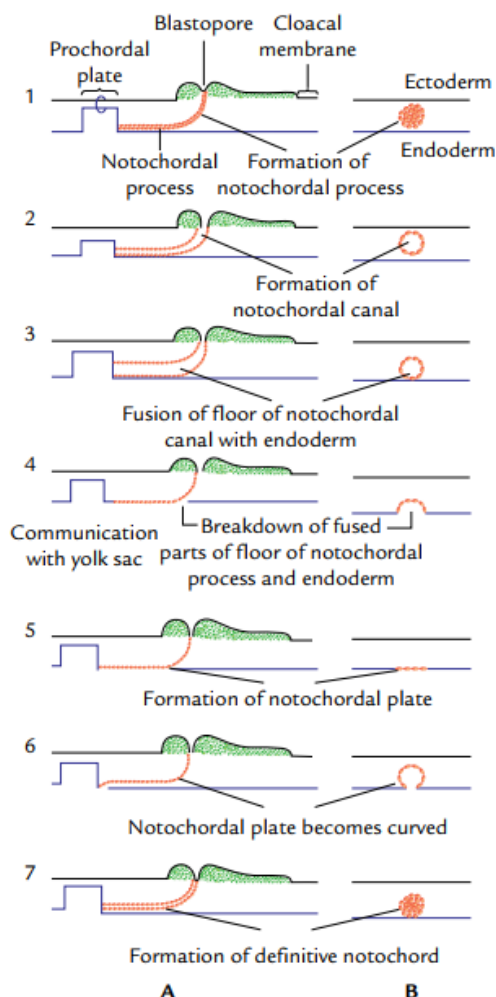
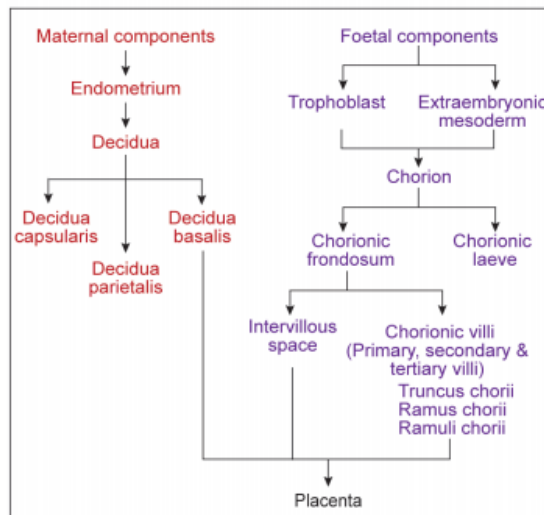


Fig. 5.2 Stages of formation of notochord. A. As seen in longitudinal section (left side). B. As seen in the horizontal section (right side).

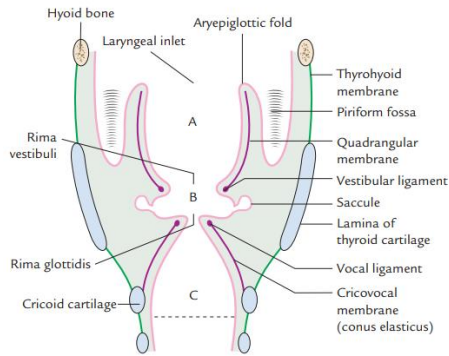


Fig. 15.10 Coronal section of the laryngeal cavity showing its subdivisions. A=vestibule, B=ventricle of the larynx, C=infraglottic compartment.

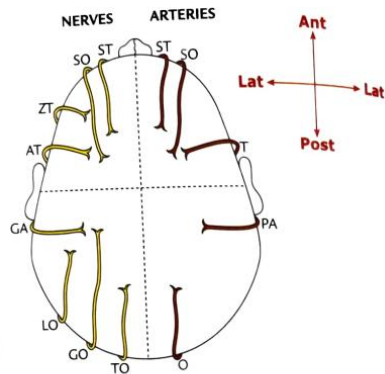
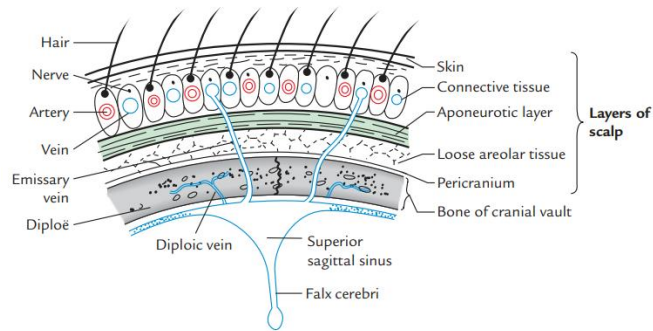


Fig. 9.2 Arteries (right half) and sensory nerves (left half) supplying the scalp. ST, supratrochlear; SO, supraorbital; T, superficial temporal; PA, posterior auricular; O, occipital; ZT, zygomaticotemporal; AT, auriculo-temporal; GA, great auricular; LO, lesser occipital; GO, greater occipital; TO, third occipital.

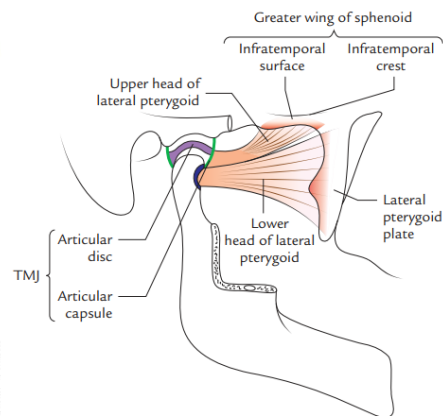


Fig. 10.4 Origin and insertion of the lateral pterygoid muscle.

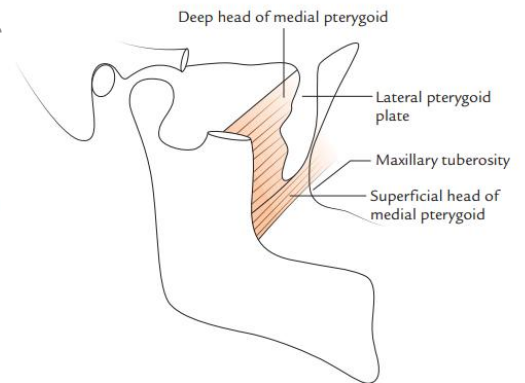


Fig. 10.6 Origin and insertion of the medial pterygoid muscle.

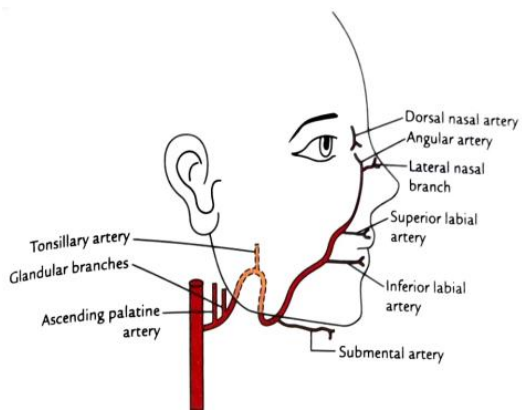


Fig. 10.11 Course and branches of the facial artery.